NYC HOME-STAT

Stakeholder Research Insights Report

Change Log

Version 1
First Release

Version 2
Cleaned up formatting and added details, overview journey map, and areas of interest chapters.

Version 3
This version includes feedback from Providers, DHS, HPD, and HRA received through written comments and additional interviews. It also documents some of the process changes underway since the launch of HOME-STAT in December 2015.

Table of Contents

2 Introduction

4 Process and Methodology

6 Journey Map

10 Service Delivery Journey

12 Identifying Street Homeless

26 Homeless Contact and Response

38 Health Crises, Hospitals, and Psychiatric Services

52 Moving onto Caseload

68 Gathering Vital Documents

80 Applying for Public Assistance

96 Securing an Income other than Public Assistance

100 Applying for Permanent Housing

118 Finding Housing Opportunities

124 Getting Linked with Housing

132 Transitioning into Housing and Aftercare

138 Areas of Interest

140 Special Populations

146 Cross-Agency Initiatives

160 Data and Systems

174 Public Perception and Action

180 Potential Enhancements

Introduction

Since 2007, the Department of Homeless Services, the Department of Health and Mental Hygiene, and their Providers have been conducting proactive street and subway outreach to individuals who are potentially homeless throughout New York City. The teams interact with anyone they encounter, but historically their focus has been on the chronically homeless, who have living on the street or subway for nine months of the past two years (see more on chronicity on page 53). They operate under a harm reduction and housing first model, with the ultimate goal of getting individuals permanently housed. Together they have moved thousands of people to permanent housing who otherwise may have remained on the street only to further decline.

These teams — the direct service providers with a shared expertise in serving the street and subway homeless — are central to the HOME-STAT initiative. This document details the process of their service delivery and key areas of interest related to their work. Additionally, it includes recommendations for enhancements to streamline and improve current processes.

"Placing people in housing gives me hope. There's nothing like giving someone the key to their apartment."

— Outreach Worker

Process and Methodology

Interviewed

37 government staff

6 NYC Agencies, 1 NY State Agency

18 program staff

5 Providers

7 clients

Shadowed

10 outreach staff

6 outreach teams

Process and Methodology for Research and Reporting

Throughout January and April 2016, the Innovation and Design Team at within the Mayor's Office of Operations (the "Ops team"):

- Interviewed Department of Homeless Services (DHS) staff;
- Interviewed DHS street and subway homeless outreach contracted Providers, including program directors, case managers, and outreach workers at:
  - Bowery Residents' Committee (BRC), contracted for subway outreach;
  - Breaking Ground (BG), contracted for Brooklyn and Queens outreach, subcontractor to MOC for Manhattan;
  - BronxWorks (BW), contracted for Bronx outreach;
  - Manhattan Outreach Consortium (MOC), comprised of Goddard Riverside, Breaking Ground and Center for Urban Community Services, contracted for Manhattan outreach;
  - Project Hospitality (PH), contracted for Staten Island outreach;
- Shadowed the outreach teams in the field;
- Met with homeless and formerly homeless individuals who are on the street, in transitional housing, and permanently housed;
- Interviewed NYPD Homeless Outreach Unit (HOU) officers;
- Interviewed the Mayor's Office of Criminal Justice (MOCJ) staff who manage the Safe Hub project.
- Interviewed Human Resources Administration (HRA) Family Independence Administration staff;
- Interviewed Housing Preservation and Development staff from Division of Tenant Resources, Policy and Operations in Asset and Property Management, and Special Needs Housing;
- Received report feedback from the HRA Placement, Assessment and Client Tracking Unit (PACT).

Based on the interviews and experiences, the Ops Team created "journey maps" to describe key points in the business process and worker and client experiences. On January 28th, 2016, the Ops Team hosted a workshop with DHS, the Providers, and other members of the Mayor's Office of Operations to validate the information contained in the journey maps and collaboratively identify areas for enhancement. The Ops team has completed follow up research and incorporated feedback from the stakeholders above.

This report is organized by major steps in the street to home process and each includes a summary, insights, detailed journey maps, and enhancement ideas. There are also area of interest chapters which detail key themes of the service delivery process.

Journey Map

This overview journey map of street homeless outreach reflects the business process and worker and client experience as it happens in current practice from initial observation, contact, case management, and placement in permanent housing. The map is displayed in eleven high-level sections each with individual sub-level sections. Summaries and details for all the sections are presented in the subsequent pages.

SECTION 1

IDENTIFYING STREET HOMELESS

Pages 12-25

- Canvassing by outreach Providers
- 311: HOME-STAT canvassing observations and notifications
- Organizations contacting Providers and interacting with homeless
- Homeless individuals reaching out for help
- Special requests for DHS Outreach Team

SECTION 2

CONTACT AND RESPONSE

Pages 26-37

- Outreach team attempts first contact
- Outreach team gathers basic info and offers services
- Outreach team repeats contact
- Person responds to outreach team

SECTION 3

HEALTH CRISES

Pages 38-51

- Person is unresponsive or needs medical attention
- Person is a danger to themselves or others and in need of involuntary removal
- Person needs psychiatric services on street

SECTION 4

MOVING ONTO CASELOAD

Pages 52-67

- Gathering more information about individual and assesses eligibility for services
- Case notes are transferred into databases and physical case files
- Case manager conducts psychosocial interview with client
- Case manager identifies transitional or permanent housing for client

SECTION 5

GATHERING VITAL DOCUMENTS

Pages 68-79

- Getting a Social Security card
- Getting a replacement benefit card
- Getting a birth certificate
- Getting a State ID
- Getting an idNYC
- Getting proof of military service
- Applying for or replacing a Permanent Resident card
- Applying for or replacing a naturalization certificate
- Securing an official name change

SECTION 6

APPLYING FOR PUBLIC ASSISTANCE

Pages 80-95

- Applying for Public Assistance
- WeCARE

SECTION 7

SECURING AN INCOME OTHER THAN PUBLIC ASSISTANCE

Pages 96-99

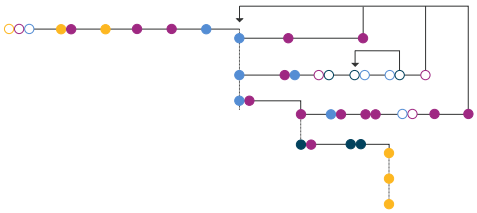
- Applying for Supplemental Security or Social Security Disability Income (SSI or SSD)
- Documenting Veterans benefits
- Documenting pension benefits
- Getting proof of income (for clients who are employed)
- Documenting informal income

SECTION 8

APPLYING FOR PERMANENT HOUSING

Pages 100-117

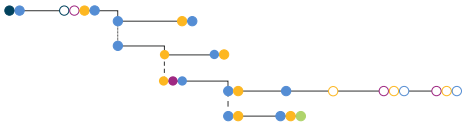
Submitting an HRA 2010e supportive housing referral



Re-submitting an HRA 2010e supportive housing referral

Applying for general population housing

Obtaining an HPD Section 8 voucher



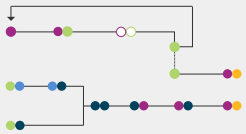
Waiting for housing to become available

SECTION 9

FINDING HOUSING OPPORTUNITIES

Pages 118-123

Finding supportive housing opportunities



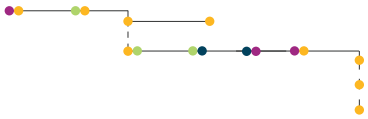
Accessing mental health housing through SPOA

SECTION 10

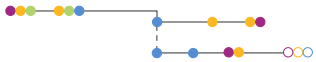
GETTING LINKED TO HOUSING

Pages 124-131

Getting linked to supportive housing



Securing a tenant-based Section 8 rental subsidy



SECTION 11

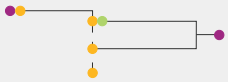
TRANSITIONING INTO HOUSING

Pages 132-137

Client applies for One Shot Deal

Client signs lease and moves in

Case manager provides aftercare



Service Delivery Journey

Section 1

Identifying Street Homeless

- Canvassing by Outreach Providers
- 311: Public & HOME-STAT Canvassing observations and notifications
- Organizations contacting Providers and interacting with homeless
- Homeless individuals reaching out for help
- Special requests for DHS Outreach Team

Enhancements: page 20

Reports of potentially homeless individuals come to the City and DHS Providers through several sources. Ongoing outreach and canvassing by City and Provider teams combined with reports from 311 and community organizations make up the majority of reports. Many of the reported individuals are not chronically street homeless. Additionally, some homeless individuals may contact the Providers themselves to request services.

Canvassing by Outreach Providers

OCCURS DAILY

The Provider outreach teams look for key signs that someone may be homeless including bedding down, sleeping on a park bench or panhandling. These individuals may not necessarily be homeless and outreach workers interact with individuals to confirm their status (see pages 28-31). (Note that this differs from the separate HOME-STAT Canvassing teams (see page 15), who do not confirm whether or not individuals are sheltered but report based on appearances.) Outreach workers also noted that some street homeless individuals tend to congregate in areas with high foot traffic from public transit lines and 24-hour stores. Some Providers have canvassing routes and others do not have capacity to canvass their full borough. Routes are largely designated by where they believe people are and have been spotted frequently.

Journey Map Key

- Potential Client / Client
- DHS Provider
- DHS
- NYC, non-DHS
- State or Fed Office
- Other Provider

- Required step
- Optional step

- Providers perform daily observations in their assigned boroughs and subway system
- When off-duty, Providers spot areas that warrant further investigation
- Providers watch the news and search online

Time
Sometimes canvassers find belongings and evidence of bedding down, but no individuals. In these instances, the outreach teams will return daily until contact has been made.

Tools and Resources
Outreach teams log details of locations, individuals and contacts during their shift. Methods for logging these notes vary across Providers from using clipboards to personal phones.

Canvassing by Outreach Providers (continued)

In the subway system, outreach teams walk the entire station and check every stairwell. The 469 stations are visited in six week intervals. They also look for doors or gates that may be ajar, and check platforms beyond the public area. When they go down into a station, they check in with the station agent and ask if the agent has seen any homeless individuals. The team records the agent's badge number and time. Outreach teams also ask MTA maintenance and cleaning workers about who is currently in the station and recent conditions.

The research team observed a handful of interactions with front line MTA staff, including:

- Station agents who were unfamiliar with BRC and say that they get moved from station to station, so they are not familiar with who is at this station.
- The MTA staff said things like "they're not going anywhere" and "tell them to take their cardboards with them."
- In another instance, an MTA worker said that verifying if they've seen a homeless individual before as part of verifying chronicity "didn't have anything to do with me."

"The homeless population is a huge challenge for us, huge challenge for our employees."

—MTA

MTA policy does not allow workers to engage directly with homeless individuals and they may be looking out for their personal safety in some of the observed interactions. Equipment cleaners have strict union rules to never engage with homeless individuals and not to disturb them. Station agents must report on individuals who have been reported by a staff member or patron. The MTA reports that employees sometimes feel like they're not seeing improvements, even though they are following procedures.

Daytime subway outreach teams do not go onto the trains as there is a daytime decline in homeless subway ridership that does not maximize resources. However, daytime subway outreach workers reported they do not to go into trains to check for homeless individuals, due to protecting people's privacy and for safety in case someone gets upset. The overnight subway outreach teams conduct joint operations with NYPD's Homeless Outreach Unit (HOU) and Transit District (TD) and canvass stations and subway cars at the end of the line. The outreach team prefers to be accompanied by law enforcement in order to do on-board outreach on a moving train. NYPD assists in waking individuals and providing transportation if someone accepts services. Outreach teams report that the NYPD TD is not always present during outreach.

The MTA has a team to do overnight counts in some terminals and report back to headquarters with a daily list. New York City Transit (NYCT) has a group that inspects all underground rooms and make sure they're secure, both to ensure system safety and security and ensure the safety of homeless individuals.

Canvassing by Outreach Providers (continued)

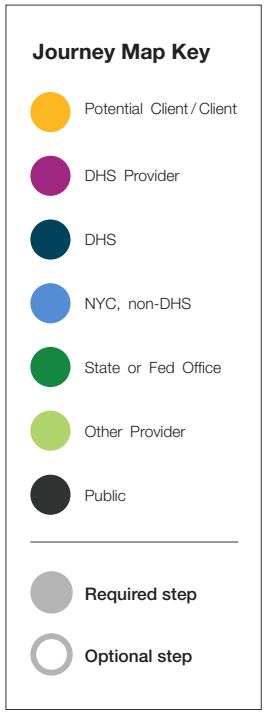
When a homeless individual is found in a private area, they are deemed to be trespassing and the NYPD is contacted to remove the individual . The MTA also attempts to bring in BRC.

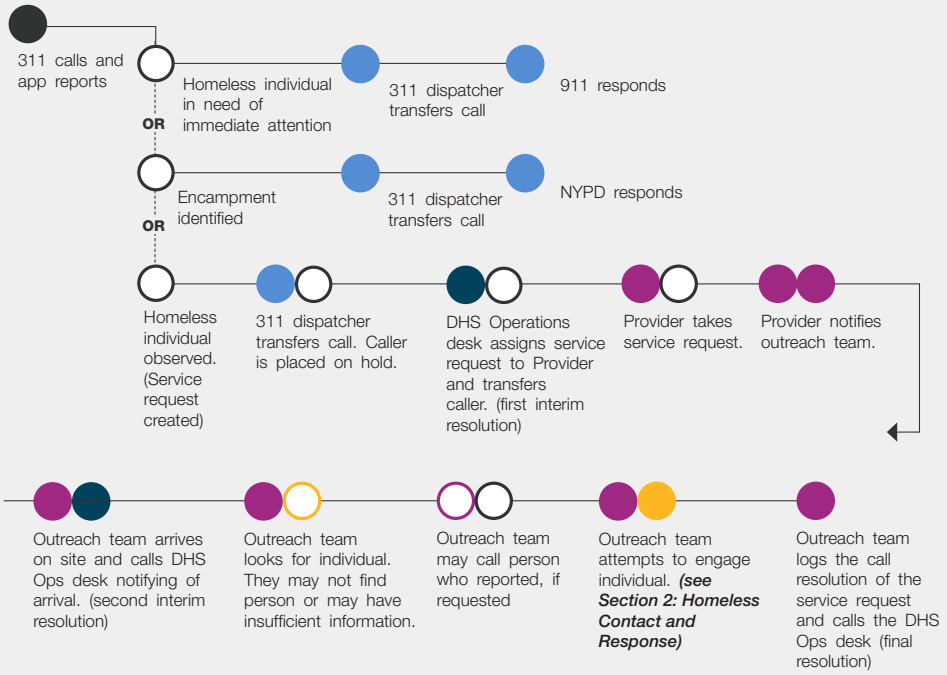
Outreach workers are often on the lookout for individuals even when they are off-duty. They sometimes spot encampments and log them to follow up on when they are back at work. Others watch the news and search online for keywords to find new individuals and also learn more about people who may be known to the outreach teams, but not providing information.

311: HOME-STAT canvassing observations and notifications

1-2 HOURS

Members of the public and the HOME-STAT canvassing team report homeless individuals through 311. The public uses both phone calls and the 311 mobile application and the HOME-STAT canvassing teams use solely the mobile app. Between January and April 2016, the Mayor's Office of Operations, DHS, 311, and the Providers worked to implement a new 311 process, which is reflected below. Continued enhancements are underway.





Note: Business process change underway

311: HOME-STAT canvassing observations and notifications (continued)

Time
Providers are required to respond to their 311 calls within an average of 1 hour. Response times may be longer if the Provider is already engaged with an individual who needs extra attention.

Requests spike during morning commute, lunch time, and evening commute.

Tools and Resources
Provider outreach teams are notified of 311 services requests (SRs) by their dispatchers who receive SRs via phone and email from the DHS Operations Desk. SRs include timestamps for when they were created, when they were assigned to a Provider, when the Provider arrived on site, and when they were closed. A final resolution describes the outcome of the SR to DHS. In addition, SR's containing information for all relevant "rapid" calls routed to NYPD are reported.

All SRs are entered in the Siebel system, and SR data can be accessed through the Citywide Performance Reporting (CPR) business intelligence tool and the NYC OpenData portal. Providers continue to maintain their own logs, as this was their only formal way of reporting prior to storing data in Siebel. Previously, reporting back to DHS was periodic, limited and not consistent across Providers.

If the person is in need of immediate medical assistance, 311 routes the calls to 911. If the call is about an encampment, 311 routes the call to NYPD. If the call is about a potentially homeless person in need of assistance, a service request is created and the call is routed to the DHS Operations desk. The DHS Operations desk assigns the service request to the appropriate Provider and transfers the caller. Providers are required to respond to their 311 calls within an average of 1 hour. Response times may be longer if the Provider is already engaged with an individual who needs extra attention. If another person has expressed willingness to come inside, Providers may de-prioritize 311 calls in order to ensure that the first individual receives services.

311 dispositions for calls requesting outreach assistance include:

- Assistance offered
- Refused assistance
- Referred to 911
- Person not found
- Insufficient information
- Out of network provider
- Out of jurisdiction

Providers offer services to anyone they encounter, however Providers report that the individuals who are found via 311 service requests are often not the chronic street homeless population.

Example

Via the 311 mobile app, reported a highly visible person with many belongings on a Manhattan street corner. Asked for a call back. Request was closed in app and no call back received.

Example

Called 311 about three male individuals who were sleeping on a mattress surrounded by several trash bags of belongings inside a subway station. Transferred to DHS' Mobile Outreach Unit, put on hold, but then transferred to a "Cisco Unity Voicemail". Left a description of the individuals and requested outreach services for them. Did not receive a follow-up.

Example

Via the 311 mobile app, reported an individual who appeared homeless, was panhandling and wearing multiple hospital bracelets walking between subway cars. Gave detailed time, train line, and appearance information. Asked for a call back. Request was closed in app and no call back received.

311: HOME-STAT canvassing observations and notifications (continued)

Several Providers reported that 311 calls can be problematic if the descriptions are not detailed enough to identify an individual. Descriptions like "homeless individual" or "old lady on sidewalk" were cited as examples of 311 reports that usually do not result in finding the individual. DHS and Providers reported that their preference is to speak with the caller, as sometimes callers and people using the 311 app will give their location instead of the location of the person in need.

There can often be multiple 311 requests within an hour about a single individual or location (likely a hotspot). Providers are trying to efficiently group requests, but if a call is about the same person/location at a different time of day, they dispatch the outreach team again. Some Providers have limited outreach capacity, making it difficult to respond to multiple calls within the average of 1 hour time limit.

DHS also reported that a number of calls unrelated to encampments are being mistakenly transferred to NYPD. These calls include 311 inquiries about potentially homeless individuals in the subways.

Calls transferred to NYPD are referred to as "rapid calls", which depend on the 311 dispatcher's interpretation of the information provided. Providers report that many of these calls are about abandoned buildings, which the outreach team are not equipped to handle. The outreach teams are able to canvas outside of the buildings, but can not go into the structure. Certain areas experience higher volumes of calls to abandoned buildings, including Staten Island and the Rockaways.

NYC 311

Example

An individual lying down on the seat of an empty subway car had a very strong odor that made caller believe they were sick and in need of services. Called 311 with a complete description of the person, which train and car and what time the train was in the station but was told there was nothing they could do. Transferred to a DHS operator and received the same response. Caller insisted to be transferred to an Outreach Provider who again said that because the individual was on the train, the call was not actionable. Information was acknowledged but there was no offer for a call back.

NYC 311

Example

Observed a man on the street who appeared homeless. Opened 311 app to request homeless assistance. Looked up address in web browser as hard to search in app. Requested assistance and immediately received alert and email that request was received, followed by an alert the outreach team was dispatched, followed by alert that team arrived, and then received both a phone call from the Provider, email, and alert that the team offered services, but the individual did not accept assistance.

Organizations contacting Providers and interacting with homeless

OCCURS DAILY

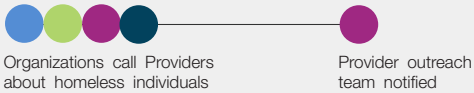
Providers are contacted directly via phone or email by the DHS Street Homeless Unit or community organization may contact them if an individual appears to need services. These calls depend heavily on the relationship the Provider has with the referring institution and awareness of the Provider's work within the community. Most Program Directors mentioned attending meetings with community boards, NYPD, or the Parks department and will direct outreach efforts based on their reports.

BRC coordinates Roundtable meetings to review conditions within all of the transit districts, and discuss conditions and plan response at every station. DHS mentioned that there are always new community stakeholders to meet.

"This program does a lot more than any other. I tell people about this one. I love these people. These people are great, but they should notice homeless people faster, they'd save more people. A lot of people just give up."

—Client linked with permanent supportive housing, awaiting move-in

NYPD, DOT, Parks, DSNY, Elected Officials, State and Federal Agencies, Faith-based Organizations, Jails, Hospitals, Banks, Community Boards, Business Improvement Districts, Non-profits, Private Property Owners, Businesses, Private Security Companies



Time
In order to help verify street homeless status, Providers will often return repeatedly over 72 hours to verify if someone is still on the street overnight.

Journey Map Key

- Potential Client / Client
- DHS Provider
- DHS
- NYC, non-DHS
- State or Fed Office
- Other Provider

- Required step
- Optional step

Homeless individuals reaching out for help

OCCURS DAILY

Sometimes individuals who are already known to the Providers will call, either directly or through 311 to let the outreach teams know where they are.

(see journey map on next page)

Homeless individuals reaching out for help (continued)

Tools and Resources
Many individuals have personal phones that they can use to contact Providers directly.

Special requests for DHS outreach team

OCCURS DAILY

DHS has roles for two outreach team members and one driver who can respond as needed to investigate potential homeless activity. DHS staff report that these types of calls may or may not be critical, but require a rapid response that derails other work. Follow up on any requests is completed by the Providers.

Time
DHS staff say that these types of calls happen often and can disrupt primary work.

Journey Map Key

- Potential Client / Client
- DHS Provider
- DHS
- NYC, non-DHS
- State or Fed Office
- Other Provider

- **Required step**
- **Optional step**

Enhancements

Identifying Street Homeless

These enhancements were generated by DHS, Providers, and the Mayor's Office of Operations and include ideas for policy, funding, program design, service delivery, technology and data. They are organized by long-term, medium-term, and short-term time frames.

MEDIUM -TERM

Supply additional translation services / language line to the outreach teams.

The biggest needs are for Mandarin, Creole, and the hearing impaired.

Next Step: Identify which teams need additional services, explore services available through the City, or look into hiring staff who speak the languages.

Revise the MTA rules and rights to enforce loitering.

The subway is sheltering a large number of people who are resistant to leave and enforcement would help along with outreach. Also look into policies in other municipalities that limit people taking the trains back and forth.

Next Step: Review policies with MTA leadership.

Revise current practice of distributing funds based on HOPE counts.

Next Step: Explore alternative ways to allocate funding.

More funding to increase number of staff, to retain staff, to train staff, and increase office space.

Next Step: Review current budgets and funding.

MEDIUM -TERM (continued)

Expand canvass approach to non-public spaces (i.e. hospitals, libraries).

Next Step: Explore best routes for getting reports from hospitals and libraries or having City workers canvass key locations.

Utilize credible messengers by bringing on formerly chronically homeless to engage resistant clients.

Next Step: Learn more about the BronxWorks peer program with a similar model. Look to some existing criminal justice projects that use credible messengers.

Also found in: Health Crises

Formally acknowledge the efforts of individuals who were formerly chronically street homeless and now act as "credible messengers" and are encouraging currently homeless individuals to seek out help and services.

Next Step: Recruit credible messengers to be part of the peer program (see previous enhancement).

Also found in: Health Crises

Create better transportation options for clients to transitional housing or service providers to let Provider staff stay at office to do other work.

Allow clients to take city-funded transportation, like Access-a-Ride or create positions for designated drivers just for these clients.

Next Step: Review transportation options and City services available.

Also found in: Moving onto Caseload

MEDIUM -TERM (continued)

Create a team with the capability to work with clients who move into abandoned buildings.

Next Step: Explore which units across the City work with abandoned buildings and develop a partnership with outreach providers.

Also found in: Cross-Agency Initiatives

Update the 311 app to prompt users to add more detailed descriptions of the individual in need and enter a location without knowing the exact address.

For example, create specific prompt language or more special fields to indicate gender, race, height, weight, clothes, subway line and station. Allow for a dragging pin on the map and searches by subway station.

Next Step: HOME-STAT canvassing team working with 311.

Also found in: Data and Systems; Public Perception and Action

Create a dedicated overnight outreach team at the Parsons/Archer stop on the E train.

Next Step: Explore capacity of BRC and NYPD HOU to expand outreach at this longterm hotspot.

Also found in: Cross-Agency Initiatives

MEDIUM -TERM (continued)

Create training elements to fold into existing MTA training and outreach.

Help share success stories, outreach and case management process, and policy and procedures.

Next Step: DHS, BRC, and MTA explore points to disperse information and engage staff.

Also found in: Moving onto Caseload; Cross-Agency Initiatives

SHORT -TERM

Provide additional training for 311 and DHS dispatchers regarding special cases for homeless assistance.

For example, reporting a person in need of assistance on a subway train.

Make outreach teams visible while still being trustworthy to homeless individuals by developing an appropriate balance between the HOME-STAT vests and no branding at all.

Differentiate between Outreach Providers and HOME-STAT canvassers.

Next Step: Create and evaluate design proposals with Providers utilizing HOME-STAT logo.

Fund more staff to check on vulnerable clients during weather emergencies (Code Red/Blue).

Next Step: Review staffing plans for weather emergencies.

SHORT -TERM (continued)

Streamline coordination between outreach shifts to ensure more targeted service delivery to vulnerable clients and clients uncounted or duplicated.

Next Step: Review hand off procedures and standardize across Providers.

Create a unified street sheet for the City with a cohesive brand and up-to-date information.

Next Step: Follow-up with Providers to get their thoughts.

Expand encampment definition to include just one individual.

For example, homeless man's encampment between highway and soccer field in Queens.

Next Step: Review encampment definitions to be shared across all parties.

Also found in: Cross-Agency Initiatives

Provide parking permits with more expansive options so that outreach teams can pull over where needed to interact with individuals.

Next Step: Contact DOT about enhanced permits.

Also found in: Cross-Agency Initiatives

Section 2

Homeless Contact and Response

Staff roles and street presence

- Outreach team attempts first contact
- Outreach team gathers basic info and offers services
- Outreach team repeats contact

Keeping track of individuals

- Person responds to outreach team

Enhancements: page 34

Outreach teams attempt to contact anyone who appears to be homeless. Many people are reluctant to give information initially, requiring the outreach teams to make repeated contacts in order to build relationships that encourage those living on the streets to share information and accept services. The outreach teams expressed that eventually, every person they encounter will seek help and shelter, but it is difficult to know when that time will come.

Staff roles and street presence

The Providers have variations in which staff roles do street outreach and their appearance and presence on the street.

BRC	BRC outreach workers conduct subway outreach, and later transition to case managers once a person has met chronicity thresholds* (see page 53). BRC also has joint clinical and outreach teams. Outreach workers wear bright orange branded jackets and drive branded vehicles. When clinical staff go out with outreach workers, they wear the orange branded jackets, but if they are going out alone, they wear plain clothes.
Breaking Ground	Breaking Ground outreach workers conduct street outreach, and later transition to case managers once client has met chronicity thresholds* (see page 53). Outreach workers wear Breaking Ground branded jackets and drive Breaking Ground wrapped vehicles.
BronxWorks	BronxWorks outreach workers conduct street outreach. They wear a mix of BronxWorks branded jackets and HOME-STAT vests and drive vehicles with a BronxWorks sticker.

* Note: Business process change underway for client thresholds for case management.

Staff roles and street presence (continued)

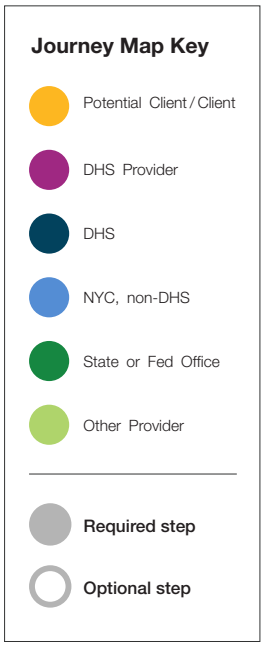
MOC	MOC case managers conduct street outreach during the day. MOC outreach workers conduct street outreach overnight. Both case managers and outreach workers wear plain clothes and drive a wrapped vehicle. DHS / NYC jackets and hats are available but not often worn.
Project Hospitality	Project Hospitality case managers conduct street outreach in addition to case management. They wear HOME-STAT vests and NYC/DHS jackets and drive unmarked vehicles.

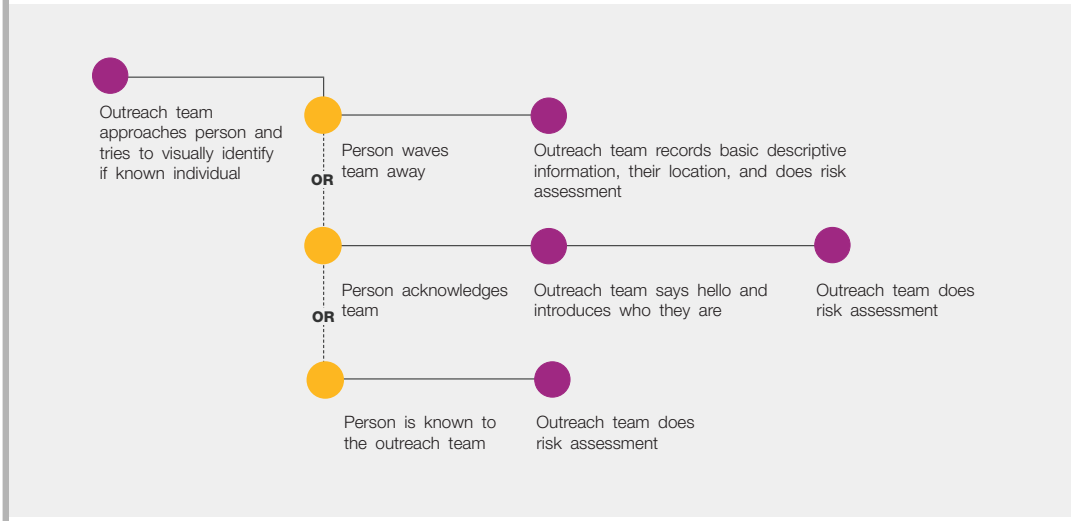
Street teams may be more focused on case management during the day and on outreach at night. In this case, there is a hand-off between the day and night teams to ensure that staff are up to date on important cases.

Outreach team attempts first contact

ONE DAY - YEARS

On first contact, outreach teams are attempting to gather basic information and do a risk assessment, regardless of how the person responds. Providers are tracking everyone they encounter, whether or not they are receptive to the outreach team. The teams log "contacts" in their respective tracking systems and in CARES. Providers assign respectful nicknames to individuals until their actual name is known (e.g., "pink blanket" or "blue hat"). As more is known, workers progressively add more details and information. Providers and DHS report that this makes for many duplicate records as teams may have different nicknames for an individual. CARES does not allow records to be edited as more information is gathered.





Outreach team attempts first contact (continued)

Time
Amount of time varies depending on how much the individual would like to engage. Some Providers reported knowing about street clients for over 10 years who refused to engage or accept services.

Tools and Resources
Providers log notes in the field using different tools:

- piece of paper
- personal phone/visit list
- paper/notebook away from client
- small notebook with client, and also contact sheet(s) for each "Tour" of a station

Several Providers have tried mobile data entry, but say they do not get good data and clients are off put and intimidated by devices.

Outreach team gathers basic info and offers services

1-2 HOURS

Information is usually gathered over time. Often individuals share information in small pieces, like a nickname or a birthday, and outreach workers will build up a case file over time that can eventually be used to help the clients access services.

Standard approach in good weather conditions

Risk Assessment

Ask Status
"Are you homeless? Do you have a place to stay?"

Try to Get Basic Identifiers
Name, DOB, SSN

Offer Placement
Initial contact: Shelter, Drop-in Center
After repeated contacts: Safe Haven, Stabilization bed, Respite bed
Special cases: Detox, ER

Offer Office Visit

Stories from the Street

The Outreach Provider continues to attempt engagement with a homeless man with severe mental illness who has been known to them for 10 years. He has never spoken to them and the only information they have is he used to be a doctor. This sparse information came from a since retired police officer who befriended him many years ago.

Outreach team gathers basic info and offer services (continued)

Standard approach in Code Blue. *Code Blue is DHS' Cold Weather Emergency Procedure when temperatures fall below 32 degrees or the wind chill is below 0 degrees (Fahrenheit). During these conditions, Outreach Teams focus closely on vulnerable clients who they deem are at greater risk of cold weather injury or death. At 20 degrees with wind-chill the teams continue to focus on their vulnerable clients and street teams double their overnight coverage for unsheltered homeless people, who are more at risk during the cold winter months. Additionally, when the procedure is in effect, these individuals may access any of the agency's adult facilities, including shelters and drop-in centers, without going through the usual intake process.*

Additional Offers and Information

- Inform individual they should not stay outside.
- Offer to take them anywhere warm including Dunkin Donuts or the ER waiting room. Some outreach teams were also observed providing Metrocards to individuals, but BRC and the MTA do not condone this practice.

Standard approach in Code Red. *Code Red is DHS' Heat Emergency Plan issued within 24 hours prior to the onset of a heat index reaching at least 95 degrees for two consecutive days or temperature above 100 degrees for any period of time as predicted by the National Weather Service or when a Excessive Heat Warning is issued. During these conditions, Outreach Teams focus efforts to protect vulnerable homeless people, who are more at risk for heat related exposure or death.*

Additional Offers and Information

- Inform individual they should not stay outside or in the subway.
- Offer to take them anywhere air conditioned including Dunkin Donuts, ER or cooling centers.
- Offer water, sunscreen.

As more information is gathered, the engagement becomes more tailored to the client's needs and situation. Offers like an office visit or stabilization bed tend to happen with clients who are already known and are close to getting onto caseload, which requires chronicity (see page 53).

The outreach teams use a daytime approach to let it be the client's decision to come inside, asking them if they'd like to come, rather than directing it. BRC provides some individuals with signed business cards with a unique number on the back. This helps them verify client reports and track individual movements. They also frequently offer dropping into the office or calling the phone number on the card. BRC offers "drop-in" hours during morning, afternoon, and evening times for clients to meet with clinical outreach workers beyond scheduled appointments.

Outreach team gathers basic info and offer services (continued)

Providers employ bilingual staff if language is a barrier. Sometimes outreach teams will use translation apps on their phones to communicate with individuals. They also sometimes call a line with interpreters provided by DHS. Outreach teams expressed additional need for Mandarin and Creole speakers.

Outreach team repeats contact

ONE DAY - YEARS

Most people require repeated contacts before the outreach teams can develop the rapport and trust that allows them to gather information. Building trust can often take time and is a main component of the work of outreach teams. The amount of time varies depending on how much the individual would like to engage. Some Providers reported knowing about street clients for over 10 years who refused to engage or accept services. Often individuals share information in small pieces, like a nickname or a birthday, and outreach workers will build up a case file over time that can eventually be used to help the clients access services.

Many individuals have experience with "the system" and Providers are working to overcome the failure of customer service, whether real or perceived. Individuals view "the system" to include government agencies and service Providers, not always making a distinction between them.

"My advice for someone living on the street is to trust the process. When (the Outreach Provider) picked me up, I was at the bottom. They were honest with me from the beginning and I knew I had to trust the process, be committed and ask questions. In the shelters, everything is decided for you. Now, I tell people about (the Outreach Provider). I still go to places like soup kitchens and tell people that even though it's hard, it's possible and it's going to be alright."

—Permanently housed individual with over 30 years of homelessness experience

Keeping track of individuals

If they are able to gather basic information, Providers reference their tracking systems and/or CARES to find out if a person is known to their organization or the shelter system. It is especially hard to identify the correct case file for individuals who have not given a name. Providers attempt to add evidence to prove chronicity (see page 53), but may not always contribute to the appropriate individual's file if they work different shifts or are less familiar with a specific geography. There are many duplicate entries in the systems that may describe the same individual.

Many Providers will designate an individual "pre-caseload" during the period of gathering more information through repeated contacts and have varying thresholds for this status. BRC also has a "hybrid" designation which is for individuals who are being tracked by their overnight outreach team, but are not on the clinical caseload. Enhancements are underway to include all homeless individuals encountered more than three times as HOME-STAT clients, regardless of chronicity.

Keeping track of individuals (continued)

Both DHS and Providers noted that it is very common for street homeless individuals to move around. Both also shared that during the Veterans initiative, several people went missing and were later found again in a different place. Individuals may also be hospitalized or incarcerated without the Provider's knowledge. Additionally, there is no formal system for transferring case files from one Provider to another.

"You never know when a client will decide to seek help. They do it on their own time."

—Provider

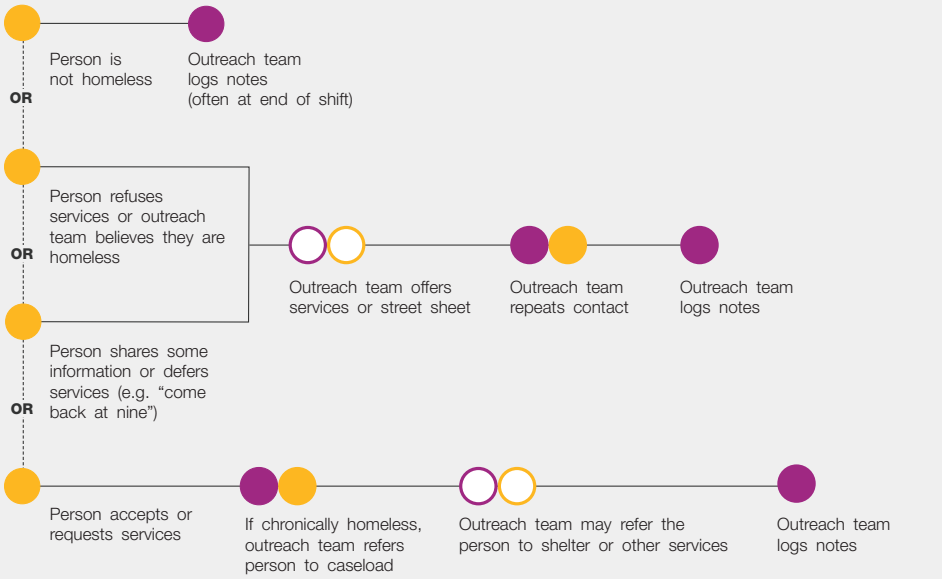
"If you hit 90 days on the street, you have an equal chance of hitting a year."

—Provider

Person responds to outreach team

ONE DAY - YEARS

Depending on how willing a person is to talk to the outreach teams, it can take days to years to gather more information. Many individuals walk away or ignore the outreach teams. At different times and across many different encounters, the same client may follow any or none of these paths. DHS and Providers report that this stage is client-driven and different for every person.



Tools and Resources
Providers offer "street sheets" that list borough-wide resources and services to anyone they encounter.

Journey Map Key

- Potential Client / Client
- DHS Provider
- DHS
- NYC, non-DHS
- State or Fed Office
- Other Provider

- Required step
- Optional step

Person responds to outreach team (continued)

Tools and Resources (continued)
Usually at the end of the shift, outreach teams log notes into digital systems:

- CARES (all Providers)
- Binders and spreadsheets
- CDB
- Excel
- MOCapp
- Contact form
- Tours (note that a single worker does data entry as part of their shift and may or may not be the person who made the observations)

According to Providers and clients, there is not typically just one thing that triggers someone to accept services. Providers often cannot really discern what it was that got someone to come inside. Catalysts can include sudden changes like illness, extreme cold, or having an encampment cleared.

"If you don't have a place for them to go when they're ready, you have to regain that moment."

—Provider

Providers identified a variety of signals that someone may be closer to wanting to move indoors. The person may:

- Talk about their belongings;
- Ask the outreach team questions about Safe Havens;
- Have a friend who is now coming inside;
- Be willing to look at photos of Safe Havens or go on a tour;
- Have made a recent change (like starting methadone treatment) and wants to get fully away from habits;
- Visit a Provider's office and accept a meal there.

Enhancements

Homeless Contact and Response

These enhancements were generated by DHS, Providers, and the Mayor's Office of Operations and include ideas for policy, funding, program design, service delivery, technology and data. They are organized by long-term, medium-term, and short-term time frames.

LONG-TERM

Create a new type of center for Providers to bring individuals for assessment.

Provide beds for 40-50 people and a 5-7 day assessment period for psychiatric screening, medical evaluation, and eligibility determination. Then create a plan for the client that is most appropriate for their circumstances which will put them in a transitional bed that keeps them engaged and moved in the direction towards permanent housing, rather than resisting their placement.

Next Step: Design new program requirements and plan.

Explore developing a mobile tool for outreach teams to use in their vehicles to add notes and reference client information.

Providers have expressed that they do not like using tech directly with clients, as it can be distancing, and their work is focused on building relationships. This will require strong internet access in the field.

Next Step: Work with Providers, DHS business, and DHS IT to develop requirements and prototype solutions.

Also found in: Data and Systems

MEDIUM -TERM

Utilize credible messengers by bringing on formerly chronically homeless to engage resistant clients.

Next Step: Learn more about the BronxWorks peer program with a similar model. Look to some existing criminal justice projects that use credible messengers.

Also found in: Identifying Street Homeless

Formally acknowledge the efforts of individuals who were formerly chronically street homeless and now act as "credible messengers" and are encouraging currently homeless individuals to seek out help and services.

Next Step: Recruit credible messengers to be part of the peer program (see previous enhancement).

Also found in: Identifying Street Homeless

Provide overarching training to outreach workers and case managers.

There should be a shared understanding of what's available and where it is, as well as the goals and expectations of programs and resources. The outreach approaches should remain unique to the Providers and the populations they serve.

Next Step: Review prior resources such as Coalition for the Homeless' "Homeless Outreach Resource" and training currently given by Providers. Develop a new curriculum and schedule trainings.

Utilize NYPD HOU as a transport resource across all Providers.

Coordinate outreach so that officers can transport individuals when needed.

Next Step: Determine how many officers could work with each Provider.

MEDIUM -TERM (continued)

Segment shelter populations by people with similar goals, values and lifestyles.

For example, create shelters that specialize in services for elderly people or people being released from jails or LGBTQ, etc.

Next Step: Propose how to segment current Shelters or identify new sites.

Also found in: Moving onto Caseload

Provide more options and safe places for LGBTQ youth and adults to take shelter and receive comprehensive services.

Next Step: Identify current resources for LGBTQ and determine where they should be expanded.

Also found in: Moving onto Caseload; Special Populations

SHORT -TERM

City issued checklist for: Code Blue/Red, 9.58 removals, and Emergencies.

Next Step: Develop standardized checklists and protocols and design a set of shared resources.

Also found in: Health Crises

Section 3

Health Crises, Hospitals, and Psychiatric Services

- Person is unresponsive or needs medical attention
- Working with Hospitals
- Chronic Inebriates
- Person is a danger to themselves or others and in need of an involuntary removal
- NYC Safe Hub
- Person needs psychiatric services on street

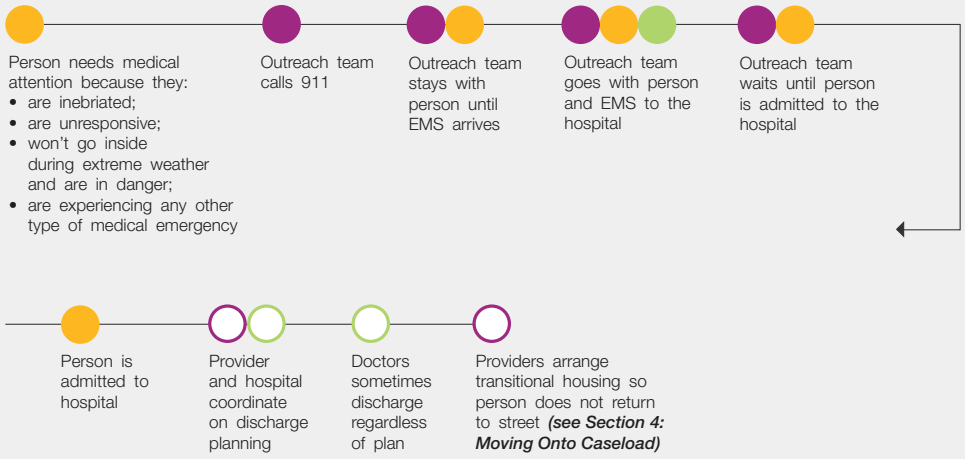
Enhancements: page 47

Street homeless individuals are at great risk for medical and psychiatric emergencies due to exposure to the elements, clinical histories, and undiagnosed or untreated conditions. The Providers are trained to recognize symptoms of both physical and mental health crises and to respond accordingly.

Person is unresponsive or needs medical attention

HOURS TO MONTHS

When an individual is having a medical emergency, Providers immediately call 911. Individuals may remain hospitalized for days or months, requiring ongoing advocacy work and discharge planning.



Tools and Resources

Make 911 call from shift phone or personal phone.

Notes logged in shift / case reports.

Journey Map Key

- Potential Client / Client
- DHS Provider
- DHS
- NYC, non-DHS
- State or Fed Office
- Other Provider

- **Required step**
- **Optional step**

Working with hospitals

The Providers have found that maintaining good, working relationships with hospitals in their boroughs lends itself to more positive outcomes. However, they expressed a need for more coordination.

Positive outcomes from working with hospitals:

- Providers are able to coordinate care and plan discharges with medical staff to coincide with a transitional placement for the individual.
- Medical staff can assist with diagnoses for special cases (e.g., documenting cognitive impairment as a disability).
- Medical facilities can provide referrals to or placement in special programs such as services for chronic inebriates and frequent ER users.
- Providers can work with psychiatric units to achieve specialized placements for clients with unique needs (e.g., geriatric psychiatric unit at Bellevue).

Specific coordinated initiatives:

- Breaking Ground coordinates with the ERs at WoodHull and Mt. Sinai in Astoria to alert Breaking Ground when frequent Medicaid users (also called "super-utilizers") are in the ER. These clients are prioritized for transitional housing.
- BronxWorks has MRT (Medicaid Redesign Team) and BAHN (Bronx Accountable Healthcare Network) Health Home programs through which they have housed frequent Medicaid users. They are also working with St. Barnabas hospital to identify frequent users.
- BRC and MOC use MRT housing for frequent Medicaid users.

Points of contention in working with hospitals:

- When hospitals discharge without planning, clients often go back to the street. On occasion, Providers will plan a discharge with one doctor and then another doctor will disregard the plan and discharge the patient without coordination.
- Hospitals do not always trust reports from Providers regarding an individual's symptoms. This is especially problematic in cases where the individual has exhibited a dramatic decline on the street, but looked improved based on being removed from their environment and cleaned up.
- Providers report that hospitals think homeless individuals do not necessarily need medical care, but are using the hospital as a respite from the street.
- Hospitals sometimes contact Providers about individuals who are not chronically homeless, but are members of a shelter population. When Providers cannot accept these clients, hospital staff become frustrated.

"We don't have good options for inebriates. These are the people who aren't coming inside."

—DHS

Chronic Inebriates

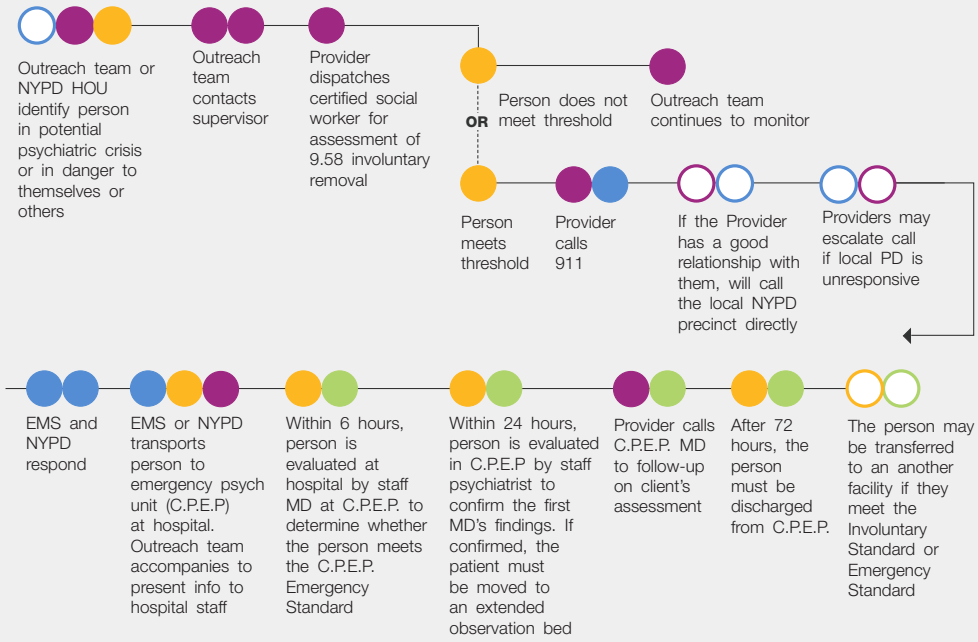
DHS and Providers reported particular difficulty placing chronic inebriates and substance abusers. These clients prefer to stay outside in order to continue using substances and so are very resistant to housing placement. Providers report that there are very few services for this population. There have been good collaborations with the hospitals in Manhattan, Queens, and Brooklyn to better serve this population.

Person is a danger to themselves or others and in need of an involuntary removal

HOURS

The New York State Mental Hygiene Law allows for individuals to be involuntarily removed and taken to a psychiatric emergency facility. Most commonly used are 9.58 removals which can be initiated by outreach teams or the NYPD and approved by a certified social worker. 9.37 removals can be initiated by psychiatrists in order to have an individual assessed at a mental health facility. Providers report that removals by a psychiatrist tend to be smoother because they are coordinated "doctor to doctor." Both types of removals are pre-planned and targeted toward individuals who are at-risk, especially in extreme weather.

9.58 Removal



Journey Map Key

- Potential Client / Client
- DHS Provider
- DHS
- NYC, non-DHS
- State or Fed Office
- Other Provider

- **Required step**
- **Optional step**

Stories from the Street

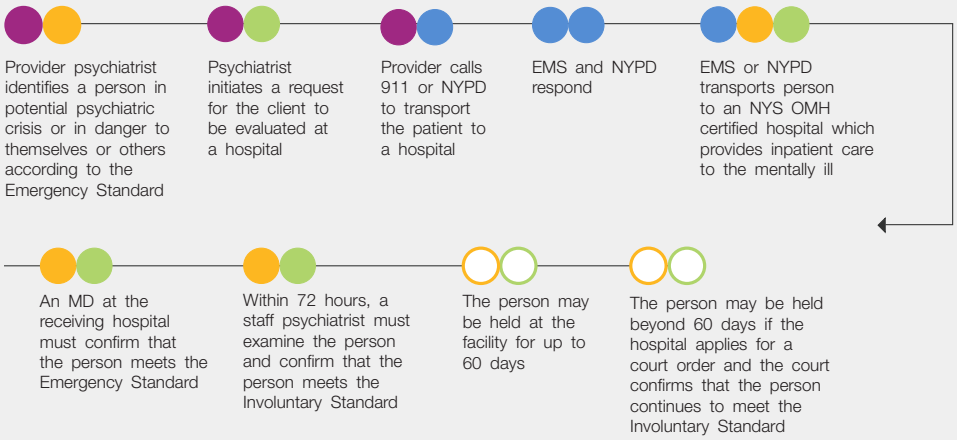
On a frigid morning, a middle-aged man, who appeared much older than his actual age, was found bundled in a drafty subway stairwell and kindly refused services from the Outreach Provider. Although he had severe mental illness, there was not enough to qualify him for a 9.58 removal. The outreach team leader decided to issue a baseline report to their clinical team who then has 24 hours to return to the man to conduct a full assessment for a 9.58 removal.

Person is a danger to themselves or others and in need of an involuntary removal (continued)

Time
When an outreach team believes a 9.58 is needed, it usually happens very quickly. However, these removals are usually pre-planned and coordinated with the appropriate service providers.

Tools and Resources
Make calls from shift phone or personal phone.

9.37 Removal



Time
When an psychiatrist believes a 9.37 is needed, it usually happens very quickly. However, these removals are usually pre-planned and coordinated with the appropriate service providers.

Tools and Resources
Make calls from shift phone or personal phone.

Journey Map Key

- Potential Client / Client
- DHS Provider
- DHS
- NYC, non-DHS
- State or Fed Office
- Other Provider

- Required step
- Optional step

DHS reports that EMS responders will sometimes determine that referred individuals do not meet the threshold for 9.58 removal. In some cases, the person may not be in crisis in the moment, but the Provider has observed a recent deterioration of their physical or mental state. If the person stays on street, they risk further decline and danger, but Providers report that medical personnel making assessments on the street and in the hospital can disregard the social worker's evaluation of the client's situation, even if the social worker has a longer term context. Outreach workers also reported that EMS workers are "rude" and say that they "do not want to deal with the smell."

Additionally, EMS does not always transport the individual to the requested hospital, but rather the closest one. The workers say that many hospitals will not actually treat the individual and will release them back to the street.

Person is a danger to themselves or others and in need of an involuntary removal (continued)

During a 9.58, NYPD helps enforce the authority to complete the involuntary removal. NYPD can also be a source of a referral for potential 9.58 removal. Providers report that establishing good working relationships with NYPD is beneficial to their work, especially in these situations. When Providers have a good working relationship with the local precinct, they also contact the precinct to let them know that a removal is going to happen. DHS reports that the NYPD is sometimes not responsive when needed in a removal and escalation has been required in order to get assistance for enforcement. Providers have also expressed that recurring training and awareness for removals amongst NYPD, EMS, and medical staff would be beneficial. Providers and DHS have conducted trainings in the past at roll calls, precinct meetings, and the Police Academy, but as new officers are brought on, this knowledge is not passed on.

Once at the hospital, the following forms are generated by medical and psychiatric personnel:

- Form OMH 474: Emergency Admission
- Form OMH 476: C.P.E.P. Emergency Admission
- Form OMH 482: Mobile Crisis Outreach Team Authorization for Transport
- Form OMH 471: Application for Involuntary Admission on Medical Certification
- Form OMH 474A: Emergency or C.P.E.P. Emergency Admission

For a 9.37 removal, the following forms are generated by medical and psychiatric personnel:

- Form OMH 475: Application for Involuntary Admission on Certificate of a Director of Community Services or Designee
- Form OMH 475A: Certificate of Examination by Director of Community Services or Designee
- Form OMH 475B: Certificate of Observation by Director of Community Services (Non-physician)
- Form OMH 475C: Examination within 24 Hours
- Form OMH 475D: Examination within 72 Hours
- Form OMH 471: Application for Involuntary Admission on Medical Certification

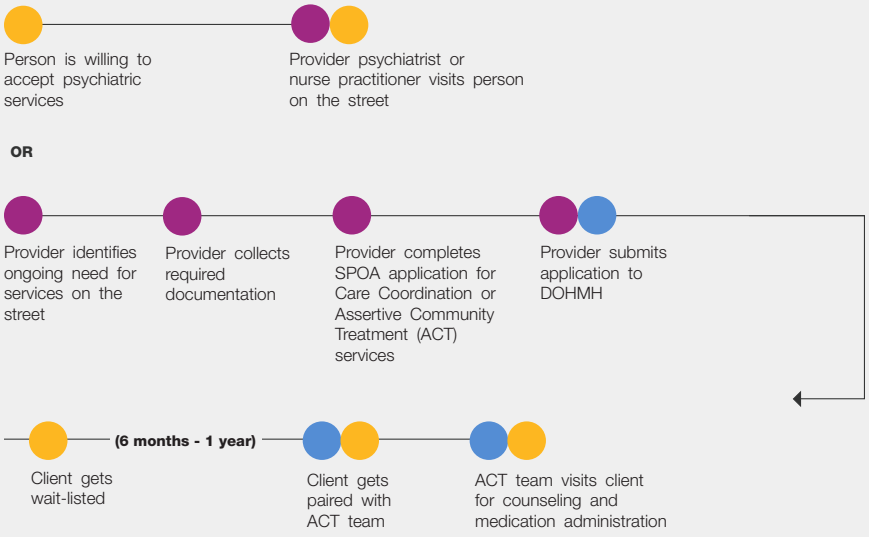
NYC Safe Hub

For clients who have a history of violence, the NYC Safe Hub program is an option. However, DHS and MOCJ report that this is a very small percentage of street homeless clients.

Person needs psychiatric services on street

ONE DAY - YEARS

The Providers are contractually required to have a psychiatrist on staff. Providers reported a wide variance in availability of psychiatric services. Three of the five Providers contract psychiatric services through The Center for Urban Community Services' Janian (CUCS Janian) practice. Several Providers reported needing additional psychiatric services to serve their clients.



Tools and Resources

- Psychiatric notes saved to digital and physical case files
- Documentation and status of SPOA application tracked in digital and physical case files
- Psychiatric evaluation tracked as part of the housing packet

Journey Map Key

- Potential Client / Client
- DHS Provider
- DHS
- NYC, non-DHS
- State or Fed Office
- Other Provider

- Required step
- Optional step

Person needs psychiatric services on street (continued)

Current psychiatric staffing

BRC	Multiple nurse practitioners and psychiatrists
Breaking Ground	Two on-site psychiatrists, each in the office once/week (CUCS Janian), no nurse practitioners
BronxWorks	Psychiatrist in office 2x/week (CUCS Janian), no nurse practitioners
MOC	Psychiatrists in office 3x/week (CUCS Janian), no nurse practitioners
Project Hospitality	On-site psych for 4 hours per week

Psychiatrists work with case managers for on-street evaluations, documents and housing applications (see page 56). Some clients are resistant to meeting with a psychiatrist in the office so some outreach teams take psychiatric staff with them while doing client visits. Providers report that on-street psychiatric services are usually performed in order to complete a psychiatric evaluation as a requirement for applying for subsidized housing (see page 101). Both psychiatrists and nurse practitioners can conduct the psychiatric evaluation. In addition, psychiatrists can do a specialized type of removal to a hospital known as a 9.37 (see page 41), which is coordinated with a doctor at the receiving facility.

Single Point of Access

Some clients have needs that can be served through supplementary programs available to New Yorkers, which Providers will apply for on their behalf. Single Point of Access (SPOA) is a program that connects people with serious mental illness to mental health services that can accommodate them. Assertive Community Treatment (ACT) is one of the services accessible through the SPOA program.

Providers report that medication compliance can be particularly difficult for street homeless. The Assertive Community Treatment (ACT) program allows medical teams to administer medication on the street as well as provide clients with intensive case management. The Providers report that gathering all of the required documentation for the application used for ACT (SPOA) can be difficult and time consuming.

Person needs psychiatric services on street (continued)

Providers report that, while the ACT services are beneficial, not many clients are eligible. The program requires a history of 6 psychiatric hospitalizations over the last 2 years. This information is not always known to the Provider facilitating the application, and even if the client was ill enough to require hospitalization, the client may not have received medical care at the appropriate time. In addition, the individual must:

- Have a diagnosed mental illness;
- Be currently receiving Supplemental Security Income or Social Security Disability Income case due to the diagnosis;
- Have a documented impairment from mental illness;
- Be reliant on psychiatric services.

There is a long wait to receive ACT services. Manhattan is estimated at one year. Brooklyn and Queens is six months. Providers report that if a client is from Brooklyn or Queens, but is placed in transitional housing in Manhattan, that client will wait one year for ACT services.

If an individual does not qualify for ACT, an alternative is Care Coordination which includes intensive case management, but only allows for medication monitoring instead of administration.

The following documentation is required to submit a SPOA application for ACT or Care Coordination services:

- Care Coordination/ACT Program Application cover sheet with signed consent
- Universal Referral Form (URF)
- CC/ACT Referral Summary
- Psychosocial summary, no more than 6 months old
- Psychiatric evaluation, no more than 30 days old for inpatient referrals
- Medical exam results
- Discharge reports
- Authorization of release of URF application to the assigned Care Coordination or ACT program

Enhancements

Health Crises, Hospitals, and Psychiatric Services

These enhancements were generated by DHS, Providers, and the Mayor's Office of Operations and include ideas for policy, funding, program design, service delivery, technology and data. They are organized by long-term, medium-term, and short-term time frames.

LONG-TERM

Develop a series of hubs (mental health, medical, substance abuse) that have a relationship with the Providers and can be used as an alternative.

For example, hospitals usually do not want someone who is going through withdrawal and will also try to discharge them before they go into withdrawal, but they should not go back onto the street.

Next Step: Look into existing City "hub" programs and pull insights from existing wet shelters operating successfully.

Allow Providers to more easily gain guardianship for those hurting themselves medically.

Next Step: Explore existing guardianship laws and specific cases where guardianship may have been beneficial.

Create a track for street homeless individuals with Adult Protective Services (APS).

Next Step: Meet with DHS, APS, and Providers to discuss cases that could use protective services and how to best create a track for them.

MEDIUM -TERM (continued)

Explore re-hiring a DHS Medical Director and hiring high level medical personnel at each Provider.

Ideally there would be one Medical Director for each borough and for after hours.

Next Step: Work with HR to fill the Medical Director role. Explore budget for additional Medical Directors and medical personnel at Providers.

Develop a coordinated discharge process specifically for street homeless with hospitals.

Next Step: Bring hospital stakeholders to the table, perhaps start with HHC.

Add "preventive social services" to hospitals (ERs) including housing, entitlements, etc.

Also found in: Applying for Public Assistance

Potentially at HHC hospitals and also health homes, provide a liaison who can access case notes about an individual.

Emergency rooms often turn people away before they can get full information.

Next Step: Look into CARES and Worker Connect use cases for HHC serving homeless individuals.

Also found in: Data and Systems

MEDIUM -TERM (continued)

Create a standardized psychosocial form for Providers.

Next Step: Develop a standardized design based on all existing psychosocial forms.

Also found in: Moving onto Caseload

Create positions for designated psychiatrists and psychiatric services that serve Providers specifically.

Next Step: Discuss additional psychiatric services from DOHMH and see if the ThriveNYC network of mental health providers can be available for these clients.

Explore creating mobile vans that go to where individuals are on street, Providers' offices, and transitional housing locations.

Services could include: medical, psychiatric services, idNYC, Homebound Public Assistance and other HRA services. Hours and location would vary by needs. The vans could also serve high need and visible areas with non-chronics (e.g. Union Square). The program needs to avoid encouraging people to stay outside while still serving service resistant street homeless.

Next Step: Review current mobile services in the City to see how they could be extended.

Also found in: Applying for Public Assistance

MEDIUM -TERM (continued)

Remove curfew requirement in shelters, or lessen requirement that belongings will be discarded.

People are rushing out of the hospital when they need services in order to meet curfew and not lose their belongings. Others will not go to shelter at all due to curfew.

Next Step: Establish a first meeting at the leadership level to begin developing shared goals and potential policy changes.

Also found in: Moving onto Caseload

SHORT -TERM

City-issued checklist for: Code Blue/Red, 9.58 removals, and Emergencies.

Next Step: Develop standardized checklists and protocols and design a set of shared resources.

Also found in: Homeless Contact and Response

Provide training for doctors about 9.58 removals and the street chronic population to prevent them from treating someone for 24 hours and then releasing to the street.

Next Step: Gather training materials from Providers and the State Department of Health. Work with HHC to develop curriculum.

Improve knowledge of 9.58s with first responders (FDNY, EMS, and NYPD) through training as part of their on-boarding and ongoing development.

Next Step: Review BRC curriculum created for MTA and NYPD and develop a training schedule.

Also found in: Data and Systems

SHORT -TERM (continued)

Appoint someone to own relationship between Providers, NYPD, and EMS. Every case to have an assigned executive leader.

Next Step: Identify a key contact for NYPD and EMS. Providers mention that Louis Molina is a helpful contact to escalate issues when NYPD is unresponsive.

Also found in: Cross-Agency Initiatives

Give Providers access to number of deceased homeless individuals.

Next Step: Confirm existing data sharing channels between DHS and DOHMH.

Also found in: Data and Systems

Publish transparent online information about the current state of the SPOA Assertive Community Treatment and Care Coordination.

Currently, the NYC.gov page links to a Center for Community Services page that states they no longer run the program and links back to the NYC.gov page.

Next Step: Find out who is responsible for managing ACT and Care Coordination services in New York City.

Also found in: Data and Systems

Section 4

Moving onto Caseload

Chronicity Thresholds

- Gathering more information about an individual and assessing eligibility for services
- Case notes are transferred into databases and physical case files
- Case manager conducts psychosocial interview with client
- Case manager identifies transitional or permanent housing for client

Safe Haven Requirements

Resistance to Transitional Housing

Transitional Bed Availability Notification

Enhancements: page 61

Some Providers utilize case managers for initial outreach and others later transition to case managers from outreach workers (see page 27). For Providers who transition to case managers, individuals are assigned a case manager when they accept services and/or meet the chronicity thresholds. Enhancements are underway to include all homeless individuals encountered more than three times as HOME-STAT clients, regardless of chronicity.

The relationship between the client and case manager is a critical component in the journey to securing permanent housing. For clients who accept services, moving onto caseload is often associated with an individual coming in off the street and to placement in transitional housing like a Safe Haven or stabilization bed. However, some caseload clients continue to live on the street during this time.

Stories from the Street

The outreach team woke a man sleeping in a subway car who wanted to accept services. The man seemed to know the MTA employee on duty and they exchanged greetings. The outreach team asked the MTA employee if they could vouch for the man to help prove his homelessness, the employee refused to acknowledge them and the man after that.

Chronicity Thresholds

Once a homeless individual has moved onto caseload per each Provider's thresholds, he or she is referred to as a client of the Provider. Under program practices prior to April 2016, most Providers would need to verify chronicity before they moved a client onto caseload and assigned a case manager. These guidelines are provided here for reference.

Chronicity Thresholds by Provider

BRC	Any homeless individuals on the subway are immediately moved to caseload unless they have only been encountered once and never seen again.
Breaking Ground	Verified either by a third party or the outreach team to have lived on the street for 9 months out of last 2 years or observed bedding down outside.
BronxWorks	Living on the street for 9 months out of last 2 years.

Stories from the Street

The outreach team spotted an elderly woman that they encounter frequently but refuses to engage. She is typically dressed in the same bathrobe and hat and hauls several bags on the subway. As soon as she notices the outreach team, she diverts by exiting onto the platform, facing away from the team, and returning to a subway car once they are at a distance.

Chronicity Thresholds (continued)

MOC	5 sightings bedded down in 3 weeks or multiple engagements with the outreach team, and living on the street for 9 months out of last 2 years.
Project Hospitality	Living on the street for 9 months out of last 2 years.

Providers also report that there are chronically street homeless individuals who refuse to engage with outreach teams but are considered to be on the caseload of the appropriate Provider. In these cases, homelessness is either verified by a third party or Provider observations.

All Providers reported that potential clients may request services but do not meet the criteria for chronic homelessness. The Providers have assess them for emergency medical or psychiatric services and refer them to other services, including City assessment shelters, drop-in centers or the individual's shelter of origin. The Providers may also give these individuals information on low-income housing, food pantries, clothing or private shelters via a "street sheet." In some cases, the Provider have assigned a case manager if the person is vulnerable, a danger to him/herself or others, or high priority client from DHS flagged by Mayoral or community referral.

The case manager will continue to update the client's case file after every meeting or new development. While all Providers enter information into CARES, case notes cannot be shared between them.

Gathering more information about an individual and assess eligibility for services

VARIES

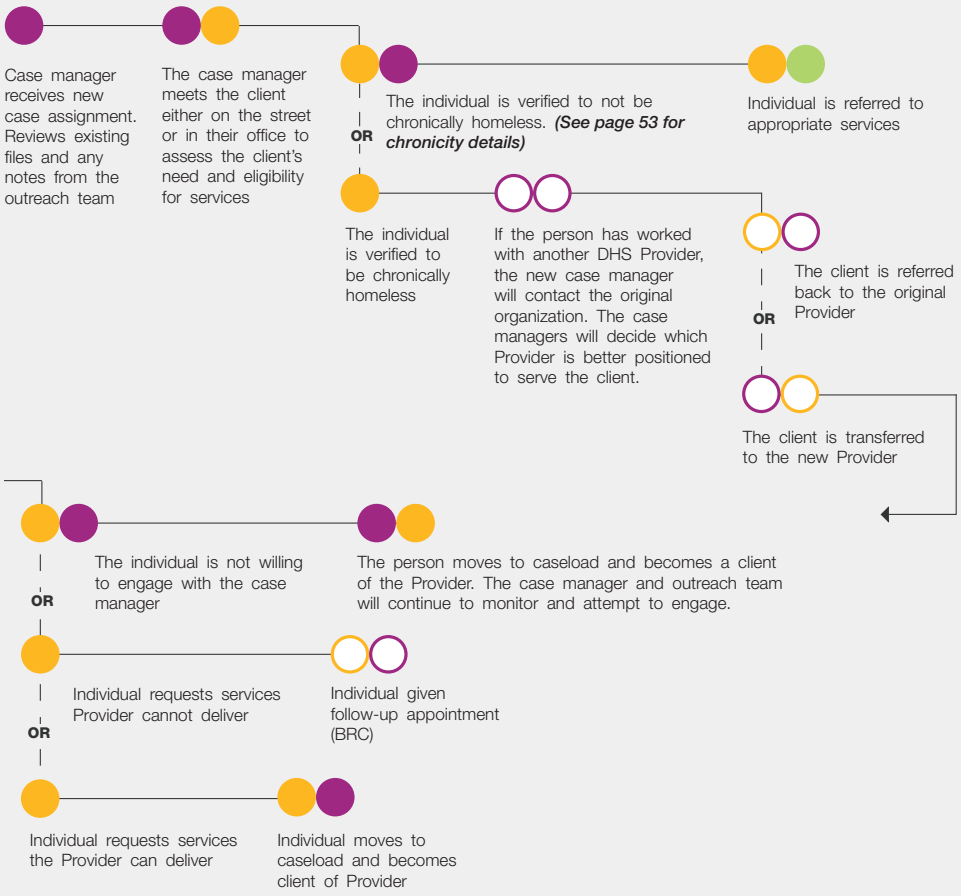
Clients may be hesitant to share any information the first time they meet the case manager, which can happen on the street or for willing clients, in the Provider's office. One case manager reported that the first office meeting might just involve offering the client a cup of coffee and sitting with them until they are done. Other clients will share any information as the case manager requests. The process for moving onto caseload involves information gathering and assessing the client's eligibility for services and transitional housing placement which can happen in any order.

To move onto caseload, Providers obtain more information from forms and surveys:

BRC	Face sheet (completed by Outreach Team), contact form (if the client has not given a name) and baseline evaluation form

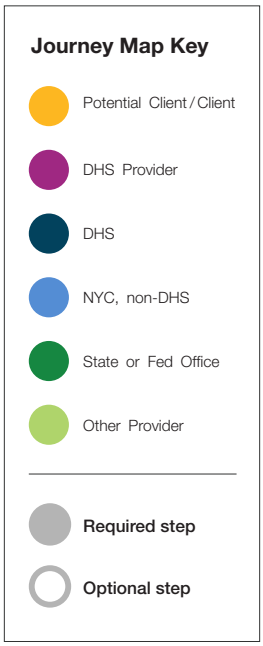
Gathering more information about an individual and assess eligibility for services (continued)

Breaking Ground	Case management consent form and survey
BronxWorks	Basic identification sheet (BID) and mental health and safety assessment
MOC	Client info sheet
Project Hospitality	Screening and intake survey



Tools and Resources

- Many clients have a mobile phone that case managers can call to get in touch
- CARES and Provider databases are referenced for any existing client information



Case notes are transferred into databases and physical files

ONGOING

Case manager adds new information to the client's case file

Select client data entered into CARES as required by DHS

Time
The case manager will continue to update the client's case file after every meeting or new development.

Tools and Resources
Providers store case files in different data systems:

- BG: CDB, CARES, Awards & shared drive
- PH: Awards and CARES
- BRC: Awards and CARES
- BW: CARES
- MOC: MOCapp, CARES & shared drive

Case manager conducts psychosocial interview with client

ONE DAY – WEEKS

The psychosocial interview is a key source of information that the case manager will need to guide the client through the permanent housing application process.

The psychosocial interview includes:

- Identifying information
- Sources of income
- Social, education, work, legal and military history
- Homeless and housing history and current situation
- Psychiatric, substance abuse, and medical history
- Goals to obtain housing
- Summary and recommendations

The psychosocial interview is long and demands the client to share significant personal information and history. This survey acts as another form of trust-building between the case manager and their client.

Providers report that while they all use the same questions for the psychosocial, each Provider uses their own forms to document the evaluation. All questions for the psychosocial are based on the information required by HRA 2010e form. (See page 101)

Journey Map Key

- Potential Client / Client
- DHS Provider
- DHS
- NYC, non-DHS
- State or Fed Office
- Other Provider

- Required step
- Optional step

Case manager conducts psychosocial interview with client (continued)

Case manager gathers information from client that will be required for future housing application

Tools and Resources

- Psychosocial documentation is saved to digital and physical case file
- Psychosocial documentation is tracked as part of housing packet

Case manager identifies transitional or permanent housing for client

ONE DAY – MONTHS

While the end goal for the outreach Providers is to permanently house clients, they also attempt to get clients off the street and in transitional housing until they are matched with a permanent home. Providers noted that not all clients will go into transitional housing before getting permanent placement, but many will require a transitional placement to achieve enough stability to be ready for permanent housing. All teams have a prioritization process, which sometimes includes a vulnerability index, to determine how clients get attached to transitional units.

Options for transitional housing include:

Safe Havens	Single, double, and some dormitory-style rooms with beds, some Safe Havens are run by DHS-contracted outreach Providers.
Stabilization beds	Provider-run rented rooms, usually without on-site services.
Drop-in centers	Provider-run 24-hour centers with chairs and shower facilities.
Respite beds	Outside organization-monitored beds accessed through a drop-in center. Night time only.
Opening Doors	Outside organization-monitored beds accessed directly by the outreach teams.
DHS shelters	City and Provider-run communal beds with shared facilities.

"I'm so tired."

—Client on waiting list for transitional bed

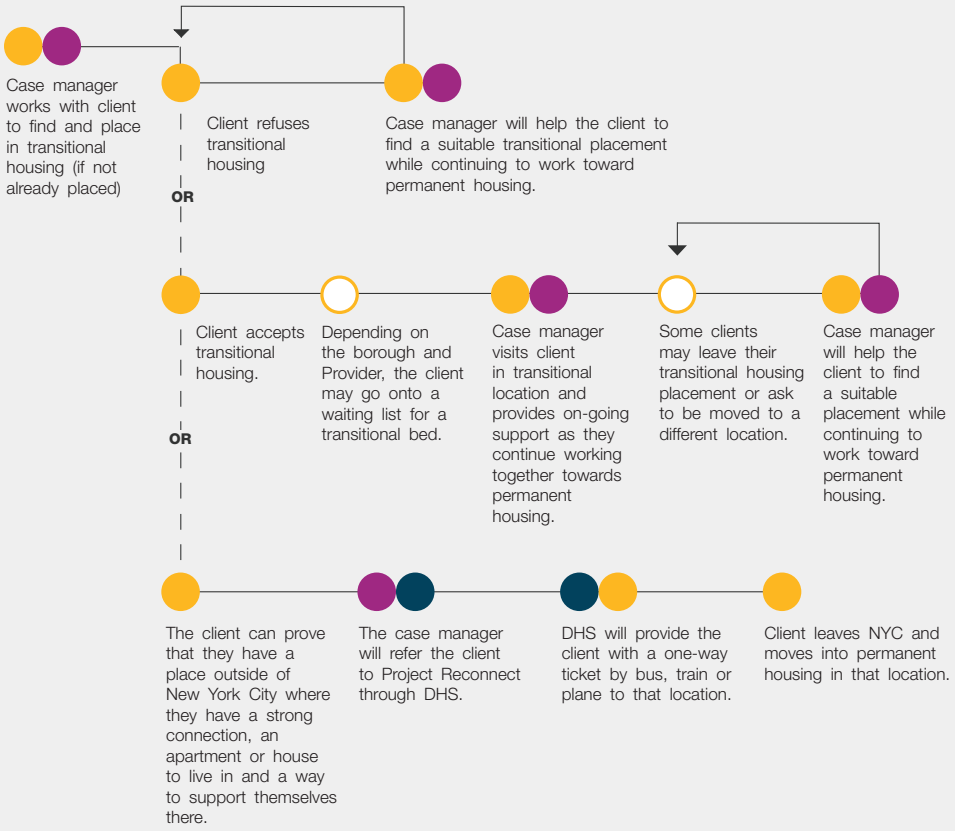
Journey Map Key

- Potential Client / Client
- DHS Provider
- DHS
- NYC, non-DHS
- State or Fed Office
- Other Provider

- Required step
- Optional step

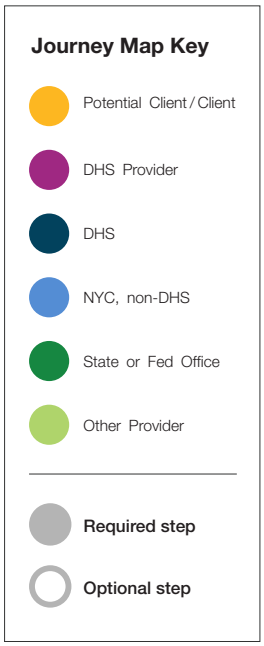
Case manager identifies transitional or permanent housing for client (continued)

There is wide variance in transitional housing availability by borough and client need. Providers in the outer boroughs report moving quickly once a client has indicated that they would like transitional housing, while Manhattan Outreach Consortium (MOC) reports a waiting list for Safe Haven beds.



Tools and Resources

- Long-term stayer letter
- Proof of homelessness



"If you're late by a minute, you'll lose your bed, your belongings will get stolen and you have to go through the process again. Discouraged me from the shelter — if I'm off by a couple of minutes, I'd lose my bed. They'd pop the lock and trash everything. That's why you see many homeless carry all their possessions. Who wants to keep starting over?"

—Permanently housed client

Shelter

For clients who have previously been placed in shelter, Providers can only place someone in other transitional housing if they have been out of the shelter for six or more months. If less than six months, the Provider can only refer the individual back to their assigned shelter. This policy was changed in the past year; it used to be a one year requirement. DHS can change shelter assignment on a case-by-case basis, especially if resistance to shelter is keeping someone on the street. Shelters require individuals to sign an Independent Living Plan, which serves almost like a contract with the shelter for the six months. Many individuals are resistant to shelter due to the six month requirement, and Providers often prefer alternatives to shelter due to limited supportive services.

Safe Haven Requirements

In order to be placed in a Safe Haven, the client must have proof of homelessness. This may be a letter from an institution such as a soup kitchen, MTA, other providers, etc. Providers may also compile evidence from multiple sources, including "unknowns" tracked as contacts by outreach teams that match that person's description. Proving homelessness can be an obstacle to getting a client into transitional housing. There are some exceptions made for high priority and individual clients. Once placed in a Safe Haven, the client receives a Long-term Stayer letter, which serves as a proof of chronic homelessness. This documentation prioritizes the client for certain kinds of HUD-funded permanent housing, including Section 8 Shelter Plus Care, Section 8 Moderate Rehabilitation SRO units and Supportive Housing.

DHS allocates the number of Safe Haven beds available to each Provider, and the Providers determine which clients are placed where. While many of the outreach Providers also manage Safe Havens, the client will not necessarily be placed in a Safe Haven that is connected to an outreach Provider. In addition to the five DHS-contracted outreach Providers, the organizations Volunteers of America, Urban Pathways and Communal Life also run Safe Havens in New York City.

Stories from the Street

A man found sleeping in the early morning hours at a subway station was thankful for the outreach team for waking him. Although he is currently placed in a single room occupancy shelter, he fell asleep at the station because he had missed their curfew. In addition, the man feared being late for his upcoming morning appointment at the nearby HRA Job Center.

Resistance to Transitional Housing

Some clients will continue to sleep outside even after being placed in transitional housing. Reasons for this can include:

- Unhappy with transitional bed location, including distance from required programs like substance abuse treatment;
- Prefer to be near people they know;

Resistance to Transitional Housing (continued)

- Resist separation from spouse or pet;
- More comfortable sleeping outside;
- still transitioning to being indoors.

Some clients will change transitional housing placements in order to be in a more suitable location or care setting. Other clients will leave transitional housing because of rule breaking or a lack of appropriate support at the current placement. In all situations, the case manager's primary goal is to help the client find permanent housing.

Transitional Bed Availability Notification

Safe Havens and Outreach Teams communicate transitional bed availability. Safe Havens call Providers when they have an opening for that team, and teams may proactively check-in with the Safe Havens. DHS may get involved if it appears that a bed has been vacant for more than 72 hours and will inquire who it is assigned to and why it is unoccupied. It can be vacant for a number of reasons including:

- Held for a client who is in a hospital or substance abuse treatment program
- Being treated by an exterminator
- Outreach Team requiring more time to get a vulnerable client into the bed

The Manhattan Outreach Consortium (MOC) shared specific example of a system-driven notification process. MOC teams are notified when transitional beds are available through the C-SPOA (Single Point of Access) notification system. The transitional provider enters a vacancy into the C-SPOA. MOC then uses its vulnerability index to prioritize clients for placement, but can advocate for other clients who may be high need or an ideal fit. Once MOC says they would like to place a client, the program has 72 hours to fill the bed.

Enhancements

Moving onto Caseload

These enhancements were generated by DHS, Providers, and the Mayor's Office of Operations and include ideas for policy, funding, program design, service delivery, technology and data. They are organized by long-term, medium-term, and short-term timeframes.

LONG-TERM

Improve shelter conditions to make them more friendly to the chronically street homeless who have better outcomes in smaller settings.

Specific goals should be:

- reduce burglary
- fewer large spaces
- improve staff service delivery
- reduce overcrowding
- consider more programs with no more than 60 beds

Next Step: Gather leadership for a review of existing shelters that are good settings for chronic population.

Improve services and discharge planning for youth turning 21 in shelter so that they do not end up on the street and become chronically homeless.

Next Step: Review current youth shelter discharge protocol.

Implement wet shelter model to provide services to inebriates and street homeless with substance abuse issues who are otherwise service-resistant.

Next Step: Research other municipalities with wet shelters.

LONG -TERM (continued)

Provide better accessibility in Safe Havens for disabled individuals.

Next Step: Identify stakeholders for reviewing building accessibility and potential modifications.

Create transitional and permanent housing solutions for individuals with pets.

Explore with new Safe haven sites.

Next Step: Look into new pet-friendly "navigation center" in San Francisco.

Also found in: Applying for Housing

Create new roles for case management for the non-chronic street homeless.

Next Step: Review funding and staffing needed for new roles.

Provide more services in shelters, including mental health and counseling, education services, job training, and life skills.

Next Step: Catalog current services, gather stakeholders to address gaps and propose additional programming.

Bring all services to Safe Haven clients: medical, psychiatric, nutrition, and applications.

Next Step: Identify highest priority services for clients.

MEDIUM -TERM

Allow for direct shelter placements by Providers that circumvent the 30th street shelter or other assessment shelters through a smoother process that can be more common.

Many individuals want to stay in their "home" borough or may get lost in the system.

Next Step: Establish a first meeting at the leadership level to begin developing shared goals and potential policy changes.

Remove curfew requirement in shelters, or lessen requirement that belongings will be discarded.

People are rushing out of the hospital when they need services in order to meet curfew and not lose their belongings. Others will not go to shelter at all due to curfew.

Next Step: Establish a first meeting at the leadership level to begin developing shared goals and potential policy changes.

Also found in: Health Crises

Create better transportation options for clients to transitional housing or service providers.

Allow clients to take city-funded transportation, like Access-a-Ride or create positions for designated drivers just for these clients. This would maximize the time of Providers to work with more clients and do other work.

Next Step: Review transportation options and City services available.

Also found in: Identifying Street Homeless

MEDIUM -TERM (continued)

Refresh the vulnerability index across all teams to inform prioritization for housing.

It should be consistent across teams and take into account the non-chronically homeless.

Next Step: Collect all vulnerability indices and prioritization processes from teams; determine which groups to target for housing.

Also found in: Data and Systems

Change the way that transitional beds are allocated to each Provider, so that clients don't have go onto waiting lists while other Providers have openings.

Next Step: Review transitional housing allocations and protocol to improve efficiency to give a bed to anyone who wants to come inside.

Create more Safe Havens.

There are particularly high needs to provide transitional beds for women and to provide facilities in Queens.

Next Step: Explore creating new Safe Havens and targeting them to particular areas and needs.

Create Safe Havens that are segmented by age and provide living spaces for different genders.

Many clients want to be with other people their age and will be more likely to come in if they know they are sharing a room with peers.

Next Step: Propose how to segment current Safe Havens or build into the plan for new sites.

MEDIUM -TERM (continued)

Segment shelter populations by people with similar goals, values and lifestyles.

For example, create shelters that specialize in services for elderly people or people being released from jails or LGBTQ, etc.

Next Step: Propose how to segment current Shelters or identify new sites.

Also found in: Homeless Contact and Response

Provide more options and safe places for LGBTQ youth and adults to take shelter and receive comprehensive services.

Next Step: Identify current resources for LGBTQ and determine where they should be expanded.

Also found in: Homeless Contact and Response; Special Populations

Create a portal showing housing available in the City by category, including drop-in and respite bed tracking.

It would provide all details and regulations about a housing opportunity and include photos. Providers would use the portal to showcase opportunities to clients and have them be fully informed before attending an interview. The housing options should promote themselves to clients in hopes of clients selecting them to be their transitional or permanent home.

Next Step: Convene housing providers. Prototype layouts for housing information.

Also found in: Finding Housing Opportunities; Data and Systems

MEDIUM -TERM (continued)

Create a portal showing programs available in the City (like detox) to help Providers make better recommendations to their clients.

Next Step: Locate all available programs in the City.

Also found in: Data and Systems

Create training elements to fold into existing MTA training and outreach.

Help share success stories, outreach and case management process, and policy and procedures.

Next Step: DHS, BRC, and MTA explore points to disperse information and engage staff.

Also found in: Identifying Street Homeless; Cross-Agency Initiatives

SHORT -TERM

Host an "Outreach Team Roadshow" to educate internal DHS groups (especially shelters), Safe Havens and other key stakeholders about the Street Homeless team and provider's services.

Next Step: Develop communication materials.

Also found in: Cross-Agency Initiatives

Section 5

Gathering Vital Documents

- Getting a Social Security Card
- Getting a replacement benefit card
- Getting a birth certificate
- Getting a State ID
- Getting an idNYC
- Getting proof of military service
- Applying for or replacing a Permanent Resident Card
- Applying for or replacing a Naturalization Certificate
- Securing an official name change

Enhancements: page 77

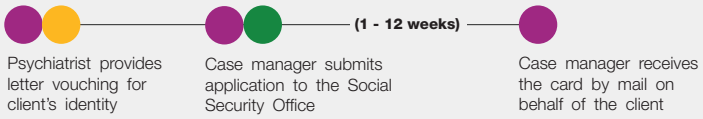
To be considered for permanent housing, a client will need proof of birth, citizenship and photo identification. Many clients are missing some or all of their vital documents, which makes it difficult to gather the others. Each client has a unique set of document needs and the case managers work to identify which documents will be most viable to attain.

Depending on the client's situation, the case manager may receive mailed documents at their office on behalf of the client. In other cases, the client may use the address of a friend, family member or another service Provider. Some clients will use a general post office (GPO) address to receive mail, although Providers do not recommend it.

Getting a Social Security card

1 – 12 WEEKS FROM SUBMISSION

Most Providers mentioned that the Social Security office will work with them to get a client their social security card. With this card, the case manager can begin to apply for other documents like a birth certificate. Sometimes clients forget their social security number or their mother's maiden name. Without these details, applying for a social security card becomes difficult.



Tools and Resources

- Application forms
- Letter from psychiatrist
- Copy of social security card saved to digital and physical case file
- Application and receipt of social security card tracked as part of housing packet

Journey Map Key

- Potential Client / Client
- DHS Provider
- DHS
- NYC, non-DHS
- State or Fed Office
- Other Provider

- Required step
- Optional step

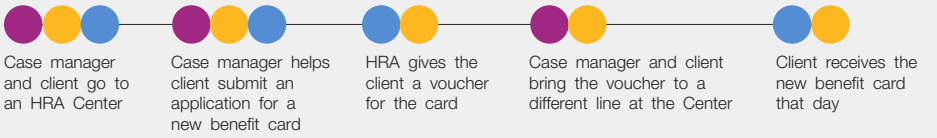
Getting a replacement benefit card

1 DAY

If a client is already receiving HRA benefits, the Provider may try to get them a replacement benefit card. This card could serve as an ID for the client (though not a photo ID) and be used to apply for the client's additional documents. Previous versions of the benefit card included a photo and could be used as photo ID.

Replacement benefit cards can be issued in person at either of HRA's two CBIC (common benefit identification card) locations, also known as their over-the-counter sites. These sites are housed within the HRA job centers, one in Manhattan and the other in Brooklyn. Case managers can also call HRA's information line to receive a new card by mail to the address on file, but report often getting disconnected because the client is undomiciled.

Providers report that getting the clients to go into the HRA Center can be very challenging. Going through security and waiting in multiple lines is difficult for clients who may be mentally or physically frail. In addition, Providers report that clients feel "bounced from office to office" and may not be willing to travel to the HRA Center.



Tools and Resources

- Application forms
- HRA voucher
- Copy of benefit card saved to digital and physical case file

Journey Map Key

- Potential Client / Client
- DHS Provider
- DHS
- NYC, non-DHS
- State or Fed Office
- Other Provider

- **Required step**
- **Optional step**

Getting a birth certificate

VARIES BY STATE

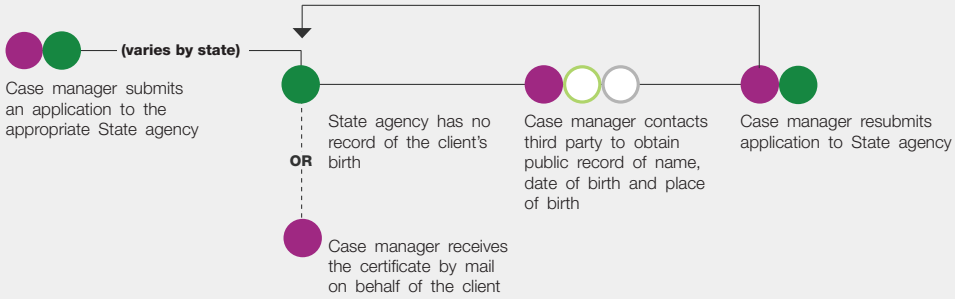
The case manager can apply for a birth certificate using the social security card. Clients may not remember the name of the place where they were born or even their birth date, which makes it difficult for Providers to know where to start. All Providers reported that if the client is not from New York state, getting a birth certificate can cause significant delays in the process.

For older clients, birth certificates may be inaccurate or in unusual formats (e.g. handwritten) which can make it difficult for Providers to use the certificate to gather other documentation.

Getting a birth certificate (continued)

For clients who were born out of the country or had home births, there may be no record at all. In cases where the state agency has no record of the client's birth, the case managers will try to obtain early school, census or hospital records in order to establish proof of name and date and place of birth for the client.

Providers mentioned that client's vital documents can sometimes be found in the CARES case file, but the system is not a reliable source of client documentation.

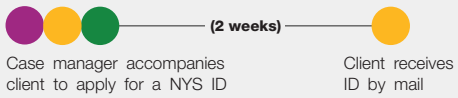


Tools and Resources
- Application forms
- Copy of birth certificate saved to digital and physical case file
- Application and receipt of birth certificate tracked as part of housing packet

Getting a State ID

2 WEEKS FROM APPLICATION

The case manager and client can apply for a State ID using the Social Security card and birth certificate. Getting a photo ID is a crucial step in meeting the requirements to apply for housing. All Providers mentioned that waiting in line at the Department of Motor Vehicles can be stressful for clients.



Tools and Resources
- Application forms
- Copy of New York State ID saved to digital and physical case file
- Application and receipt of photo ID tracked as part of housing packet

Journey Map Key

- Potential Client / Client
- DHS Provider
- DHS
- NYC, non-DHS
- State or Fed Office
- Other Provider

- **Required step**
- **Optional step**

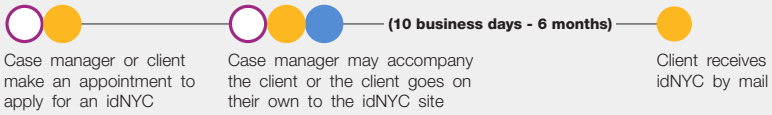
Getting an idNYC

10 BUSINESS DAYS – 6 MONTHS FROM SUBMISSION

> **"NYC ID is tough, because you need a photo ID to get one."**
> — Provider

Many clients do not have the photo ID required for idNYC and no Provider mentioned the idNYC as a first choice for getting a photo ID for a client. However, one case manager offered to accompany a client to apply for an idNYC so that he could have a photo ID to keep on him, while she kept his green card in his file. She also shared that she had another client apply for an idNYC on her own.

Providers report that the advance appointment required for idNYC can pose a serious hurdle for the client.



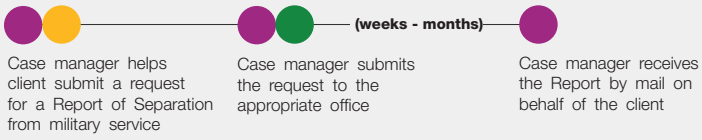
Time
One Provider reported a 6 month wait for the card. NYC.gov quotes 10 to 14 business days from submission.

Tools and Resources
- Application forms
- Copy of idNYC saved to digital and physical case file
- Application and receipt of photo ID tracked as part of housing packet

Getting proof of military service

WEEKS TO MONTHS FROM APPLICATION

A Report of Separation is used to verify military service for benefits and can allow eligible clients access to specialized services for Veterans. DHS reports that, if a client has their social security number, the Report of Separation can be easily obtained. However, Providers have faced unique challenges obtaining proof of military service, including one case where a Veteran was de-identified as a part of his service, making his vital documents nearly impossible to obtain.



Tools and Resources
Copy of Report of Separation save to digital and physical case file

Journey Map Key

- Potential Client / Client
- DHS Provider
- DHS
- NYC, non-DHS
- State or Fed Office
- Other Provider

- Required step
- Optional step

Getting proof of military service (continued)

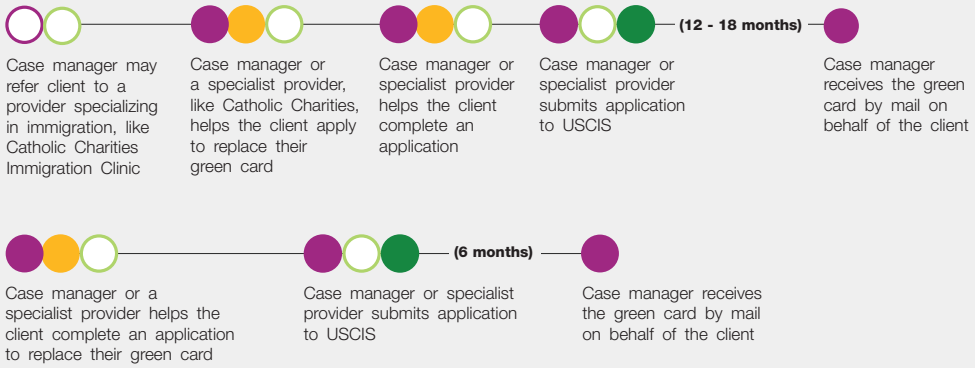
The following forms are required to request a military service record:

- Standard Form 180: Request Pertaining to Military Records
- DD Form 214: Report of Separation

Applying for or replacing a Permanent Resident Card

12 – 18 MONTHS FROM SUBMISSION FOR ELIGIBLE CLIENTS

Providers help some clients who are not United States citizens obtain authorization to live and work in the United States on a permanent basis. Many Providers rely on other Providers, such as Catholic Charities' legal services, in order to help clients with immigration-related needs. In some cases, it is possible to get a fee waiver for green cards.



Tools and Resources

- Permanent Resident Card held by case manager on behalf of client
- Copy of card saved to digital and physical case file
- Application and receipt of card tracked as part of housing packet

The following forms are required to apply for a green card:

- Form I-485: Application to Register
- Permanent Residence or Adjust Status
- $0 - $1,070 fee

Journey Map Key

- Potential Client / Client
- DHS Provider
- DHS
- NYC, non-DHS
- State or Fed Office
- Other Provider

- **Required step**
- **Optional step**

Applying for or replacing a Permanent Resident Card (continued)

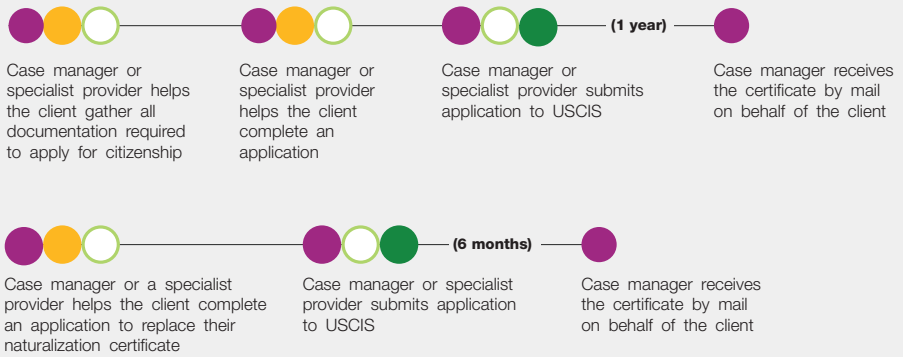
- An original official copy of the each of the client's arrest, sentencing, probation or parole records and/or any court orders removing such items from the client's record, if applicable
- Foreign birth certificate
- Copy of passport page with non-immigrant visa
- Form I-693: Report of Medical Examination and Vaccination Record, if applicable
- Form G-325A: Biographic Information Sheet
- Evidence of eligibility
- Evidence of public assistance received in the US

The following forms are required to replace a green card:

- Form I-90: Application to Replace
- Permanent Resident Card
- $365 filing fee
- $85 biometric services fee, if applicable

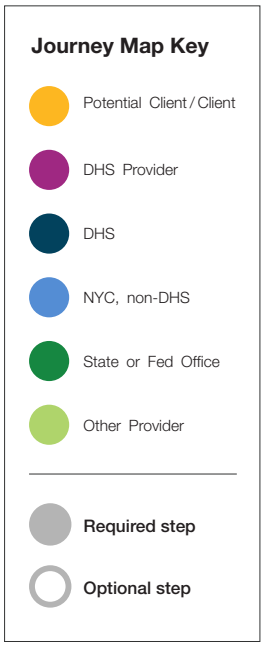
Applying for or replacing a Naturalization Certificate

AT LEAST 1 YEAR FROM SUBMISSION FOR ELIGIBLE CLIENTS



Tools and Resources

- Copy of Naturalization Certificate saved to digital and physical case file
- Application and receipt of certificate tracked as part of housing packet



Applying for or replacing a Naturalization Certificate (continued)

The following forms are required to apply for a naturalization certificate:

- Permanent Resident Card
- Form N-400: Application for Naturalization
- Non-refundable $680 fee
- Evidence of financial support of dependents, if applicable
- Proof of military service, if applicable
- An original official copy of the each of the client's arrest, sentencing, probation or parole records and/or any court orders removing such items from the client's record, if applicable
- Form N-648: Medical Certification for Disability Exceptions, no more than 6 months old, if applicable
- Any correspondence with the IRS regarding failure to file taxes, if applicable
- "Status Information Letter" from the Selective Service, if applicable
- Form I-912: Request for Fee Waiver

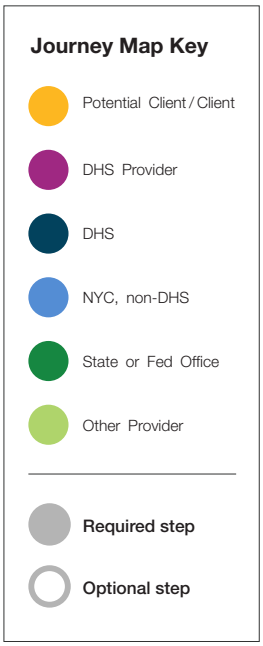
The following forms are required to replace a naturalization certificate:

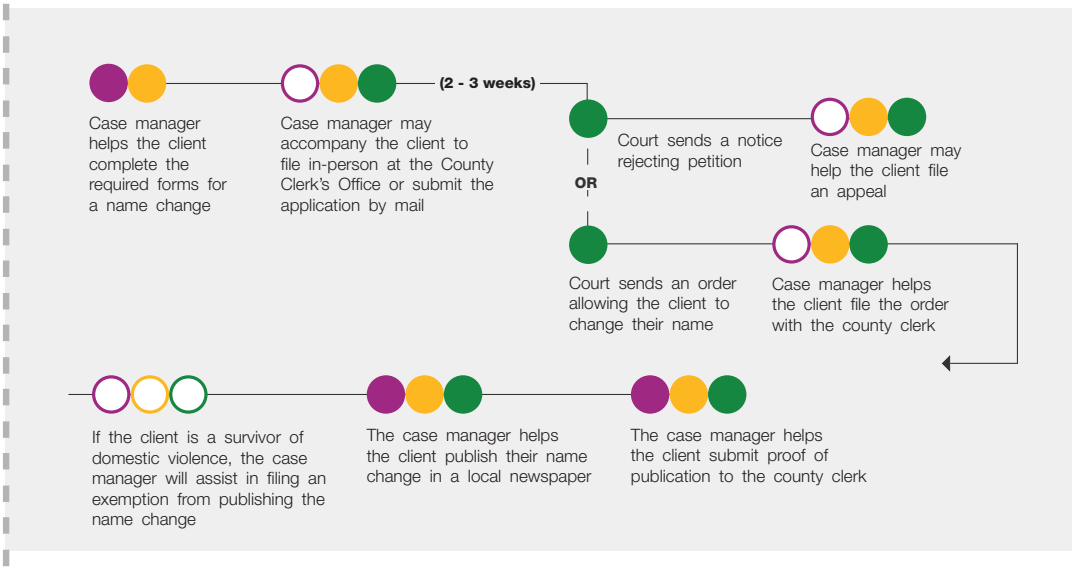
- Form N-565: Application for Replacement Naturalization Citizenship Document
- $345 filing fee

Securing an official name change

UP TO 6 MONTHS

Sometimes clients will want to change their names or have been known by a name that is not the name on their official documentation. The case managers can help them through the process and, often times, cannot get the other required documents listed above until the name change is official.





Securing an official name change (continued)

Tools and Resources
Copy of court order or rejection notice saved to digital and physical case file

The following forms are required to secure an official name change:

- Name change petition
- Name change order
- Request for Judicial Intervention
- Index Number Application
- $305 filing fee, or Affidavit in Support of Application to Proceed as a Poor Person
- Affidavit of Publication, if applicable

Enhancements

Gathering Vital Documents

These enhancements were generated by DHS, Providers, and the Mayor's Office of Operations and include ideas for policy, funding, program design, service delivery, technology and data. They are organized by long-term, medium-term, and short-term timeframes.

LONG-TERM

Implement more fee waivers for citizenship applications and documents.

Next Step: Find out which fees pose the biggest barriers.

Also found in: Special Populations

Allow required and validated documents from a City agency, such as HRA, stand in place as acceptable documents for another agency, such as HPD or DHS.

Next Step: Convene a working group to develop a plan of action.

Also found in: Applying for Housing

MEDIUM -TERM

Enhance immigration services for all Providers.

Next Step: Estimate how many undocumented clients are working with each outreach team. Meet with MOIA to explore options available through the City.

Consider appointing an immigration liaison to assist with specialized barriers for undocumented clients, including assisting with obtaining documentation and funding for fees.

Also found in: Special Populations

MEDIUM -TERM (continued)

Make IDNYC more user-friendly for the street homeless.

Ideas include a mobile application that goes out to homeless, opportunity to get ID without making an appointment, and avenues for clients to get IDs before they may get other photo IDs.

Next Step: Meet with IDNYC team to discuss population needs and programmatic solutions.

Also found in: Applying for Public Assistance, Cross-Agency Initiatives

Revise the benefit card replacement process so that Provider staff and clients do not get disconnected from HRA's information line nor need to go into an HRA Center to receive one.

Next Step: Determine why the Providers and clients can not use the 311 card replacement service request.

Also found in: Applying for Public Assistance; Cross-Agency Initiatives

Section 6

Applying for Public Assistance

Applying for Public Assistance

HRA Job Centers

WeCARE

Homebound Program

Sanctions

Recertification

Medicaid

Accessing vital documents in CARES

Enhancements: page 90

To be eligible for permanent housing, a client must have an income, usually in the form of Public Assistance or Social Security. Case managers work to secure these supports for a client, but enrolling homeless individuals in benefits programs can be a difficult process due to traveling to appointments and meeting all requirements.

Applying for Public Assistance

45 DAYS FROM INITIAL APPLICATION

Public Assistance, SNAP, and Medicaid are available from the Human Resources Administration (HRA). Public Assistance is needed for ongoing expenses and often to fund their eventual housing placement. HRA also refers to Public Assistance as Cash Assistance. If needed, emergency SNAP benefits are available for clients who are screened and deemed eligible. HRA will provide the client with a referral letter to visit one of the City's two CBIC (common benefit identification card) locations to obtain a benefits card and receive immediate SNAP assistance.

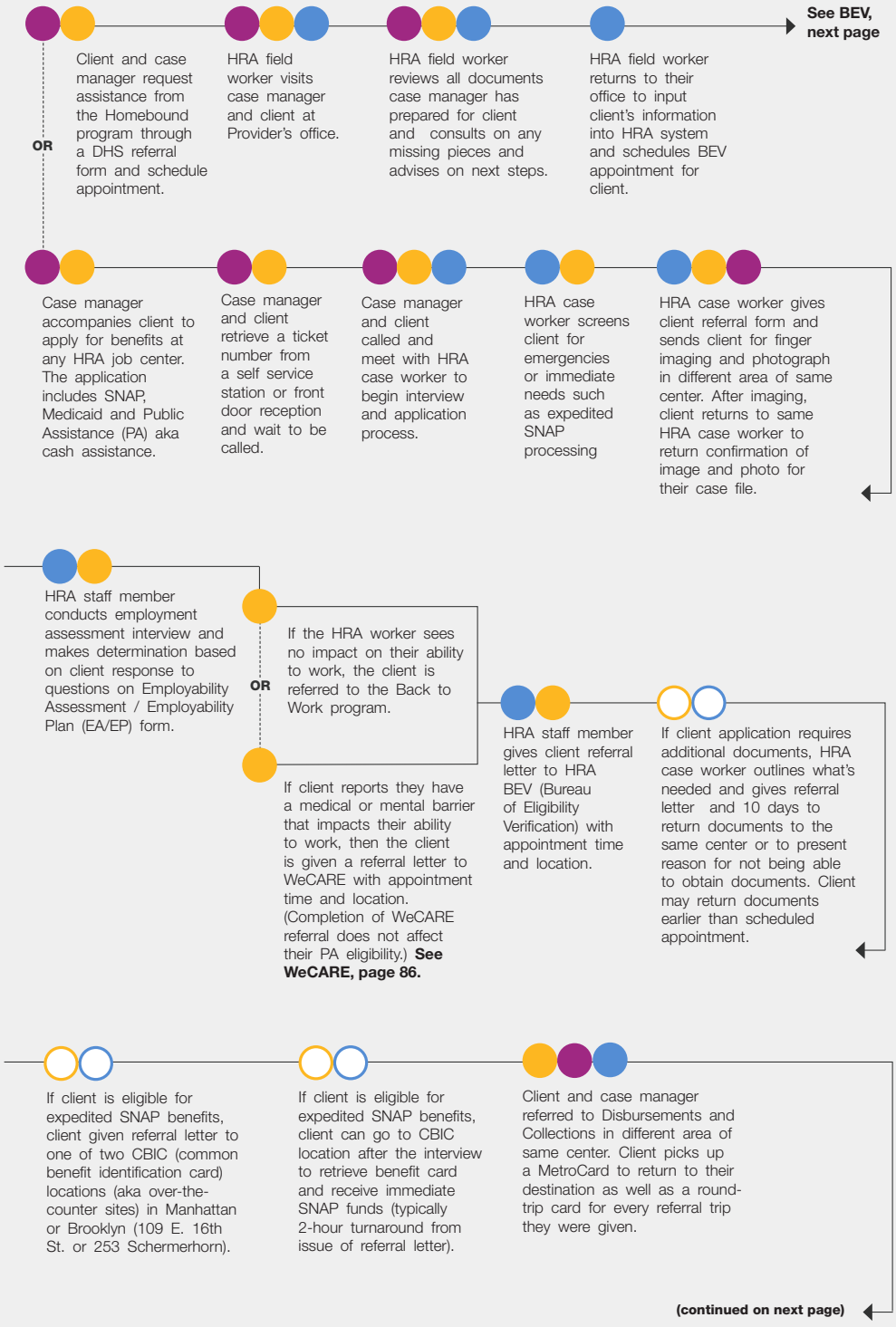
In an ideal case, the Public Assistance application process involves at least four appointments, though Providers cite a baseline of six due to needing to return with additional documents or information. In most cases, the case manager will accompany the client to all appointments during the benefits application process. Providers report that at every step, there is a risk that clients may be reluctant or unable to attend the appointment, which could require starting the process over again.

HRA will process the application and make a decision 25-30 days after initial submission. If documents are missing or referrals are incomplete, the client's application can be denied. Clients and case managers can apply for a fair hearing if they disagree with the cause for rejection. If the application is complete, benefits will be activated, on or after 45 days as mandated by the state of New York. Prior to the 45th day and if the application has been accepted, HRA will mail the client two separate letters, one with their benefit card and the other with the card's pin number so they may begin accessing benefits as soon as they are activated.

"In a perfect world, Public Assistance takes 45 days."

— Provider

Applying for Public Assistance (continued)



Journey Map Key

- Potential Client / Client
- DHS Provider
- DHS
- NYC, non-DHS
- State or Fed Office
- Other Provider

- **Required step**
- **Optional step**

Applying for Public Assistance (continued)

BEV (continued)

2 weeks

Client attends scheduled interview at HRA BEV (Bureau of Eligibility Verification).

BEV requests additional documents and updates the client's status in Mapper.

Case manager and client return for follow up interview at BEV.

OR

BEV makes recommendation to deny for various reasons.

OR

BEV advises HRA to accept the application.

BEV updates client's case file in Mapper system.

(25-30 days)

Client completes referrals. Each referral is updated in HRA's eligibility systems (NYCWAY, Mapper, etc.).

HRA processes application. This typically occurs 25-30 days after their initial application appointment.

Client does not complete referrals. HRA eligibility systems shows infraction and application is denied.

HRA mails client a notice.

Client and case manager have option to apply for a fair hearing if they disagree with cause for rejection.

OR

Client completes all referrals, HRA mails benefit card and pin number in 2 separate letters to client.

On the 45th day or prior, HRA activates the clients case. Client's card can be used on or after 45th day to access benefits.

If the client does not receive mailings after 45 days, they can check on the status of their card at any CBIC (common benefit identification card) location or pick up their card if it is there waiting for them. If the client does not have ID, HRA will verify identity through finger imaging and photograph.

On occasions where the client needs proof of PA for an imminent housing opportunity, DHS will try to work with HRA internally to obtain the budget letter.

Case manager or client can also request a budget letter over the phone or in person at the HRA center after a case has been activated.

For Public Assistance, HRA requires new applicants to recertifiy 4 months after their initial application, followed by a 6 month mailer. A full recertification is required 6 months after that and then at least once per year. For SNAP, recertification is required once per year.

Journey Map Key

- Potential Client / Client
- DHS Provider
- DHS
- NYC, non-DHS
- State or Fed Office
- Other Provider

- **Required step**
- **Optional step**

Applying for Public Assistance (continued)

Providers report that for a client living in a Safe Haven, Public Assistance is $45 per month compared to $264 per month for clients living on the street. This disparity, coupled with program conditions, sometimes makes it difficult to convince clients to go through the effort of applying and maintaining a public assistance case and also stay in transitional housing.

The following documents are required to apply for Public Assistance:

- Application forms
- Proof of identity
- Proof of age
- Proof of residence, if applicable
- Proof of citizenship or alien status (original)
- Proof of income, if any
- Documentation of medical expenses, if any
- Proof of health insurance, if any
- Social Security card (not required for cash assistance)
- *For Homebound program only:* residency letter, support letter and verification of homelessness

Although proof of identity is required, HRA reports that their threshold for proof is low. If a client does not possess an ID card, HRA reports that a client can confirm identity through their social security number, if they know it. The client can also provide collateral contacts or notarized letters. For clients born in New York City, HRA can run a match with the Department of Health and Mental Hygiene's vital records to obtain a copy of their birth certificate.

If at any point, a client can not provide follow-up documents on time or at all, HRA can provide extensions if notified or assist the client in obtaining the required documents. According to HRA's Duty To Assist policy, all HRA staff are required to assist all clients in accessing public assistance at any point in their application or recertification, whenever possible. This may be in the form of:

- Finding birth certificates (for New York City and other locations)
- Obtaining information from collateral contacts
- Referring them to the Social Security Administration for proof of identity
- Data matching for employment verification

HRA Job Centers

Note: Independent process changes underway regarding HRA Center 37 requirements

"Any appointment is difficult if you don't have a home."

— Provider

In New York City, any individual can apply for public assistance at any of the City's HRA job centers. All clients are guaranteed to be seen that day, regardless of arrival time. If a client cannot stay and wait to be seen, HRA will preserve their filing date for 7 days. For homeless individuals, services have recently been centralized at Center 37 in Long Island City with the intention of providing specialized homeless-related expertise. DHS and the Providers expressed frustration and complications with visiting Center 37 including:

- Clients and case managers routinely wait 5-7 hours per appointment at the center. This is particularly challenging for clients who are employed and take time off to go to the HRA Center. In addition, this wait time precludes case managers from working on any other cases that day. Breaks are also not allowed and food is prohibited.
- The single center is far from where many clients and Providers are, especially the Bronx and Staten Island. Clients are often reluctant to leave the borough they are in. Providers report that convincing clients to come in only to experience long waits and unresponsive staff can damage the relationship between the case manager and client and discourage the clients from moving forward with the permanent housing process.
- Clients and case managers bring all documentation with them to every appointment in instances where they need to provide proof that they made a previous visit or completed required documentation if HRA's data systems are not updated by their staff. Returning multiple times was cited as a major obstacle in completing this process.
- The rules and processes are opaque and inconsistent. While their impression was that the Center's staff were intended to be experts in assisting the homeless population, Providers described the experience as daunting and insensitive.
- Providers and DHS report that there is no protocol to address issues with HRA.

Many Providers continue to visit other job centers with their clients. Providers report that they rarely have positive service experiences at HRA job centers and routinely engage with staff who are agitated, discouraging, refusing to provide services, and who may not fully understand all of HRA's policies and rules. During a morning visit to the DeKalb Job Center in Brooklyn, clients appeared to have been waiting for extended periods of time, some expressing frustrations with the slow service and the unresponsiveness of the staff. Further extending their stays, the center requires clients to visit two different floors for intake and processing to apply for cash assistance, each with their own waiting periods. The number of clients in the waiting areas on each floor outweighed the amount of staff that were available to serve them.

HRA Job Centers (continued)

HRA recognizes that their centers are understaffed and that some workers may unknowingly provide clients with incorrect or outdated information, delaying the public assistance process further. Of the limited number of staff that were present at the DeKalb Job Center visit, many appeared to be working as diligently as their environment would allow. The center was outdated,uncomfortably warm and without sufficient air circulation. HRA staff were also working in the presence of several security guards that serve to respond to client assaults. Staff also reported uncomfortable interactions that are risks to their own health due to the health conditions of clients.

WeCARE

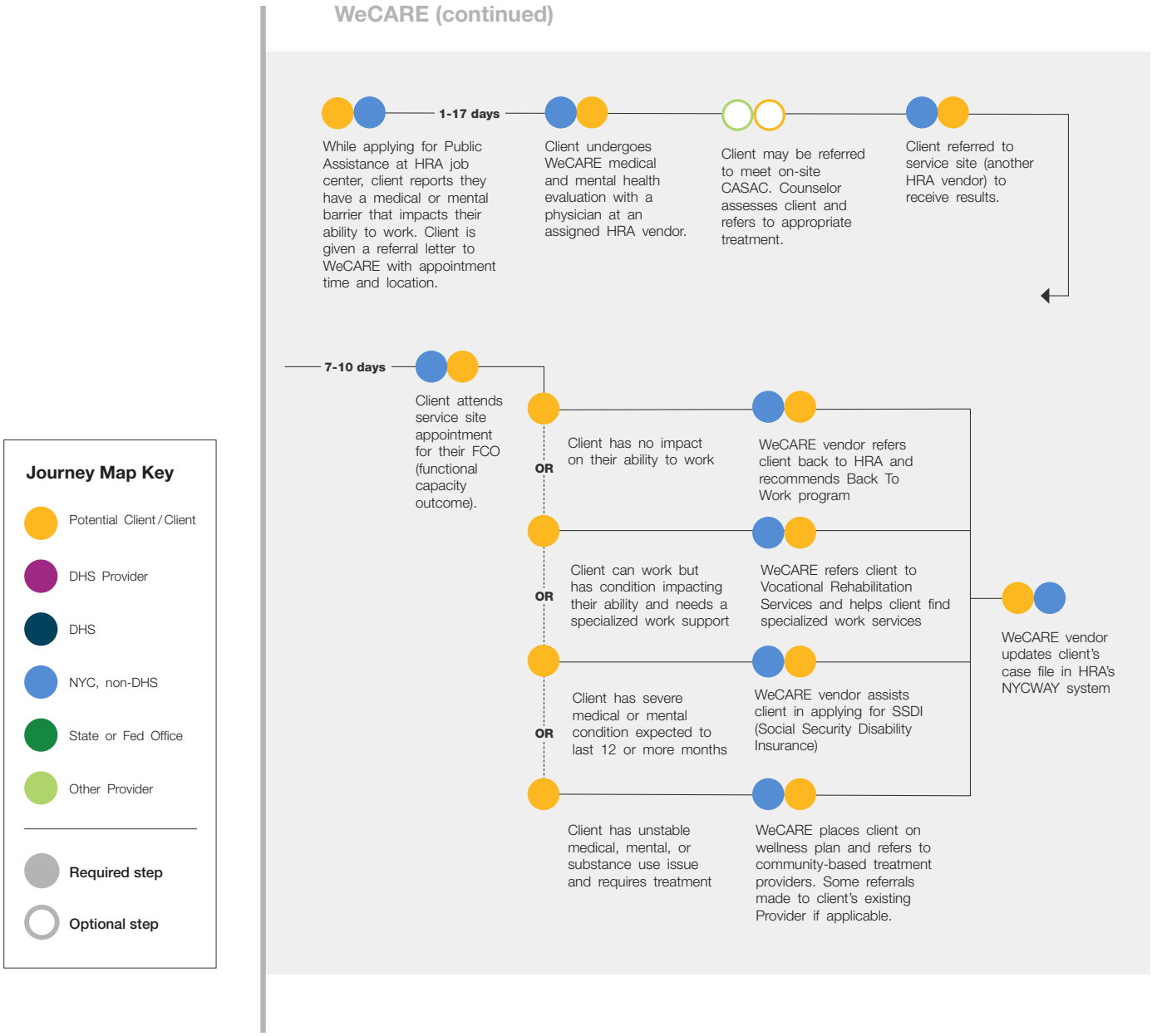
As part of the application process, clients are interviewed and assessed for employment. If the HRA worker determines that the client is able to work, they are required to participate in the Back to Work program. If the client reports having a medical or psychological barrier to self-sufficiency, they are referred to WeCARE (Wellness Comprehensive Assessment Rehabilitation Employment) for a medical and mental health evaluation by a contracted service provider. At their assigned WeCARE appointment, the client is seen by a physician and undergoes the following:

- A physical
- A psychosocial interview
- Urine and blood testing
- A psychiatric assessment by a psychiatrist if needed
- Review of any medical documentation they provide the physician

After the examination, the physician will determine the client's functional capacity outcome (FCO). If the client has medical or mental condition expected to last 12 or more months, WeCARE will assist the client in applying for Federal Disability Benefits (SSI/SSDI), including obtaining additional supporting medical documentation. If the client needs medical treatment to stabilize their condition, they will be placed on a Wellness Plan, which includes linking with appropriate treatment providers, monitoring and facilitating compliance and progress in treatment, case management, and health education.

If the physician determines the client can engage in work-related activities, they are referred back to HRA for the Back To Work program, or if they can work but require specialized work support, to Vocational Rehabilitation Services that help clients find work that accommodates their condition in addition to case management, medical treatment, health education, and job retention services.

Clients are given 1-5 days to respond to the physician's recommendation. If they are unable to respond in that time period, they are given an additional 11 days to follow-up and in rare cases may request even more time. WeCARE is not a determinant for Public Assistance eligibility.



Homebound Program

All Providers reported that the Homebound program is the most effective for their clients. The Homebound program has no work or substance abuse treatment requirement and also provides field workers from HRA who can travel to the client to guide them through the application process and submit their information into HRA's case systems. Providers or their clients may request assistance from Homebound at any point in the process. HRA has recently made Homebound easier for homeless individuals to access through an improved DHS referral form. Providers must submit documentation of medical or mental limitations and a letter identifying the client.

"No clients should have to work everyday before they have a home."

— Provider

Homebound Program (continued)

Providers are developing good working relationships with staff from the program and report that they are knowledgeable and communicate pertinent information that has been helpful to their client's cases.

Once the benefits are active, the clients must adhere to the requirements of their Public Assistance. Clients who aren't on the Homebound program will have either work or substance abuse treatment requirements as a condition of their public assistance case. In some cases, Providers report that clients with ongoing substance abuse issues will be required to attend work programs. These programs require regular attendance and, for back to work programs, professional dress. Especially for clients who are still living on the street, meeting these standards is nearly impossible. If any component of the required program is missed, the case will be sanctioned and the client's Public Assistance income will be turned off.

Sanctions

Providers report that sanctions and closed cases happen very rapidly. If this happens, the client's housing packet will be put on hold until they have an active income again. Without this, the client will not be considered for most permanent placements, including LINC and supportive housing. HRA has recently begun conducting extensive reviews before issuing employment-related sanctions in addition to giving the client 10 days to engage with their case to address this matter. They are also working with the Office of Temporary and Disability Assistance to further develop the process.

Recertification

At least once per year, clients are required to recertify their benefits. If the client misses any of the recertification appointments, the case will be sanctioned and the clients Public Assistance income will be turned off. DHS reports that there is no straightforward way to check when a client's Public Assistance needs to be recertified, making it difficult to plan to avoid any sanctions.

"If someone chooses the wrong [Medicaid] plan, then it's almost impossible to get them care."

— Provider

Medicaid

In addition to Public Assistance, the client will also need active health insurance to be eligible for some types of housing. Clients are given 90 days to select a Medicaid plan. If a plan is not chosen, they are auto-enrolled in one with the option to change their plan within 30 days. Providers reported that while applying for Medicaid is straightforward, choosing the wrong plan can lead to obstacles in the future.

Medicaid (continued)

One Provider reported a case where a client was ready for addiction rehabilitation, but the constraints of their Medicaid meant that there were almost no facilities in New York where the client could receive treatment. According to HRA, there are some Medicaid health plans that include HRPs (health and recovery plans) that are specific to clients with mental health and substance abuse issues. These plans provide greater flexibility and access to clinics that serve the HRP network specifically. Providers report not being aware of these plans and are unclear if it is the responsibility of HRA workers to share this information when a client is asked to choose a plan.

Accessing vital documents in CARES

DHS reported that, once a client's benefits application is processed, the Agency should be able to generate a Public Assistance budget letter via the CARES link to WMS. DHS Analysts report that the WMS link in CARES is unreliable and usually does not reflect the latest information in WMS. DHS reports HRA does not accept a budget letter generated via CARES as official documentation. Instead, the case manager and client must return a fifth time to the HRA Center to retrieve the official budget letter.

Enhancements

Applying for Public Assistance

These enhancements were generated by DHS, Providers, and the Mayor's Office of Operations and include ideas for policy, funding, program design, service delivery, technology and data. They are organized by long-term, medium-term, and short-term timeframes.

LONG-TERM

Design a Public Assistance process and track just for chronically homeless people to better deliver services where they are and within their needs.

Next Step: Convene HRA, DHS and Providers to create an ideal process flow. Follow with a plan of action.

Also found in: Cross-Agency Initiatives

Allow all Provider-vouched for, chronic street homeless clients to be on the Homebound program until proven otherwise.

Next Step: Follow-up on the most common reasons that clients are rejected from Homebound program.

Also found in: Cross-Agency Initiatives

Balance the Public Assistance allotment between Safe Haven ($45/month) and street client's ($264/month) to keep from discouraging people to move inside.

Next Step: Follow-up on the factors that affect the allotment decisions for Public Assistance.

Also found in: Cross-Agency Initiatives

LONG-TERM (continued)

Create a resource guide/database to the different processes and policies for entitlements and housing.

Be able to track types of inquiries for understanding where pain points are.

Next Step: Work with Providers and DHS business to identify top questions and needs.

Also found in: Applying for Permanent Housing; Data and Systems

Shorten the statutory time period (currently 45 days for singles) to receive benefits so that clients are not left waiting and can begin the housing application and matching process.

Next Step: Review Public Assistance wait times with HRA.

Also found in: Cross-Agency, Applying for Permanent Housing

MEDIUM -TERM

Add "preventive social services" to hospital ERs including housing, entitlements, etc.

Also found in: Health Crises

Tie the Public Assistance and Housing application processes more closely together.

This could help avoid situations where Public Assistance is turned off for clients in the middle of their housing application process.

Next Step: Establish a first meeting at the leadership level to begin developing shared goals.

Also found in: Applying for Permanent Housing; Cross-Agency Initiatives

MEDIUM -TERM (continued)

Make HRA centers more friendly.

Allow lunch breaks or snacks, provide food on site, limit the length of time people are required to stay for appointments, and text clients or case managers when their turn comes while waiting.

Next Step: Explore possibilities for improving the user experience. Develop relationships with food vendors.

Also found in: Cross-Agency Initiatives

Provide more staffing at HRA centers. HRA to also consider staffing up during busy time periods.

Next Step: HRA to develop plan to increase staff and analyze current flow at job centers.

Also found in: Cross-Agency Initiatives

Create policy so that one visit to HRA will cover all necessary required steps so clients do not have to go back multiple times.

All HRA centers should have the same procedures and offer the same types of Public Assistance-related services.

Next Step: Convene HRA, DHS and Providers to create an ideal process flow. Follow with a plan of action.

Also found in: Cross-Agency Initiatives

MEDIUM -TERM (continued)

Make work requirements and substance abuse treatment for Public Assistance more flexible for the street homeless.

It is challenging for those still on the street to keep appointments if they do not have a home, place to shower, or keep clean clothing. HRA should consider a suspension until a client is housed. Providers report that current requirements undermine the relationship between caseworker and client.

Next Step: Review work requirements with HRA.

Also found in: Health Crises

Explore creating mobile vans that go to where individuals are on street, Providers' offices, and transitional housing locations.

Services could include: medical, psychiatric services, idNYC, Homebound Public Assistance and other HRA services. Hours and location would vary by needs. The vans could also serve high need and visible areas with non-chronics (e.g. Union Square). The program needs to avoid encouraging people to stay outside while still serving service resistant street homeless.

Next Step: Review current mobile services in the City to see how they could be extended.

Also found in: Health Crises

Make IDNYC more user-friendly for the street homeless.

Ideas include a mobile application that goes out to homeless, opportunity to get ID without making an appointment, and avenues for clients to get IDs before they may get other photo IDs.

Next Step: Meet with IDNYC team to discuss population needs and programmatic solutions.

Also found in: Cross-Agency Initiatives

MEDIUM -TERM (continued)

Revise the benefit card replacement process so that Provider staff and clients do not get disconnected from HRA's information line nor need to go into an HRA Center to receive one.

Next Step: Determine why the Providers and clients can not use the 311 card replacement service request.

Also found in: Gathering Vital Documents; Cross-Agency Initiatives

SHORT -TERM

Assign one HRA Center in each borough that serves homeless individuals.

Next Step: Identify best centers per borough with HRA.

Also found in: Cross-Agency Initiatives

Implement a protocol for DHS Analysts to contact HRA when there are issues with Public Assistance or housing applications.

Next Step: Create new position at DHS to manage new protocol and identify key HRA contacts for Public Assistance and 2010e issues.

Also found in: Cross-Agency Initiatives

Designate HRA Center Supervisors or leadership as key contacts for Provider staff and leadership to escalate issues and give feedback.

Next Step: Identify staff contacts from Providers and HRA.

Also found in: Cross-Agency Initiatives

Section 7

Securing an Income Other than Public Assistance

- Applying for Supplemental Security or Social Security Disability Income
- Documenting Veterans benefits
- Documenting pension benefits
- Documenting informal income
- Getting proof of income (for employed clients)

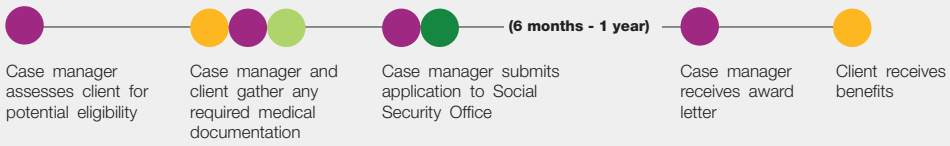
While Public Assistance is the most common income to secure for the chronically street homeless, some may be eligible for Social Security or Veterans benefits, and others may be receiving employment income, informal income, or pension benefits.

Applying for Supplemental Security or Social Security Disability Income (SSI or SSD)

6 MONTHS – 1 YEAR FROM APPLICATION

Social Security is an option for many clients and the application process can only be started once vital documents are obtained. Providers report that some clients will refuse Public Assistance if they are in the process of appealing an SSI case, out of fear of having their SSI appeal rejected due to the new income.

A Provider also told a story of a woman who was receiving enough social security to pay for housing, but they were encountering issues in obtaining documents so she remained on the street.



Tools and Resources

- Award letter added to digital and physical case file
- Application and status of SSI or SSD tracked as part of the housing packet

Journey Map Key

- Potential Client / Client
- DHS Provider
- DHS
- NYC, non-DHS
- State or Fed Office
- Other Provider

- Required step
- Optional step

Applying for SSI or SSD (continued)

The following documentation is required to apply for SSI or SSD:

- Application forms
- Social Security card
- Birth certificate
- Photo ID
- Medical documentation

Documenting Veterans benefits

Clients receiving VA benefits can submit those benefits as proof of income in their housing application.

Documenting pension benefits

Clients receiving pensions can submit those benefits as proof of income in their housing application.

Getting proof of income (for clients who are employed)

1 DAY

Some clients are employed. These clients can submit their earnings as proof of income in their housing application.

Journey Map Key

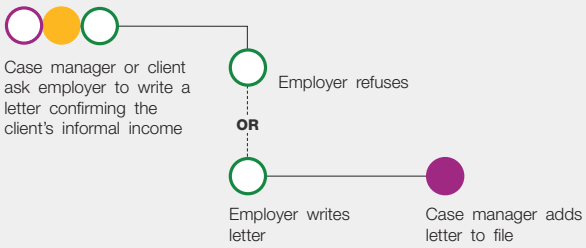
- Potential Client / Client
- DHS Provider
- DHS
- NYC, non-DHS
- State or Fed Office
- Other Provider

- **Required step**
- **Optional step**

Documenting informal income

VARIES

DHS reported that documented informal income can be used to help a client qualify for certain programs. However, employers may be reluctant to provide a letter because of the tax or legal implications.



Tools and Resources

- Proof of income added to digital and physical case file
- Proof of income tracked as part of the housing packet

Journey Map Key

- Potential Client / Client
- DHS Provider
- DHS
- NYC, non-DHS
- State or Fed Office
- Other Provider

- **Required step**
- **Optional step**

Section 8

Applying for Permanent Housing

- Submitting an HRA 2010e Supportive Housing Referral
- Resubmitting an HRA 2010e Supportive Housing Referral
- Applying for general population housing
- Obtaining an HPD Section 8 voucher
- Living in Communities (LINC) Rental Assistance Program
- HPD'S Homeless Set Aside Units
- Waiting for housing to become available

Enhancements: page 113

There are a range of housing types that clients may be eligible for including different levels of supportive housing, specialized placements for groups like veterans or people living with HIV/AIDS, or general affordable housing with a range of funding sources from Section 8 to the Living in Communities (LINC) Rental Assistance Program. To access most opportunities, the Provider must submit a housing packet to the Human Resources Administration (HRA) or Housing Preservation and Development (HPD) on the client's behalf.

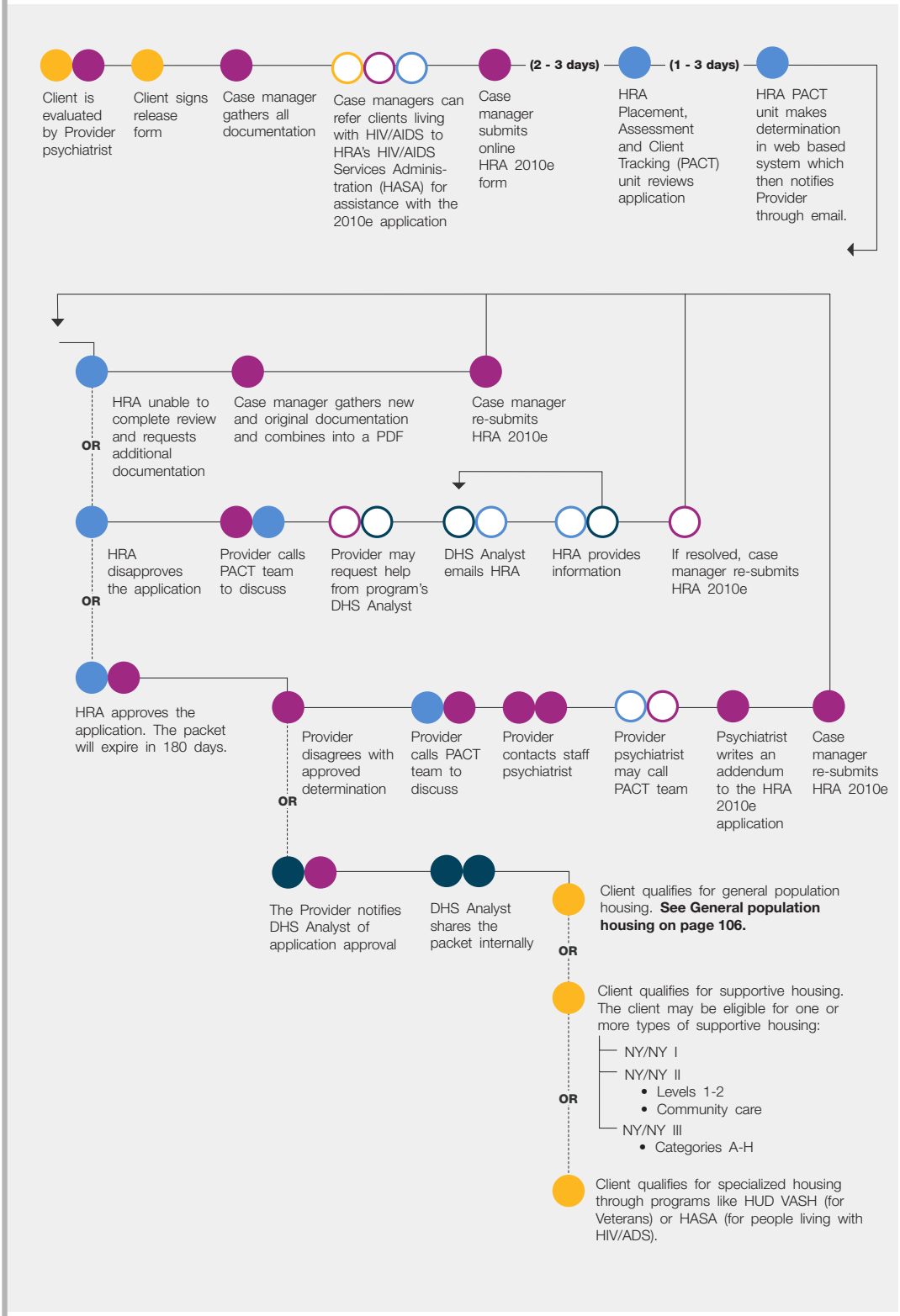
Submitting an HRA 2010e Supportive Housing Referral

1 - 6 MONTHS

Providers report that 60-70% of their clients will require supportive services in their permanent placement. Many of these clients will apply for supportive housing through the HRA 2010e process. This provides access to permanent housing, coupled with varying levels of supportive services to help the client live independently.

(see journey map on next page)

Submitting an HRA 2010e Supportive Housing Referral (continued)



Journey Map Key

- Potential Client / Client
- DHS Provider
- DHS
- NYC, non-DHS
- State or Fed Office
- Other Provider

- **Required step**
- **Optional step**

Submitting an HRA 2010e Supportive Housing Referral (continued)

Tools and Resources

- Status of individual housing packet is noted in digital and physical case file
- Copy of HRA Approval Letter is added to digital and physical case file

The status of housing placements is reported to DHS on a daily to weekly basis

A completed HRA 2010e Supportive Housing Referral packet includes:

- Cover letter
- Birth certificate or naturalization papers
- Photo ID
- Social Security card
- Proof of active health insurance
- Award or Budget letter or proof of income from the last 30 days
- Physical Exam Results, no more than 1 year old
- Psychosocial evaluation, no more than 6 months old
- Psychiatric exam, no more than 6 months old
- Proof of homelessness
- Long term shelter stay letter
- Signed release form

The HRA 2010e Supportive Housing Referral packets are reviewed by HRA's Office of Health and Mental Health Services/Placement, Assessment and Client Tracking Unit (OHMHS/PACT).

The psychiatric evaluation is used to inform the recommended level of care the client will need in their permanent housing placement. This level of care is a key factor in the types of housing for which a client can qualify. Providers mentioned that some clients are hesitant to meet with the staff psychiatrists. This can lead to some delay until the client is ready for the evaluation.

Many clients will require supportive services in order to remain in their permanent housing placement. For these clients, Providers will include a recommendation for placement into a supportive housing facility as part of the 2010e submission. Providers report that clients who require supportive services do not always receive that designation from HRA. Clients who need support to live independently but are only considered eligible for general population housing face major challenges to remaining housed after placement. Especially for clients who are persistently mentally ill, getting an appropriate determination can be particularly challenging.

Submitting an HRA 2010e Supportive Housing Referral (continued)

Providers mentioned that the online HRA 2010e form requires them to repeat much of the information covered in the supporting documents. In addition, if any changes are requested, the online form must be refilled in it entirety and re-submitted again. This can be a time consuming process for Providers.

"For any type of housing, the supply just doesn't meet the demand."

—Provider

Sometimes housing packets are disapproved or will "get stuck" in the HRA process and DHS Analysts will step in to help. Analysts mentioned that, while there are HRA liaisons at DHS, there is no specified protocol for contacting HRA about issues with housing packets. Hold ups like this were cited as a major obstacle to clearing up any issues that might arise during processing. Both Providers and Analysts reported that reasons for disapproval by HRA can be unclear and difficult to follow up on.

The availability of types of housing is also a key consideration for Providers and clients when choosing types of housing. For instance, clients with a determinations for NY/NYIII Category E (active substance users) are unlikely to receive that kind of housing, because the supply in New York City is very low. Providers will either try to get the determination changed to Category A (Chronically homeless single adults who are diagnosed with a serious and persistent mental illness (SPMI)) or as mentally ill and chemically addicted (MICA), which has a higher supply of housing, or encourage those clients to go into general population housing, where they will not have all of the supports in place that they may need to live independently.

Providers reported that inappropriate determinations from the PACT team have recently become less of an obstacle, however they are reporting inconsistencies in determinations. They cited recent cases where clients had similar conditions but were given different housing designations and PACT reviewers said there was not enough evidence for Category A. The Providers are often unclear why this happens or how to plan for it. They emphasized the importance of training case managers and psychiatrists in how best to document a client's condition in order to secure the most appropriate determination.

Once a supportive housing application has been approved, the client receives an HRA Approval Letter which includes the client's housing eligibility information.

All Providers report the status of housing packets to DHS on a monthly basis. Movement of housing packets and placement into housing are critical goals for all of the programs.

BRC Housing packets are tracked in AWARDS

Submitting an HRA 2010e Supportive Housing Referral (continued)

Breaking Ground	Housing packets are tracked in Excel "Assembly Line" spreadsheet that includes estimated time to housing packet completion between 1 - 4 months and estimated months until placement for completed packets
BronxWorks	Housing packets are tracked in Excel spreadsheet
MOC	Housing packets are tracked in MOCapp
Project Hospitality	

Resubmitting an HRA 2010e Supportive Housing Referral

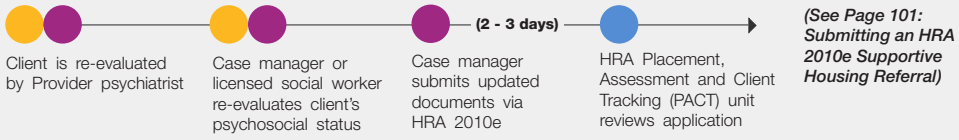
EVERY 180 DAYS

"You can't be doing too well."

—Provider

HRA 2010e housing packages need to be updated and submitted every 6 months after the determination date. The main application must be updated, but the supporting documents can be a few months behind. DHS staff reported that most clients will have to submit at least two housing packets before being housed because original applications expire. Because clients will usually wait through multiple housing package cycles, a client's condition will sometimes change between evaluations.

Providers shared examples of clients receiving services to address mental health or substance abuse issues while waiting for permanent housing. When their package is renewed, the HRA determination may change to a lower level of care. From the Provider's perspective the client's condition has improved because of those services, while the HRA determination assumes that the client no longer needs them. Providers report that challenging the HRA determination, especially for particularly vulnerable clients, can be time consuming and frustrating.



(See Page 101: Submitting an HRA 2010e Supportive Housing Referral)

Tools and Resources

- Updated psychosocial evaluation
- Updated psychiatric evaluation
- The status of housing placements is reported to DHS on a daily to weekly basis

Journey Map Key

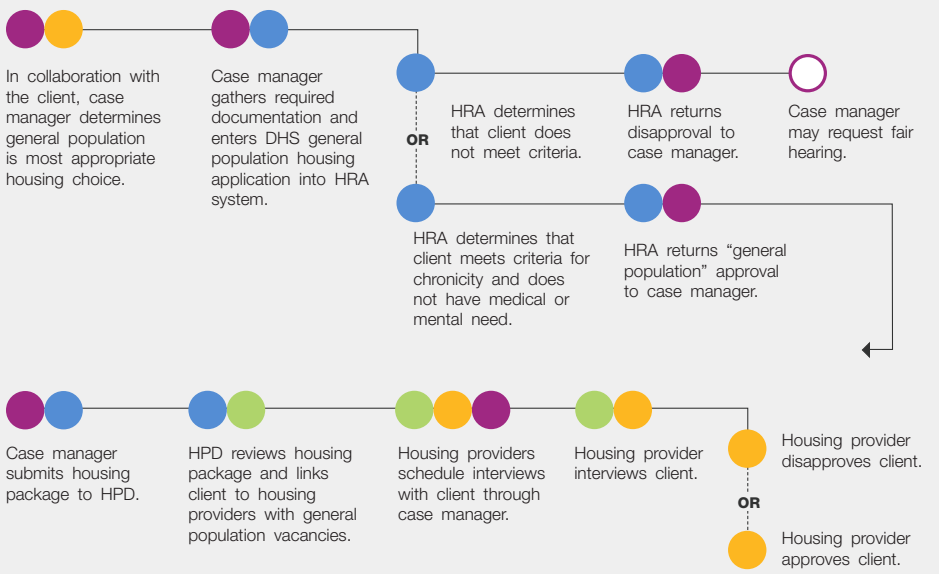
- Potential Client / Client
- DHS Provider
- DHS
- NYC, non-DHS
- State or Fed Office
- Other Provider

- Required step
- Optional step

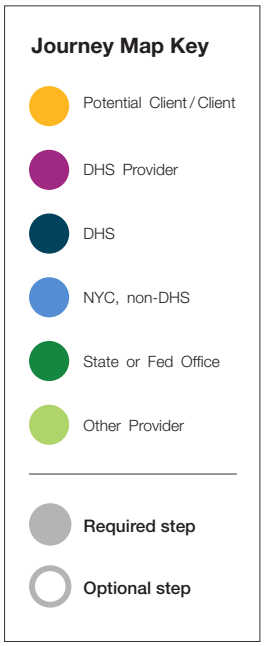
Applying for general population housing

6 MONTHS

For clients who are capable of living independently, general population housing may be the best option for permanent housing. Providers report encouraging some clients who have received determinations like Category E (ongoing substance abuse issues) from HRA, to apply for this type of housing. General population housing is housing without NY/NY categories and requirements. Some general population housing units provide social services but not as extensive as NY/NY. Recommendations for general population housing are made by DHS and the Providers in collaboration with the client, based first on the absence of a medical or mental need and second if the client meets the criteria for chronicity. HPD reports that they there is a lack of transparency in how DHS makes these recommendations.



Tools and Resources
The status of housing applications and placements is reported to DHS on a daily to weekly basis.



A complete DHS general population housing package includes:

- Cover letter
- 7 page application
- Psychosocial interview
- Proof of chronicity

Applying for general population housing (continued)

- Birth certificate
- Social Security card
- Proof of income, no more than 30 days old
- Photo ID

The application, to include all its contents, are valid for 6 months from the date the psychosocial assessment was completed. Psychiatric assessments are not required for general population housing applications.

Obtaining an HPD Section 8 voucher

6-9 MONTHS ON AVERAGE

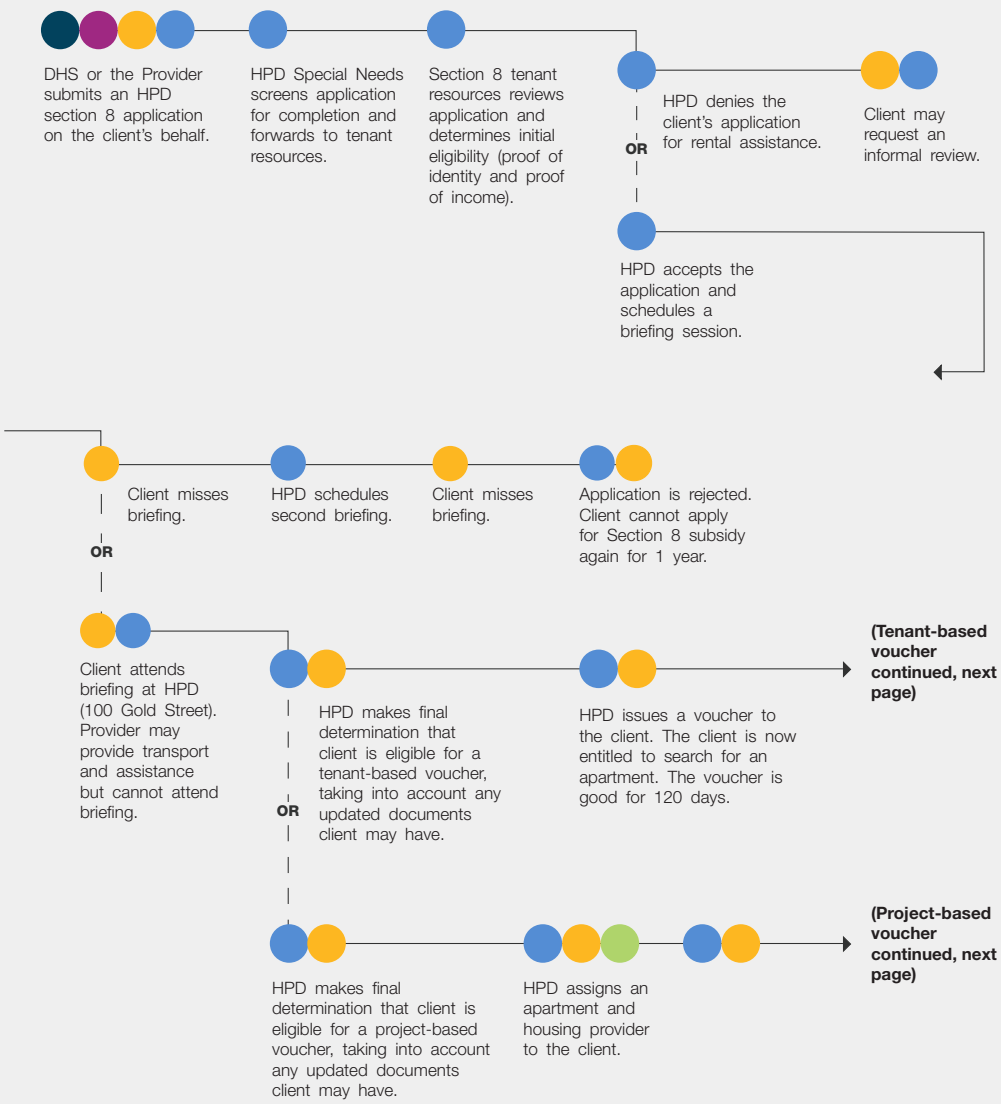
For eligible clients, the City may provide an HPD Section 8 rental subsidy for their permanent placement. Section 8 was created to protect the homeless and burdened families and prevent displacement through the development of supportive housing. HPD's Section 8 includes about 33,000 tenant-based units that are closed to the public and are for HPD programmed developments and DHS referrals. 40% of their clients are low income and 60% are from shelters that are referrals straight from DHS. Last year HPD assigned 500 tenant-based placement spots specifically to the shelter population. There are two primary types of Section 8 programs: project-based or tenant-based. HPD tracks all Section 8 supportive housing client information, applications, vacancies, placements in their system called ELITE.

Project-based vouchers are assigned to specific units and will not apply to the client if they decide to move to another placement within a year. When a client is approved for this type of rental subsidy, HPD will assign a unit to the client. In these cases, the client is now "linked" to a placement. After one year, clients have the option of moving to a market-rate unit where their voucher then becomes tenant-based. The original unit will remain tied with project-based vouchers.

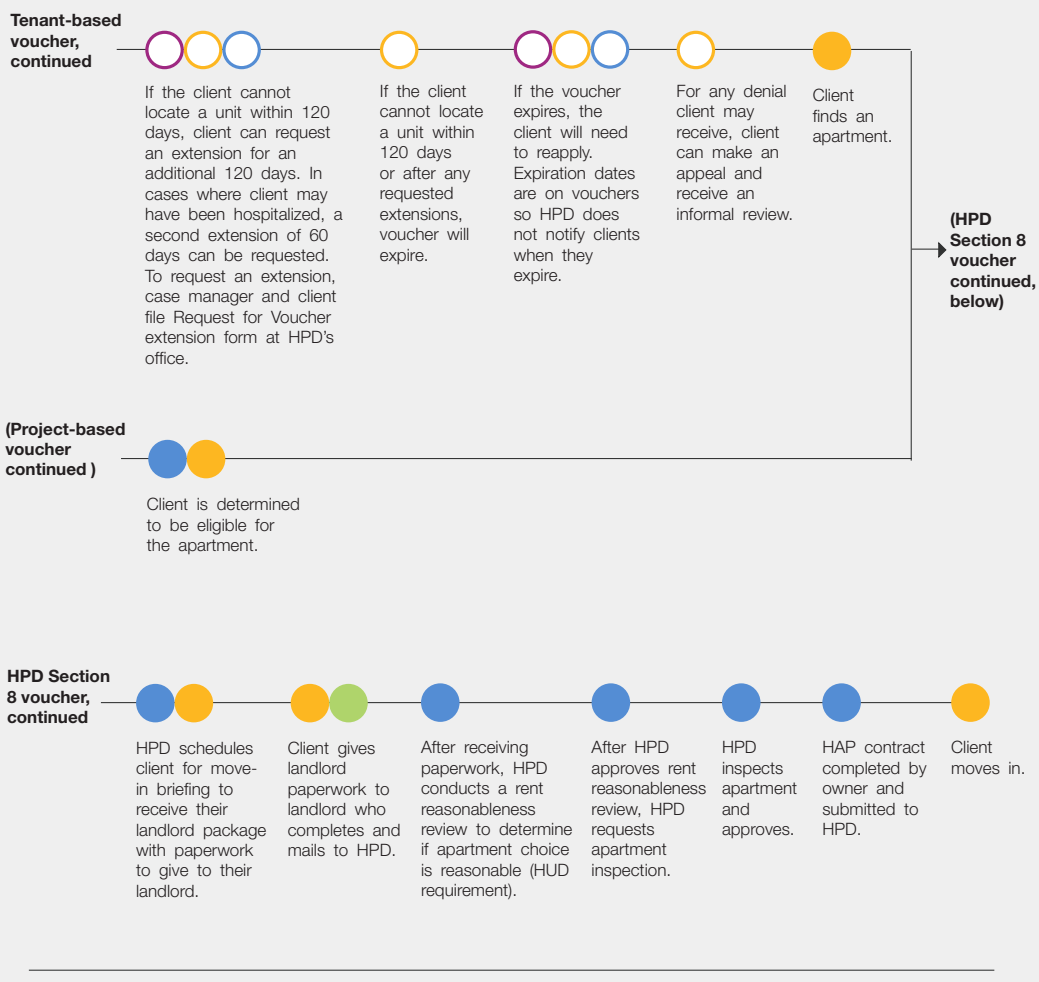
Two common types of housing available to street homeless clients through project-based vouchers include Shelter Plus Care and Section 8 Moderate Rehabilitation Single Room Occupancy (SRO Mod) for Homeless units. Shelter Plus Care housing is intended for hard-to-serve homeless individuals with disabilities and includes supportive services at the placement. SRO Mod housing is designed to provide safe housing for homeless individuals in privately-owned buildings and does not necessarily include supportive services as part of the placement.

Obtaining an HPD Section 8 voucher (continued)

Tenant-based vouchers are provided directly to the client and can be applied to any unit, as long as the unit meets NYC Housing Preservation and Development's (HPD) Housing Quality Standards and the terms of the lease are approved by HPD. The largest and most commonly used tenant-based subsidy initiative is called the Housing Choice Voucher program, which assists very low-income individuals and families in obtaining private, market-rate housing. If a client is found eligible for a tenant-based Section 8 voucher, they will be required to attend a briefing at the HPD office before they can receive the voucher.



Obtaining an HPD Section 8 voucher (continued)



Tools and Resources
The status of housing packets is reported to DHS on a daily to weekly basis.

In order to apply for Section 8 assistance, applicants must provide:

- Proof of citizenship or eligible immigration status
- Proof of social security number

HPD is serves approximately 5,400 clients per month at 100 Gold Street, where as they were previously only serving around 1,000. HPD conducted a recent survey in response to the perception of inconvenience and their overwhelming attendance at the service center (on the 1st floor of 100 Gold Street).

Obtaining an HPD Section 8 voucher (continued)

"Briefings are always held at HPD's offices on 100 Gold Street which is limited in parking and difficult to get into."
—Provider

The survey included residents from Manhattan, the Bronx, and Brooklyn and most clients chose 100 Gold Street as their preferred location as opposed to being in the boroughs. HPD has asked the Department of Citywide Administrative Services (DCAS) to find them a new centralized space for their growing program. HPD cited the close location to the Fulton Transit Center as an amenity. Providers, however reported 100 Gold Street as a challenging location to bring clients to due to limited parking, as they transport many clients in their vehicles.

If the client is still deemed eligible following the briefing, HPD will issue a voucher. The voucher will be active for 120 days. The voucher does not admit the client to the Section 8 program, but rather entitles the client to search for a placement using the voucher. The housing voucher details all of the program requirements that the apartment will need to meet in order to be approved for the rental subsidy.

Risks for client missing a briefing:

- Client loses contact with case manager
- Hospitalization
- Jailed
- Entered in detox program
- Moving around or back on the street

After the briefing, the client receives:

- Section 8 voucher
- HPD Landlord Package

For clients who are applying for public housing via the Section 8 program, there are three main barriers that may keep them from receiving a rental subsidy. These include:

- Sex offender status
- Previous convictions for manufacturing methamphetamines in federally-funded housing
- Owing rent to HPD or any housing authority or the federal government

Other barriers that may keep clients from gaining access to other housing opportunities include:

- Bad credit
- Criminal justice system involvement
- Previous eviction from federally-funded housing
- History of arson
- Previous termination for Section 8 assistance
- Drug use or related activity

Obtaining an HPD Section 8 voucher (continued)

- Reasonable cause to believe that substance use, including alcohol, may threaten the right to peaceful enjoyment of the premises by other residents
- Failing to provide information
- Failing to attend a briefing

Living in Communities (LINC) Rental Assistance Program

The Living in Communities (LINC) Rental Assistance program is another housing subsidy option for Providers and their clients to choose from. LINC was created jointly by HRA and DHS to target homeless individuals and families. To be eligible for LINC, clients must have an active case in CARES as of April 1, 2016, be employed for at least 30 days or receiving federal disability, and have an active or single issue public assistance case. HRA runs a daily match with DHS to generate LINC certificates for clients who meet the eligibility requirements.

LINC is different from Section 8 and provides different incentives to landlords for participating in the program. With LINC, landlords are offered a signing bonus if they participate and can evict a tenant after 5 years if they cannot continue to afford the unit. LINC also has a less stringent building inspection process. HPD reports that based on these differences and tenant income, landlords have become particular about choosing tenants which has created competition in an already saturated market. This extends the amount of time it takes to find housing for clients eligible for LINC.

HPD's Homeless Set Aside Units

HPD's Homeless Set Aside units are normal affordable housing units with no additional social services. These units are set aside by regulatory agreement for housing projects and are restricted for individuals or families exiting shelter. It is highly unlikely that Providers would seek out this type of housing for their clients.

Waiting for housing to become available

MONTHS - YEARS

All Providers reported a critical shortfall of housing opportunities for their clients. Especially for certain types of housing, wait lists can be months to years long. For Example, NY/NY11 Category F Housing (for clients who have completed a substance abuse program) only has about 600 units city-wide and turnovers and vacancies are rare.

During this time, clients may be moving between transitional settings, completing treatment or rehab programs, or living on the street. Providers reported that in this stage, clients are often not interested in day-to-day updates, but want to know bigger details like major changes in their case and when they might be permanently housed.

At this stage, clients will continue to face risks to their housing placement, including:

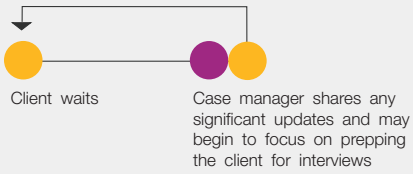
- Hospitalization
- Arrest and jail time
- Detox
- Benefits case sanctioned and closed
- Missed appointments
- Losing contact with case manager for unknown reasons

"You can complete this process and then not receive housing."
—DHS

Journey Map Key

- Potential Client / Client
- DHS Provider
- DHS
- NYC, non-DHS
- State or Fed Office
- Other Provider

- Required step
- Optional step



Time
Depending on the type of housing, clients can wait for long periods of time in this stage.

Tools and Resources
The status of housing placements is reported to DHS on a daily to weekly basis.

Enhancements

Applying for Permanent Housing

These enhancements were generated by DHS, Providers, and the Mayor's Office of Operations and include ideas for policy, funding, program design, service delivery, technology and data. They are organized by long-term, medium-term, and short-term time frames.

LONG-TERM

Providers expressed a strong need for more housing so that clients do not have to wait as long to get matched with permanent housing.

Next Step: Review past housing applications to establish which populations need the most new units.

In new housing agreements, ensure that breakdown of units allocated for a specific category (i.e. Cat A, Cat E) matches the need of the actual population looking for housing (i.e. enough Cat E units).

Next Step: Review past housing applications to establish which populations need the most new units and work with HPD's supportive housing task force to streamline the selection process.

Create a resource guide/database to the different processes and policies for entitlements and housing.

Be able to track types of inquiries for understanding where pain points are.

Next Step: Work with Providers and DHS business to identify top questions and needs.

Also found in: Applying for Public Assistance; Data and Systems

LONG-TERM (continued)

Transition housing and housing subsidy applications currently on paper to digital with the option to pre-populate information from other City data systems.

Next Step: Create a working group with HPD IT to begin plan of action.

Also found in: Data and Systems

Allow required and validated documents from a City agency, such as HRA, stand in place as acceptable documents for another agency, such as HPD or DHS.

Next Step: Convene a working group to develop a plan of action.

Also found in: Gathering Vital Documents

Create transitional and permanent housing solutions for individuals with pets.

Explore with new Safe Haven sites.

Next Step: Look into new pet-friendly "navigation center" in San Francisco.

Also found in: Moving Onto Caseload

Create a housing subsidy for undocumented New Yorkers, who often have income, but are reluctant to pay fair market rent and are ineligible for supportive or gen-pop housing.

Next Step: Discuss subsidy with MOIA.

Also found in: Special Populations

MEDIUM -TERM (continued)

Extend the 2010e application/packet expiration date to at least one year.

Next Step: Meet with PACT on updates to 2010e process. Follow-up on which policies govern the expiration dates of 2010e packets.

Also found in: Cross-Agency Initiatives

Base housing placements on mental and physical condition of client on application date, and consider services and supports they have needed to achieve an improved state by expiration date.

Next Step: Meet with PACT on updates to 2010e process.

Also found in: Cross-Agency Initiatives

Allow Providers to update an expired or expiring housing packet without the risk that the client's housing designation with change to an inappropriate designation.

Next Step: Establish a first meeting at the leadership level to begin developing shared goals.

Also found in: Cross-Agency Initiatives

Increase communication and access between Providers and the PACT team to get clients into appropriate housing.

Next Step: Establish a first meeting at the leadership level to begin developing shared goals.

Also found in: Cross-Agency Initiatives

MEDIUM -TERM (continued)

Tie the Public Assistance and Housing application processes more closely together.

This could help avoid situations where Public Assistance is turned off for clients in the middle of their housing application process.

Next Step: Establish a first meeting at the leadership level to begin developing shared goals.

Also found in: Applying for Public Assistance; Cross-Agency Initiatives

Allow for HRA PACT and Providers to case conference around housing packets.

Next Step: Research current PACT work flow to understand the potential impact on current processes.

Also found in: Cross-Agency Initiatives

Section 9

Finding Housing Opportunities

- Finding supportive housing opportunities
- Accessing mental health housing through SPOA

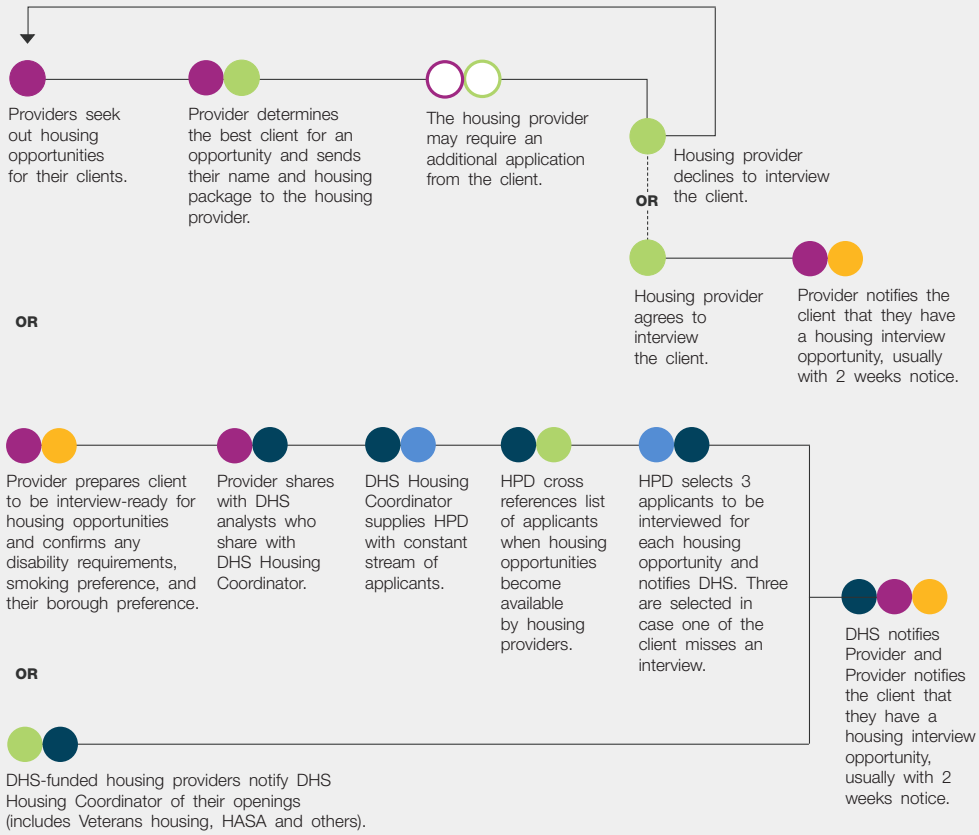
Enhancements: page 122

Once the HRA 2010e Supportive Housing Referral has been approved or Section 8 voucher obtained, Providers work to find housing opportunities for the client.

Finding supportive housing opportunities

VARIES

DHS and Providers become aware of supportive housing opportunities through a number of channels. The paths diagrammed here are not all inclusive, but represent three of the most common ways that housing opportunities are identified for clients. Depending on the funding source of the housing opportunity, there may be more applications, interviews or documentation steps required than are outlined here. This may in turn extend the housing application and placement process further.



Journey Map Key

- Potential Client / Client
- DHS Provider
- DHS
- NYC, non-DHS
- State or Fed Office
- Other Provider

- **Required step**
- **Optional step**

Finding supportive housing opportunities (continued)

Time
- Interview opportunity is noted in digital and physical case file
- Status of housing placements is reported to DHS on a daily to weekly basis

DHS manages a central waiting list of clients and uses a system based on program size to allocate housing units across the Providers

Supportive housing providers are always funded by a combination of HPD, DHS, and DOHMH. HPD pays capital while DHS and DOHMH cover social services. DHS reports that the different funding streams usually dictate how the City finds out about housing opportunities. One Provider reported that about 40-50% of interview opportunities per year come from DHS, while the Providers themselves seek out the remaining opportunities.

"There is too much reporting. We don't have enough time to do the work."
—HPD

HPD reports the status of housing progress, availability, and placements to DHS on a daily to weekly basis, sometimes twice a day. This is a change HPD has made to help keep DHS more informed but have found the increased reporting to be time consuming.

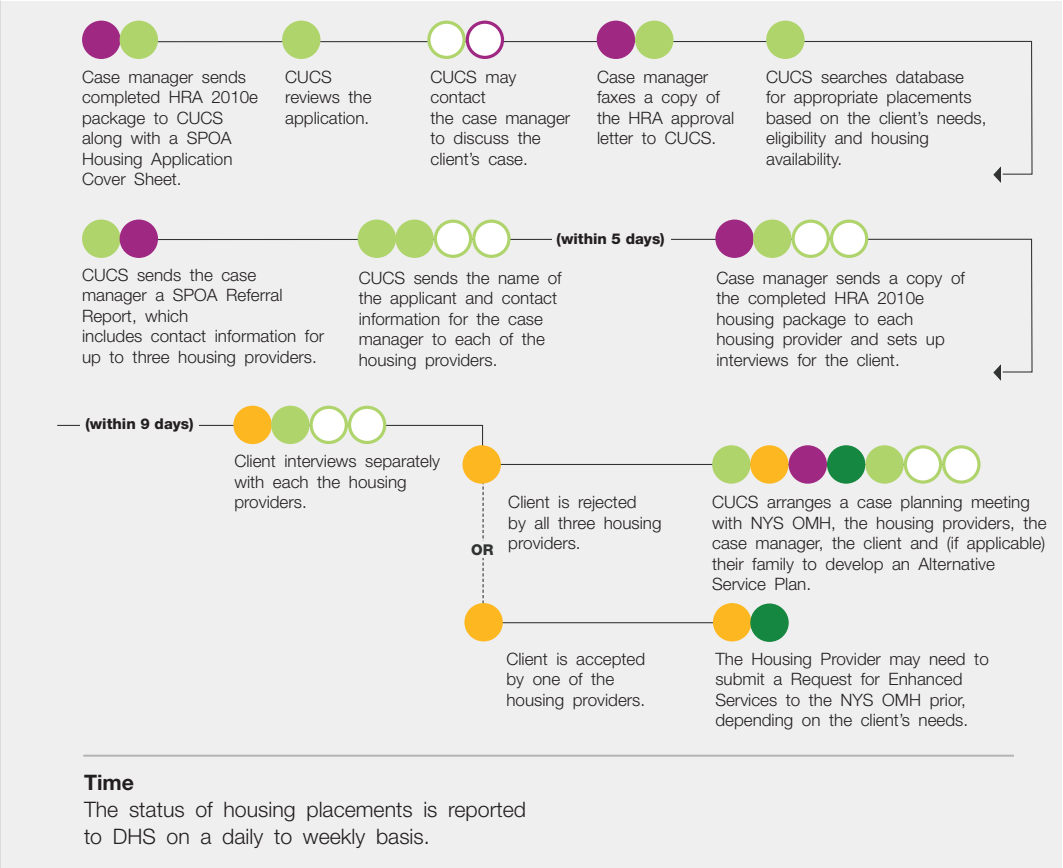
When Providers are notified about housing opportunities, they have to consider a number of factors when choosing which client to recommend for the placement, including:
- Type of opportunity
- Ability of the client to thrive in that particular opportunity
- How long the client has been waiting for placement

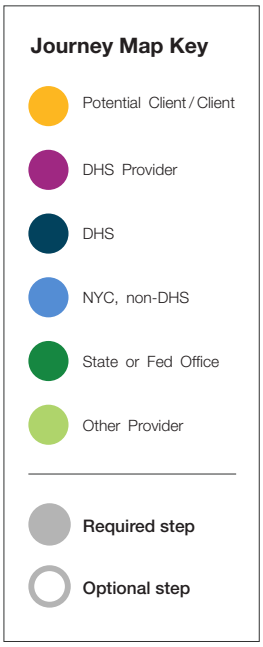
The Providers have a vulnerability index to prioritize clients for housing opportunities. Case managers can also advocate for particular clients to be placed early. Similarly, for specialized placements, analysts can negotiate for particular clients who would thrive in that placement. DHS reports that at times the allocation process is fluid and informal, based on the needs of a particular client or Provider.

Accessing mental health housing through SPOA

2 WEEKS FROM SUBMISSION

For clients with serious mental illness who are not NY/NYI or NY/NYII eligible, Single Point of Access (SPOA) housing, operated by the New York State Office of Mental Health (OMH) and Center for Community Urban Services (CUCS), may be an option. The SPOA program manages a database of vacancies in the mental health housing system in all five boroughs. To qualify, a client must have a serious mental illness, be discharged from a state-operated residence, center or treatment facility and be at risk of homelessness.





The following forms are required to access mental health housing through SPOA:

- Completed HRA 2010e housing packet
- SPOA Housing Application Cover Sheet
- HRA Approval Letter
- SPOA Referral Report
- Housing Provider Response Form

DHS reports that this type of housing may be underutilized due to its extensive application process, but is a good option for eligible clients requiring a higher level of care.

Enhancements

Finding Housing Opportunities

These enhancements were generated by DHS, Providers, and the Mayor's Office of Operations and include ideas for policy, funding, program design, service delivery, technology and data. They are organized by long-term, medium-term, and short-term time frames.

LONG-TERM

Create a portal showing housing available in the City by category, including drop-in and respite bed tracking.

It would provide all details and regulations about a housing opportunity and include photos. Providers would use the portal to showcase opportunities to clients and have them be fully informed before attending an interview. The housing options should promote themselves to clients in hopes of clients selecting them to be their transitional or permanent home.

Next Step: Convene housing providers. Prototype layouts for housing information.

Also found in: Moving onto Caseload; Data and Systems

Conduct research on the history of how landlords are choosing which housing subsidies to accept from clients.

Next Step: Meet with HPD to discuss who can lead this research.

Section 10

Getting Linked with Housing

- Getting Linked to supportive housing
- Rejections by housing providers
- Rejections by clients
- Housing availability and information
- Securing a tenant-based Section 8 rental subsidy

Enhancements: page 130

After clients submit their housing applications, it may take months to years before they are linked with a housing opportunity. There is no indication of when a housing unit may become available and once they are, many steps must be taken between various stakeholders for client selection and interviews. The long wait times between steps present challenges to keeping clients on track.

Getting linked to supportive housing

MONTHS - YEARS

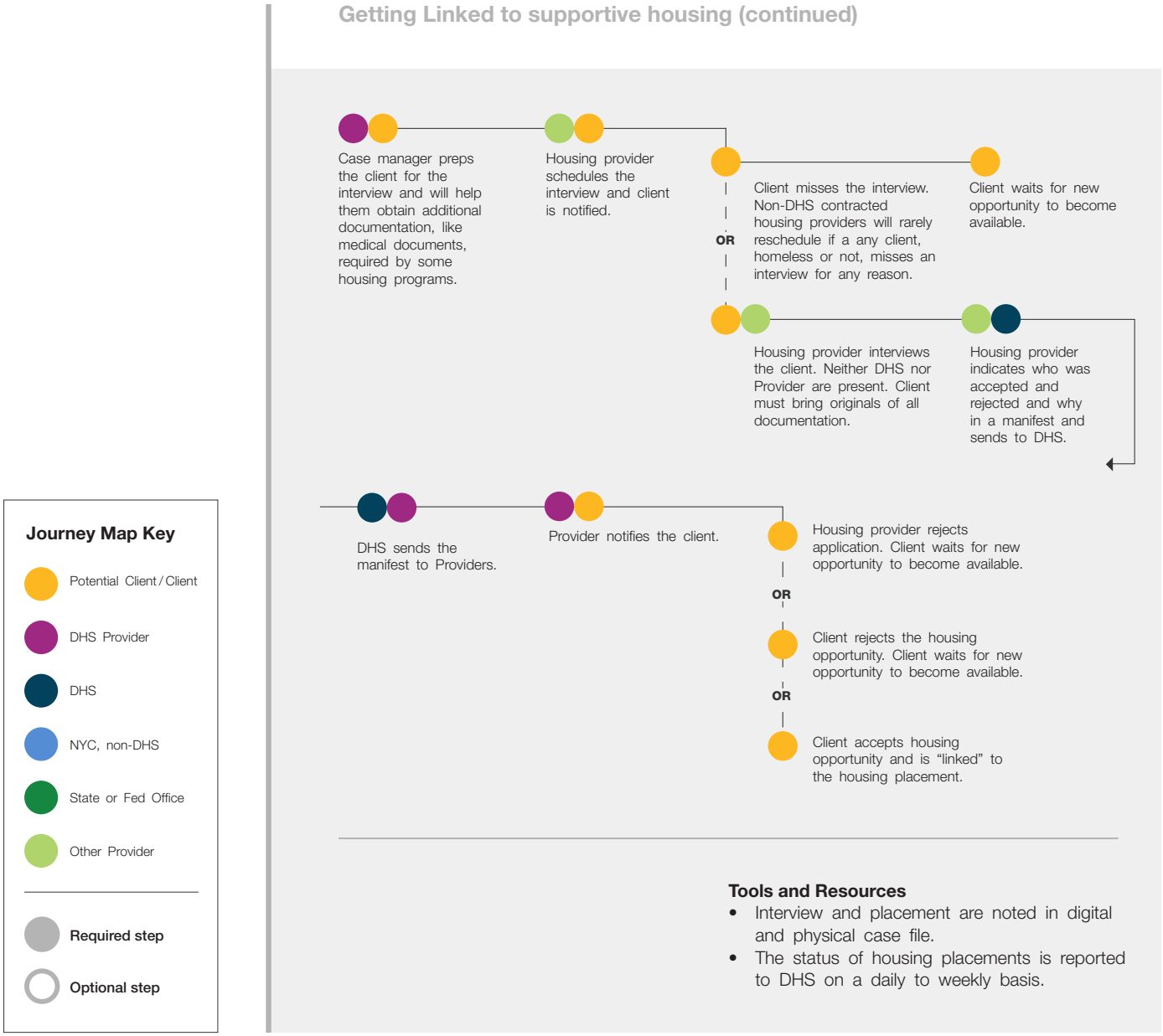
In order to get linked to a supportive housing unit, clients must attend an interview with the housing provider. Interviews can be a difficult process for the client. Clients must also attend their interviews without their case managers and bring all of their documents with them. Case managers will provide support as needed to help clients prepare ahead of the interviews, including:

- Working through traumas
- Coaching them to be honest about their histories with foster care, jail, etc.
- Practice interviews
- Sending them on multiple interviews in a short period

A large percentage of clients have severe mental illness or heavy substance use issues. These conditions can make it difficult for clients to recognize how important the housing interview is to securing permanent housing. While clients are waiting, BRC (Bowery Residents' Committee) will coach them on self-sufficiency and how to live as independently as possible.

HPD requires three candidates for every housing opportunity, increasing the likelihood of a match, but also that two clients will not be accepted.

(see journey map on next page)



Rejections by housing providers

HPD requires that DHS and Providers send three eligible clients to be interviewed for every vacant unit. This makes the interview process competitive and rejections common. HPD reports that it is typical for at least one client to miss an interview so having other candidates creates a better chance of filling an available unit. If a housing provider rejects all three, they must provide HPD with ample reason to receive a second round of candidates.

Rejections by housing providers (continued)

All Providers report that housing providers often reject clients for the same reasons that the client needs supportive housing. Common reasons for rejection include "does not have insight into their mental illness" or "needs higher level of care" and often do not include further explanation. All Providers objected to these reasons, arguing that a key characteristic of mental illness is a lack of insight into the condition. Alternatively, some Providers like Breaking Ground and BRC are able to offer clients placement in facilities that they run.

"Certain housing providers want homeless people without homeless people problems."
—Provider

Breaking Ground tracks acceptance and rejection metrics and reports that outside housing providers will accept clients about 25% of the time. They estimate that if they want to make 100 placements, they will need 400 interviews. Several Providers reported wanting to develop better relationships with housing providers, but have not had the resources to do so.

Rejections by clients

Sometimes clients will reject housing opportunities based on location, size, shared arrangements or vicinity to certain institutions like hospitals. Especially for Staten Island or the Bronx, clients may be reluctant to leave or move to a particular borough. Many clients also reject congregate living situations.

Many times clients will attend interviews for housing units they know they will decline only to preserve their chances for fear of losing their place in line. HPD has made efforts to avoid these situations by requesting that clients choose their preferences in the following categories for housing units:

- Borough location or exclusion
- Disability requirements
- Smoking or non-smoking

Housing availability and information

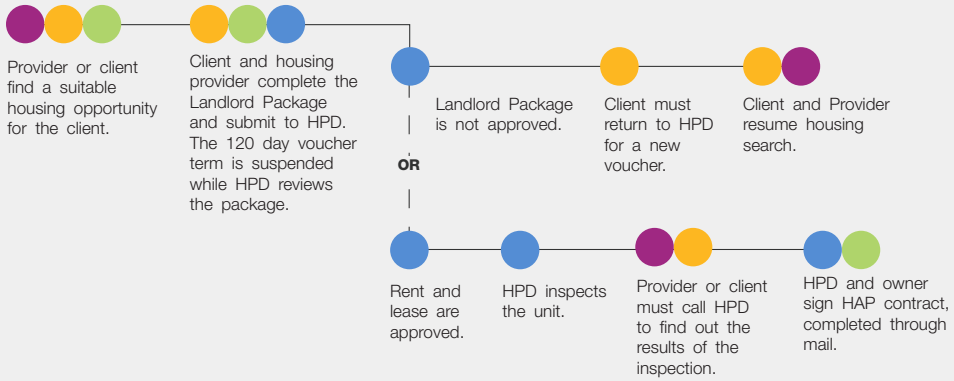
All Providers reported a serious lack of willingness from housing providers to deliver housing to their clients. For supportive housing in particular, housing providers are contracted to deliver services to these clients with all NY/NY agreements setting aside funding and units for mentally ill New Yorkers.

Providers report a lack of information about particular housing opportunities. For instance, when choosing a client to send, the Providers often do not have information about the full range of services at a location, the regulations in that particular building (e.g. curfews, metal detectors) or even the actual location of the building. A few Providers reported maintaining a catalog of units and buildings, but find the process difficult to maintain with limited resources.

Securing a tenant-based Section 8 rental subsidy

WEEKS

For clients who have received a voucher for tenant-based Section 8 rental assistance, there are a number of steps that must be completed before signing a lease. Missing any of these steps can lead to a denial of assistance from HPD and losing the housing opportunity.



Tools and Resources

- Status of housing placements is reported to DHS on a daily to weekly basis

Journey Map Key

- Potential Client / Client
- DHS Provider
- DHS
- NYC, non-DHS
- State or Fed Office
- Other Provider

- Required step
- Optional step

Once the client has found an apartment, the client and landlord complete and submit an HPD Landlord Package, which includes basic information about the unit and lease as well as proof that the rent proposed for the unit is reasonable.

Providers mentioned that clients moving into new buildings face special challenges in this phase. An active Certificate of Occupancy is required for all buildings in the City, including Section 8 subsidy approved units. HPD reports that the Certificate of Occupancy is almost always delayed which further delays a client's Section 8 voucher.

The final step in obtaining a Section 8 tenant-based subsidy is the execution of a Housing Assistance Payment (HAP) contract between the landlord and HPD. The contract describes the landlord's responsibilities under the Housing Choice Voucher program and must be signed before HPD will provide rental assistance.

Securing a tenant-based Section 8 rental subsidy (continued)

After the lease and HAP contract are signed, HPD will send the client a Rent Breakdown Letter, which specifies the dollar amount that the client will have to pay the landlord each month, after the subsidy has been applied.

The following documents are required to secure a tenant-based Section 8 rental subsidy:

- Section 8 voucher
- Housing Assistance Payment (HAP) contract
- HPD Section 8 Landlord Package

Enhancements

Getting Linked with Housing

These enhancements were generated by DHS, Providers, and the Mayor's Office of Operations and include ideas for policy, funding, program design, service delivery, technology and data. They are organized by long-term, medium-term, and short-term time frames.

LONG-TERM

Give Providers the ability to place clients in housing outside of New York City, like in Long Island or Westchester.

Next Step: Look into feasibility of placements and transitioning care to Providers in potential areas outside of NYC using City subsidies.

Change the dynamic between housing providers and clients to be more client-focused.

Process should be less about housing provider accepting the client and more about the client accepting the placement.

Next Step: Establish a first meeting at the leadership level to begin developing shared goals between DHS and other agencies managing the housing providers.

Streamline Certificate of Occupancy process especially with new sites coming along.

Next Step: Work with City agencies including DOB, HPD, DHS, and others who oversee new buildings to enhance the COO process.

Also found in: Cross-Agency Initiatives

MEDIUM-TERM

Publish transparent data about housing providers' client acceptances and rejections for permanent housing.

Create metrics and goals for outreach Providers based on those housing provider rates.

Next Step: Source Provider acceptance and rejection data.

Also found in: Data and Systems

Reduce barriers to Section 8 briefing.

Streamline the process and implement in small steps. Consider virtual briefings (e.g. Skype).

Next Step: Meet with HPD on updates to briefings process.

Also found in: Cross-Agency Initiatives

Develop a mechanism to hold housing providers accountable for unreasonable rejections.

Next Step: Collect data from DHS and Providers on housing providers and historical placements.

Section 11

Transitioning into Housing & Aftercare

- Client applies for One Shot Deal
- Client signs lease and moves in
- Case manager provides aftercare

Enhancements: page 136

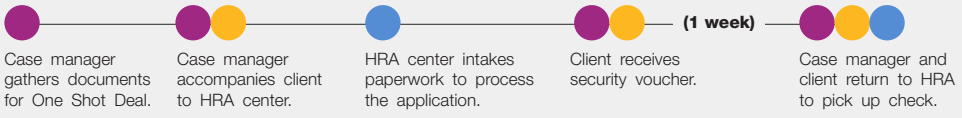
Once a client is placed into permanent housing, the case managers will continue to check in for 90 days to one year. Providers reported that many clients must re-learn key skills and often require long-term care to live independently. Some people will fall out of their housing and need to restart the process again.

Providers report that this stage of the process varies widely for different clients. Providers and DHS report that the process is almost never straightforward and that what is represented here, is an ideal procedure.

Client applies for One Shot Deal

DAYS – WEEKS

For many clients, the initial costs of moving into an apartment, like a security deposit, first and last month's rent or a furniture deposit, are too high to meet without further assistance. To secure those funds, many clients will apply at HRA for a One-Shot Deal, a one-time emergency payment by HRA meant to address these needs.



Tools and Resources

- Status of housing placements is reported to DHS on a daily to weekly basis.

Case managers accompany clients to an HRA Center for this application. Providers report that the experience at the HRA Centers can be stressful for clients. Wait times at the HRA Center are typically long and, sometimes, case managers and clients experience clerical errors that keep them from moving forward with the housing placement, and losing a housing opportunity. (See pages 85-86 for more on HRA centers.)

Stories from the Street

We made a stop at a small park that the outreach team frequents on their daily tours. Three of the people we saw are known to the team and they engage in friendly conversation. The outreach worker explains after that all three have housing and return to the park everyday because of familiarity and habit, sometimes for years. They emphasize how hard it is to "retrain" clients to live independently and take care of themselves.

Journey Map Key

- Potential Client / Client
- DHS Provider
- DHS
- NYC, non-DHS
- State or Fed Office
- Other Provider

- Required step
- Optional step

Client applies for One Shot Deal (continued)

They may have to return multiple times. Providers generally accompany clients to the HRA Center to ensure they don't give up during the long process and submit their application. Providers report that there is no mechanism to escalate these issues and that there is an ongoing need to train HRA staff in how to deal appropriately with this client population.

Client signs lease and moves in

1 – 3 WEEKS

Even at the lease-signing stage, Providers report that a housing opportunity may fall through or get delayed. For new buildings, getting Certificates of Occupancy and other documentation can delay move in by months. In other cases, Providers report that the team can't always find the client to sign their lease.

- Case manager schedules a lease signing with the housing provider and contract signing with HPD.
- The client and case manager attend the lease signing with the housing provider and pick up keys.
- For tenant-based Section 8 subsidy recipients, HPD sends a Rent Breakdown Letter to the client and the housing provider.
- The client moves into their new home with help of case manager.

Tools and Resources

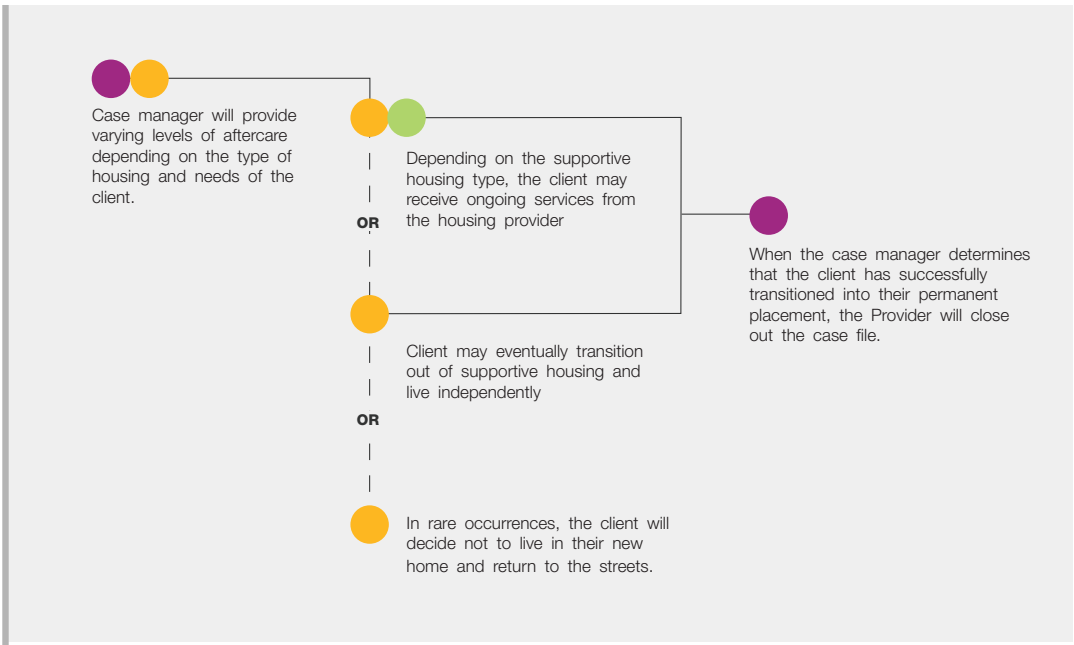
- Lease agreement and Rent Breakdown Letter, if applicable, saved to digital and physical case file
- Status of housing placements is reported to DHS on a daily to weekly basis

Case manager provides aftercare

3 MONTHS – 1 YEAR

Most types of housing for the formerly chronically street homeless require some kind of aftercare. The length of aftercare varies by Provider and is often driven by grant reporting requirements rather than City requirements.

(see journey map on next page)



Once a client is determined to have successfully transitioned into their placement, the Provider will close their case file. Criteria for closing a case file may include up to date rent payments, compliance with medication and a strong relationship with their new case manager or housing provider. This criteria is usually based on the Provider and the case manager's assessment of the client's progress.

Providers report even when permanently housed, some clients may continue to return to the locations they frequented while homeless. Both DHS and Providers shared that it is not uncommon to receive a 311 call about or encounter a former client who is now permanently housed but is perceived to be homeless because they are panhandling or appear to be unoccupied.

For programs like LINC, which has no aftercare, clients will sometimes struggle to live independently and are at risk of falling out of housing and starting the process over again. In these cases, some Providers give extensive aftercare to keep clients permanently housed. For all permanent housing placements, clients will pay their own rent either with personal income and or government assistance.

Supportive housing is designed to help clients live independently. In some cases, these clients will graduate out of the supportive setting and live on their own. For other clients, they will continue to live in a supportive setting where they can receive ongoing care.

Journey Map Key

- Potential Client / Client
- DHS Provider
- DHS
- NYC, non-DHS
- State or Fed Office
- Other Provider

- **Required step**
- **Optional step**

Enhancements

Transitioning into Housing & Aftercare

These enhancements were generated by DHS, Providers, and the Mayor's Office of Operations and include ideas for policy, funding, program design, service delivery, technology and data. They are organized by long-term, medium-term, and short-term timeframes.

LONG-TERM

Provide training for clients to reintroduce them to living back in a home including activities of daily living, money management, and keeping a key.

Next Step: Create a program that will hire staff to assist in training clients before they are housed and once they are placed.

Transition housing all housing providers to a pay for success model (if they are not already using one).

Next Step: Follow-up on current housing provider contract structures and performance incentives.

Create better avenues to banking and financial counseling, similar to the NYCHA Family Self-Sufficiency Program.

Next Step: Follow-up on current state, results and learnings from the NYCHA program.

MEDIUM -TERM

Standardize aftercare across the Providers and programs.

Current range is 90 days to 1 year based on grant reporting requirements rather than City-rewarded incentives.

Next Step: Review aftercare across providers, establish metrics for success and ways to monitor.

Develop a standard set of services for each subset of supportive housing.

For example: All NY/NYIII Category A providers should provide on-site psychiatric and medical services. All Cat E providers should have linkage to substance abuse treatment programs in their community.

Next Step: Conduct an assessment of what helps individuals become successful in supportive housing and explore if funding can stipulate what services the housing providers should deliver.

Create additional programs for clients to enroll or participate in while waiting for housing to become available and after they are placed in permanent housing.

For example: volunteer opportunities, education or training programs, public service opportunities or working for soup kitchens or Providers, etc.

Next Step: Follow-up with NYC Service and outreach Providers on their thoughts.

Areas of Interest

Areas of Interest

Special Populations

Undocumented Immigrants

Criminal Justice-Involved

LGBTQ

Enhancements: page 143

In the chronically homeless population, there are special groups that face unique barriers to engagement and placement in housing.

Undocumented Immigrants

Many Providers engage individuals who are undocumented immigrants. Some of the individuals, especially in the outer boroughs, are day laborers who are living on the street to be close to where they work and not have to travel for early morning shifts. They are often encountered in small group encampments. The encampments are physically dangerous, even if well constructed, due to their heat source and often isolated locations. Providers report there is a sizable portion of the population who use substances, in particular alcohol. It is difficult for Providers to treat their medical and substance use issues. They are also inappropriate for involuntary removals during Code Blue as many are simply on the street waiting to go to work.

"Being undocumented is basically insurmountable."

— Provider

There are very few temporary and permanent housing options for undocumented individuals. Most individuals are resistant to shelter, but some are willing to go to Safe Havens. However, as there are few permanent housing options and they are not eligible for subsidized housing, undocumented individuals can end up staying for long periods in what would normally be a transitional setting as there is nowhere else for them to go. In one instance, DHS reported an undocumented client who applied for housing in 2009 and, because of his undocumented status, has not been placed. Additionally, many undocumented individuals have income but are unable or unwilling to pay full market rent for apartments.

In some cases, Providers are able to assist individuals in applying for permanent residency or naturalization or replacing a Permanent Residency Card or Naturalization Certificate (see pages 73-74).

Criminal Justice-Involved

While the street homeless population is generally non-violent, individuals do get arrested and there is a subset of the population who frequently cycles between jail and the street. The Providers usually find out from others on the street when someone gets incarcerated, but sometimes individuals disappear and Providers do not know where they have gone. The Providers sometimes use the Department of Correction online inmate look up system when they cannot find someone. They have poor communication with Rikers, with the main connection being a phone number that connects to voicemail. Many individuals will go off their medications while incarcerated and be unable to communicate on their own what they need. When ready for release, many homeless individuals are discharged back onto the street; and the opportunity is missed for the Providers to try to place them into a transitional housing setting. Some of the Providers occasionally receive referrals from Rikers.

Incarceration can also cause those who previously had housing to become street homeless. They may have had housing, gone to jail, been able to temporarily stay with a family member or friend, and then go out on the street.

LGBTQ

All Providers serve LGBTQ individuals and have a limited set of transitional and permanent housing placements that serve their needs. DHS Policy allows individuals to go to a shelter consistent with how they identify. NYPD HOU reports that LGBTQ individuals who attempt to seek shelter may be turned away, but they have more success when they go with an officer. The LGBTQ Youth shelter also closes at 10 pm.

Enhancements

Special Populations

These enhancements were generated by DHS, Providers, and the Mayor's Office of Operations and include ideas for policy, funding, program design, service delivery, technology and data. They are organized by long-term, medium-term, and short-term time frames.

LONG-TERM

Implement more fee waivers for citizenship applications and documents.

Next Step: Find out which fees pose the biggest barriers.

Also found in: Gathering Vital Documents

Provide more options and safe places for LGBTQ youth and adults to take shelter and receive comprehensive services.

Next Step: Identify current resources for LGBTQ and determine where they should be expanded.

Also found in: Homeless Contact and Response; Moving onto Caseload

Create a housing subsidy for undocumented New Yorkers, who often have income, but are reluctant to pay fair market rent and are ineligible for supportive or gen-pop housing.

Next Step: Discuss subsidy with MOIA.

Also found in: Applying for Housing

MEDIUM-TERM

Create an alert system that lets Providers know when a client has been arrested or admitted to the hospital.

For arrests, the alert could let Provider's know that the client's name has been added to DOC's inmate lookup database. For HHC hospitals, CARES could pull data from a City source.

Next Step: Explore data sources and possibility of using the Worker Connect Common Client Index.

Also found in: Health Crises; Data and Systems

Enhance immigration services for all Providers.

Next Step: Estimate how many undocumented clients are working with each outreach team. Meet with MOIA to explore options available through the City.

Consider appointing an immigration liaison to assist with specialized barriers for undocumented clients, including assisting with obtaining documentation and funding for fees.

Also found in: Gathering Vital Documents

Work with Rikers for improved discharge planning of known homeless individuals so they do not go back onto the street.

Next Step: Work with DOC to identify a liaison.

Areas of Interest

Cross-Agency Initiatives

Encampment Outreach and Cleanups

Training

Healthcare

Vital Documents, Entitlements, and Housing Applications

New Housing

Case Conference

Enhancements: page 150

DHS and the Outreach Providers work closely with City, State, and Federal agencies and other Providers to serve their client population.

Encampment Outreach and Cleanups

The Providers work closely with NYPD to enter encampments to help with enforcement and ensure safety of the outreach teams. They report very positive outcomes from this collaborative approach.

Depending on the location, encampment cleanups are coordinated across different agencies, including DHS, Providers, NYPD, DSNY, City and State DOT, Parks, and private property owners. The Providers often learn about encampments from staff at the agencies or the Providers bring the encampments to the agencies' attention. They then work together for a clean up strategy and further mitigation to ensure new people do not move in after the clean up. Mitigation can include installing new fences, removing street furniture, scheduling frequent cleaning, or securing doors and openings. Providers report that these strategies are crucial and successful for encampment removal.

Providers like to give clients a few weeks notice that the encampment is going to be cleared, but sometimes agencies come through only a few days later and clear out everyone and their belongings. This can erode trust between clients and the Providers, and has also caused individuals to lose copies of vital documents. They report that this is not frequent, but it does happen.

Training

BRC has done trainings with the NYPD on their protocols, how to do 9.58 removals, expectations of police officers in conducting homeless outreach and BRC services and how to connect individuals to them. They have been successful in taking the trainings to where the police are and have trained the last four graduating classes from the Transit Police Academy. BRC reports that coordination with NYPD has improved following the trainings.

Healthcare

The Providers work closely with first responders (EMS and NYPD) in the case of emergencies or involuntary removals. Additionally, the Providers have found that maintaining good, working relationships with hospitals in their boroughs lends itself to more positive outcomes. However, they expressed a need for more coordination. Many Providers also contract with other Providers for their psychiatric services. See pages 38-51 for Section 3: Health Crises, Hospitals, and Psychiatric Services.

Vital Documents, Entitlements and Housing Applications

The Providers work to gain replacement vital documents from City, State, and Federal agencies (See pages 68-79 for Section 5: Gathering Vital Documents), apply for entitlements and/or document income from City and Federal agencies (see pages 80-95 for Section 6: Applying for Public Assistance and pages 96-99 for Section 7: Securing an Income other than Public Assistance), and apply for housing through City agencies (see pages 100-117 for Section 8: Applying for Permanent Housing).

New Housing

As new housing gets closer to opening, Providers are reliant on City agencies finishing inspections and certifications. They report that a building can often get finished, but will wait months for the Certificate of Occupancy. Any wait in the process is a chance that a client may disappear, get hospitalized or incarcerated, and lose their spot in permanent housing.

Case Conferences

Many Providers case conference across their teams internally to share updates on individuals or strategize for solutions and prioritization. The Providers also sometimes discuss cases with the other Providers for an individual who may be moving between boroughs or the street and subway. They also continue the sharing of information and client status as people more indoors. For example, BRC has a Safe Haven supervisor who has a monthly meeting with the case managers at each Safe Haven to discuss cases and how clients are progressing on permanent housing and any behavioral developments.

DHS and the Providers have also conducted case conferences with other agencies on specific initiatives such as encampment clean ups, with a focus on priority locations and clients. They found that this model did not take off as these clients require a long term commitment and engagement and their issues were not going to get solved in a few meetings.

One strong example is used to coordinate across the subway system. NYPD, MTA, NYCT, DHS, BRC gather for Roundtable meetings where they discuss and plan homeless conditions within each of the Transit Districts. Every station is discussed and review. They have also gathered a wider group for Herald Square area coordination, including Building Managers, PATH, and Port Authority police in addition to the core group.

Enhancements

Cross-Agency Initiatives

These enhancements were generated by DHS, Providers, and the Mayor's Office of Operations and include ideas for policy, funding, program design, service delivery, technology and data. They are organized by long-term, medium-term, and short-term timeframes.

LONG-TERM

Design a Public Assistance process and track just for chronically homeless people to better deliver services where they are and within their needs.

Next Step: Convene HRA, DHS and Providers to create an ideal process flow. Follow with a plan of action.

Also found in: Applying for Public Assistance

Allow all Provider-vouched for, chronic street homeless clients to be on the Homebound Public Assistance track until proven otherwise.

Next Step: Follow-up on the most common reasons that clients are rejected from Homebound Public Assistance.

Also found in: Applying for Public Assistance

Balance the Public Assistance allotment between Safe Haven ($45/month) and street client's ($264/month) to keep from discouraging people to move inside.

Next Step: Follow-up on the factors that affect the allotment decisions for Public Assistance.

Also found in: Applying for Public Assistance

LONG-TERM (continued)

Streamline certificate of occupancy process especially with new sites coming along.

Next Step: Work with City agencies including DOB, HPD, DHS, and others who oversee new buildings to enhance the COO process.

Also found in: Getting Linked with Housing

Consider shifting DHS Outreach unit to HRA as much of their work is social services delivery.

Next Step: Establish a first meeting at the leadership level to begin developing shared goals.

Develop more publicly accessible bathrooms and showers.

Next Step: Look into solutions from other municipalities, such as shower bus in San Francisco.

Also found in: Public Perception and Action

Shorten the statutory time period (currently 45 days for singles) to receive benefits so that clients are not left waiting and can begin the housing application and matching process.

Next Step: Review Public Assistance wait times with HRA.

Also found in: Cross-Agency, Applying for Permanent Housing

MEDIUM-TERM

Create a team with the capability to work with clients who move into abandoned buildings.

Next Step: Explore which units across the City work with abandoned buildings and develop a partnership with outreach providers.

Also found in: Identifying Street Homeless

Create a dedicated overnight outreach team at the Parsons/Archer stop on the E train.

Next Step: Explore capacity of BRC and NYPD HOU to expand outreach at this longterm hotspot.

Also found in: Identifying Street Homeless

Create training elements to fold into existing MTA training and outreach.

Help share success stories, outreach and case management process, and policy and procedures.

Next Step: DHS, BRC, and MTA explore points to disperse information and engage staff.

Also found in: Identifying Street Homeless

Make IDNYC more user-friendly for the street homeless.

Ideas include a mobile application that goes out to homeless, opportunity to get ID without making an appointment, and avenues for clients to get IDs before they may get other photo IDs.

Next Step: Meet with IDNYC team to discuss population needs and programmatic solutions.

Also found in: Gathering Vital Documents; Applying for Public Assistance

MEDIUM-TERM (continued)

Revise the benefit card replacement process so that Provider staff and clients do not get disconnected from HRA's information line nor need to go into an HRA Center to receive one.

Next Step: Determine why the Providers and clients can't use the 311 card replacement service request.

Also found in: Gathering Vital Documents; Applying for Public Assistance

Make HRA centers more friendly.

Allow lunch breaks or snacks, provide food on site, limit the length of time people are required to stay for appointments, and text clients or case managers when their turn comes while waiting.

Next Step: Explore possibilities for improving the user experience. Develop relationships with food vendors.

Also found in: Applying for Public Assistance

Create policy so that one visit to HRA will cover all necessary required steps so clients do not have to go back multiple times.

All HRA centers should have the same procedures and offer the same types of Public Assistance-related services.

Next Step: Convene HRA, DHS and Providers to create an ideal process flow. Follow with a plan of action.

Also found in: Applying for Public Assistance

MEDIUM-TERM (continued)

Make work requirements and substance abuse treatment for Public Assistance more flexible for the street homeless.

It is challenging for those still on the street to keep appointments if they do not have a home, place to shower, or keep clean clothing. HRA should consider a suspension until a client is housed. Providers report that current requirements undermine the relationship between caseworker and client.

Next Step: Review work requirements with HRA.

Also found in: Applying for Public Assistance

Tie the Public Assistance and Housing application processes more closely together.

This could help avoid situations where Public Assistance is turned off for clients in the middle of their housing application process.

Next Step: Establish a first meeting at the leadership level to begin developing shared goals.

Also found in: Applying for Public Assistance; Applying for Permanent Housing

Extend the 2010e application/packet expiration date to at least one year.

Next Step: Meet with PACT on updates to 2010e process. Follow up on which policies govern the expiration of 2010e packets.

Also found in: Applying for Permanent Housing

MEDIUM-TERM (continued)

Base housing placements on mental and physical condition of client on application date, and consider services and supports they have needed to achieve an improved state by expiration date.

Next Step: Meet with PACT on updates to 2010e process.

Also found in: Applying for Permanent Housing

Increase communication and access between Providers and the PACT team to get clients into appropriate housing.

Next Step: Establish a first meeting at the leadership level to begin developing shared goals.

Also found in: Applying for Permanent Housing

Allow Providers to update an expired or expiring housing packet without the risk that the client's housing designation with change to an inappropriate designation.

Next Step: Establish a first meeting at the leadership level to begin developing shared goals.

Also found in: Applying for Permanent Housing

Allow for HRA PACT and Providers to case conference around housing packets.

Next Step: Research current PACT workflow to understand the potential impact on current process.

Also found in: Applying for Permanent Housing

MEDIUM-TERM (continued)

Reduce barriers to Section 8 briefing.

Streamline the process and implement in small steps. Consider virtual briefings (e.g. Skype).

Next Step: Meet with HPD on updates to briefings process.

Also found in: Getting Linked with Housing

Find a liaison at DOT on encampment issues, confirming jurisdiction of property lines.

Next Step: Work with DOT to identify a liaison.

Hire a DHS Data Manager who will oversee all internal reporting mechanisms and respond to any internal or external inquiries for data.

They will also serve as the liaison on data with DHS OIT, Mayor's Office of Operations, City Hall, and Department of Social Services.

Next Step: Work with HR to develop Data Manager role.

Also found in: Data and Systems

SHORT -TERM

Expand encampment definition to include just one individual.

For example, homeless man's encampment between highway and soccer field in Queens.

Next Step: Review encampment definitions to be shared across all parties.

Also found in: Identifying Street Homeless

Provide parking permits with more expansive options so that outreach teams can pull over where needed to interact with individuals.

Next Step: Contact DOT about enhanced permits.

Also found in: Identifying Street Homeless

Appoint someone to own relationship between Providers, NYPD, and EMS. Every case to have an assigned executive leader.

Next Step: Identify a key contact for NYPD and EMS. Providers mention that Louis Molina is a helpful contact to escalate issues when NYPD is unresponsive.

Also found in: Health Crises

Host an "Outreach Team Roadshow" to educate internal DHS groups (especially shelters), Safe Havens and other key stakeholders about the Street Homeless team and provider's services.

Next Step: Develop communication materials.

Also found in: Moving onto Caseload

SHORT -TERM (continued)

Assign one HRA Center in each borough that serves homeless individuals.

Next Step: Identify best centers per borough with HRA.

Also found in: Applying for Public Assistance

Implement a protocol for DHS Analysts to contact HRA when there are issues with Public Assistance or housing applications.

Next Step: Create new position at DHS to manage new protocol and identify key HRA contacts for Public Assistance and 2010e issues.

Also found in: Applying for Public Assistance

Designate HRA Center Supervisors or leadership as key contacts for Provider staff and leadership to escalate issues and give feedback.

Next Step: Identify staff contacts from Providers and HRA.

Also found in: Applying for Public Assistance

Areas of Interest

Data and Systems

CARES

Accessing Vital Documents in CARES

CARES Reporting

Provider Systems

Provider Reporting

Technology in the Field

Enhancements: page 167

Data and case management systems are central to the work of the Outreach Providers and DHS team. There are a multitude of systems and practices in place, with little standardization across them. The systems largely track the outputs of first client engagement through placement in permanent housing, but not the input of the newly homeless.

CARES

The Client Assistance and Re-Housing Enterprise System (CARES), is the centralized case management system for the Department of Homeless Services. Its primary function is for shelter bed and sheltered client case management. The Street Outreach Unit has limited functionality.

"CARES is a nightmare."

— Provider

Providers are contractually mandated to enter each contact they have with a street or subway homeless individual into the system. They use the "business load" document to enter as much known data as possible including first and last name, date of birth, reason for homelessness, gender, race, and assessment of income, employment, mental health, etc. They do not log any case notes into CARES about Outreach clients due to the short character limit. Instead, each provider uses their own systems as their system of record for case management.

Both Providers and DHS expressed that they do not feel CARES has good data integrity as it is not the system of record for Providers and also easily allows duplicates. A few reasons duplicates occur include: the names of new individuals are frequently unknown; CARES does not allow names to be updated once they are known; and individuals who may be moving around or interacting with different outreach shifts or Providers.

Providers expressed frustration with using CARES as it frequently logs them out, freezes, and is difficult to navigate. They also report that there are many defects in the system that prevent them from accessing information, such as an inability to print full history for a client rather than just a few days worth. It is also frequently down overnight, when outreach teams are very active.

Usage of drop-in centers and respite beds is not tracked in CARES and uses a separate web-based system which the Providers report into daily.

Accessing Vital Documents in CARES

"CARES doesn't do us a lot of good."
— Provider

DHS reported that, once a client's benefits application is processed, CARES is supposed to allow the Agency to generate a Public Assistance budget letter via the CARES link to the State Welfare Management System (WMS). DHS Analysts report that the WMS link in CARES is unreliable and usually does not reflect the latest information in WMS. DHS reports HRA does not accept a budget letter generated via CARES as official documentation.

CARES Reporting

"We are tracking so much that it can't be meaningful."
— DHS Analyst

In many cases, DHS asks Providers directly for data or information. DHS views outreach data in CARES as more quantitative counts and the Provider systems for qualitative information. While CARES does have reporting functionality, DHS analysts express frustration about its rigidity, difficulty in extracting data, and inability for ad hoc reporting. Providers are unable to extract data or run reports from CARES.

Catalog of DHS Reports

	Program	Frequency	**Title**	Description	Source	Format	Current Status	Future Status
1	All	Monthly	**Mayor's Management Report (MMR)**	Various data	Various	Excel	Active	Active
2	All	Weekly	**110 Day Report**	Client cases about to close due to 120 days inactivity	CARES	iReport	Active	Active
3	Drop-In	Daily	**Drop-In and Respite Bed**	Census	Providers	Database	Active	Active
4	Drop-In	Date Range	**DropIn_Placement_ Detail**	Placements entered in CARES	CARES	iReport	Active	Active
5	Drop-In	Monthly	**Client Enrollment Status**	Long-Term Stayers	Providers	Excel	Active	CARES iReport
6	Drop-In	Monthly	**DHS Monthly Report**	Census, Demographics	Providers	Excel	Active	CARES iReport

Catalog of DHS Reports (continued)

	Program	Frequency	**Title**	Description	Source	Format	Current Status	Future Status
7	Drop-In	Monthly	**Fire Drill Report**	Fire Drill	Providers	PDF	Active	TBD
8	Drop-In	Monthly	**Staff List**	List of current staff	Providers	Word	Active	TBD
9	Outreach	Daily	**Contact Brief Services**	Contacts/Brief Services Data Entry Records	CARES	iReport	Active	Active
10	Outreach	Daily	**Daily_Indicators**	Outreach Indicators	CARES	iReport	Active	Active
11	Outreach	Date Range	**Caseload_Details**		CARES	iReport	Active	Active
12	Outreach	Date Range	**Caseload_Summary**		CARES	iReport	Active	Active
13	Outreach	Date Range	**Medical_Vulnerable**		CARES	iReport	Active	Active
14	Outreach	Date Range	**Placement_Details**	Placements entered in CARES	CARES	iReport	Active	Active
15	Outreach	Monthly	**Monthly 311 Callbacks Month YYYY**	311 Reports	Providers	Excel	Active	TBD
16	Outreach	Monthly	**Monthly Encampment Month YYYY**	Encampments	Providers	Excel	Active	CARES
17	Outreach	Monthly	**Monthly Stabilization Bed Month YYYY**	Stabilization Bed Client Data	Providers	Excel	Active	TBD
18	Outreach	Monthly	**Stabilization Beds**	Stabilization Bed Data and Tracking	Providers (Internal Tracking)	Excel	Active	TBD
19	Outreach	Weekly	**DOHMH**	Client cases without DOHMH indicators	SQL	Excel	Active	Active

Catalog of DHS Reports (continued)

	Program	Frequency	**Title**	Description	Source	Format	Current Status	Future Status
20	Outreach	Weekly	**NYC Outreach and Housing Placement Program Weekly Statistics_m.d.yyyy-m.d.yyyy**	Weekly Placements, Year to Year Comparison	Manual	Excel	Active	TBD
21	Safe Haven	Annually	**RSRI**	Routine Site Review Inspection	FMD	PDF	Active	Active
22	Safe Haven	Annually	**Safe Haven Monitoring Instrument**	Annual site inspection and review	Analyst	Word	Active	TBD
23	Safe Haven	Daily	**Month YYYY DHS Safe Haven Daily Stats**	Daily Census	Providers and IVC	Excel	Active	CARES
24	Safe Haven	Monthly	**Monthly Safe Haven Report yyyy_mm**	Census, Placements, Demographics	Eric Koyama/ Policy and Planning	Excel	Active	Active
25	Safe Haven	Weekly	**Offsite Checked In**	Roster data - client dates of offsite checked in	SQL	Excel	Active	Active
26	Outreach	Daily	**DHS Outreach Daily Report**	Clients, Contacts, Placements, Referrals	Providers	Access	Inactive	CARES iReport
27	Outreach	Monthly	**100K Homes**	Placements	Providers	Email	Inactive	Inactive
28	Outreach	Monthly	**Monthly Indicators FYyy**	Caseload Data	Providers (Internal Tracking)	Excel	Inactive	Caseload Summary Rerport
29	Outreach	Monthly	**Monthly Caseload Data Month Year**	Caseload Data	Providers	Excel	Inactive	Caseload Summary Report

Provider Systems

While Providers enter data into CARES, they also maintain their own systems, which they consider their systems of record. Many of the Provider systems have databases that go back further than when CARES launched in 2010. In all cases, the Providers do double data entry. For Breaking Ground's subcontract to MOC to do outreach in midtown Manhattan, they do triple data entry into CARES, CDB (the Breaking Ground database), and MOCapp (the MOC app). The Providers also store information that is not included in CARES including client photos and hotspot information. The MOCapp includes a "hotspot view" which shows details, location, and photos of the hotspots.

	MOC	BW	BRC	BG	PH
Data Sources	MOCapp (web-based app)	Excel, CARES (contacts), AWARDS (Db for Health Homes only)	AWARDS (Db - caseload services), TOURS (Db-outreach, visits), Excel, Paper (face sheets - contact data), CARES (contacts, pre-caseload), NexTraq (fleet)	CDB (Access Db), MOCApp, Excel (Housing Status), CARES,	Excel, Paper (physical binder)
Coverage	Manhattan	Bronx	Subway	Brooklyn/ Queens	Staten Island

Provider Reporting

"We send an Excel to DHS monthly but not sure what they do with it."

— Provider

Some of the Providers are using performance management techniques and tools to measure the success of their programs. For example, Breaking Ground and BRC set overall program goals and individual worker goals and then measure against them. Breaking Ground has developed resources such as an "assembly line" Excel sheet that illustrates where all of their clients are in the housing process. BRC works with their data manager to clean and test the data, and then has their program teams report back quarterly.

DHS makes frequent reporting requests from the Providers, including daily reports during Code Blue, bi-weekly locations and people of interest and monthly 311 logs and stabilization beds. DHS sometimes repeats requests for data that the Providers have already sent.

Technology in the Field

Several of the Providers send technology, including tablets and laptops, out into the field for outreach teams to reference case or location information. They have tried case note entry in the field, but due to the unreliability of the technology, Internet connection, and nature of the work, they find that they do not get good data entry. Many workers also expressed hesitation to use any technology while engaging with homeless individuals as it can be alienating. Most outreach teams take notes on small notepads or paper while with individuals, or immediately after speaking with them, and then log full case notes at the end of their shift at the office. Other workers take notes on their personal phones and then transcribe them into case notes later on. See page 29 for more information on keeping track of individuals.

Enhancements

Data and Systems

These enhancements were generated by DHS, Providers, and the Mayor's Office of Operations and include ideas for policy, funding, program design, service delivery, technology and data. They are organized by long-term, medium-term, and short-term timeframes.

LONG-TERM

Unify provider databases with DHS data system so there is a centralized portal with information across Providers and includes information on client, their location, vulnerability, entitlements, documents, and hospital status.

Historical and real-time. "If this is the only thing that comes out of this meeting, everyone will be happy."

Next Step: Develop catalog of data and systems in use. Work with Providers, DHS business, and DHS IT on a plan for a centralized solution.

Make enhancements to the CARES system.

- Allow changes to fields (e.g. to correct misspelled names).
- Allow privileges.
- Allow Providers to consolidate duplicate entries.
- Eliminate the character limit for case notes and other fields.
- Develop the ability to print history and other reports.
- Allow for wildcard search as part of client search function.
- Display which Provider Outreach team the homeless person is working with. Currently, the field just says "outreach".
- Add stabilization beds.
- Track the progress of clients' housing packets.
- Allow import function to eliminate Provider double data entry.

Next Step: Review updates with DHS IT and DHS Business

LONG-TERM (continued)

Explore developing a mobile tool for outreach teams to use in their vehicles to add notes and reference client information.

Providers have expressed that they do not like using tech directly with clients, as it can be distancing, and their work is focused on building relationships. This will require strong internet access in the field.

Next Step: Work with Providers, DHS business, and DHS IT to develop requirements. Prototype solutions.

Also found in: Homeless Contact and Response

Create a portal showing housing available in the City by category, including drop-in and respite bed tracking.

It would provide all details and regulations about a housing opportunity and include photos. Providers would use the portal to showcase opportunities to clients and have them be fully informed before attending an interview. The housing options should promote themselves to clients in hopes of clients selecting them to be their transitional or permanent home.

Next Step: Convene housing providers. Prototype layouts for housing information.

Also found in: Moving onto Caseload; Finding Housing Opportunities

Create a portal showing programs available in the City (like detox) to help Providers make better recommendations to their clients.

Next Step: Locate all available programs in the City.

Also found in: Moving onto Caseload

LONG-TERM (continued)

Create a resource guide/database to the different processes and policies for entitlements and housing.

Be able to track types of inquiries for understanding where pain points are.

Next Step: Work with Providers, DHS, and HRA business to identify top questions and needs.

Also found in: Applying for Public Assistance; Applying for Permanent Housing

Explore further reporting mechanisms in HHS Accelerator, including tracking budgets over time.

Next Step: Develop HHS Accelerator reporting needs with DHS and Accelerator team.

Transition housing and housing subsidy applications currently on paper to digital with the option to pre-populate information from other City data systems.

Next Step: Create a working group with HPD IT to begin plan of action.

Also found in: Data and Systems

MEDIUM -TERM

Update the 311 app to prompt users to add more detailed descriptions of the individual in need and enter a location without knowing the exact address.

For example, create specific prompt language or more special fields to indicate gender, race, height, weight, clothes, subway line and station. Allow for a dragging pin on the map and searches by subway station.

Next Step: HOME-STAT canvassing team working with 311.

Also found in: Identifying Street Homeless; Public Perception and Action

Potentially at HHC hospitals and also health homes, provide a liaison who can access case notes about an individual.

Emergency rooms often turn people away before they can get full information.

Next Step: Look into CARES and Worker Connect use cases for HHC serving homeless individuals

Also found in: Health Crises

Create an alert system that lets Providers know when a client has been arrested or admitted to the hospital.

For arrests, the alert could let Provider's know that the client's name has been added to DOC's inmate lookup database. For HHC hospitals, CARES could pull data from a City source.

Next Step: Explore data sources and possibility of using the Worker Connect Common Client Index.

Also found in: Health Crises; Special Populations

MEDIUM -TERM (continued)

Refresh the vulnerability index across all teams to inform prioritization for housing.

It should be consistent across teams and take into account the non-chronically homeless.

Next Step: Collect all vulnerability indices and prioritization processes from teams; determine which groups to target for housing.

Also found in: Moving onto Caseload

Publish transparent data about housing providers' client acceptances and rejections for permanent housing.

Create metrics and goals for outreach Providers based on those housing provider rates.

Next Step: Source Provider acceptance and rejection data.

Also found in: Getting Linked with Housing

Define a set of metrics to measure progress and performance across all Providers.

Variables could include:

- number of housing packages in process, including tracking all required elements
- number of placements from street to transitional housing
- number of people placed in permanent housing and maintained for 2 years (currently 1 year)

Next Step: Review current metrics and data points being tracked by Providers and DHS to find commonalities and create an agreed upon set of metrics. Decide on where and how they will be tracked.

MEDIUM -TERM (continued)

Hire a DHS Data Manager who will oversee all internal reporting mechanisms and respond to any internal or external inquiries for data.

They will also serve as the liaison on data with DHS OIT, Mayor's Office of Operations, City Hall, and Department of Social Services.

Next Step: Work with HR to develop Data Manager role.

Also found in: Cross-Agency Initiatives

SHORT -TERM

Give Providers access to number of deceased homeless individuals.

Next Step: Confirm existing data sharing channels between DHS and DOHMH.

Also found in: Health Crises

Publish transparent information online about the current state of the SPOA Assertive Community Treatment and Care Coordination.

Currently, the NYC.gov page links to a Center for Community Services page that states they no longer run the program and links back to the NYC.gov page.

Next Step: Find out who is responsible for managing ACT and Care Coordination services in New York City.

Also found in: Health Crises

SHORT -TERM

Develop common tracking terms for defining clients and outcomes.

Next Step: Identify all current definitions and propose a consolidated and shared set.

Remove filter in Worker Connect that is blocking Outreach information from the CARES data.

Next Step: Review legal history on CARES and Worker Connect Integration.

Promote Worker Connect as a resource and conduct a training for the DHS Outreach team.

Next Step: Review provisioned users and schedule a training.

Create a map of 311 requests for homeless assistance, locations of known clients and staff overlay for each area.

Next Step: Export "contact" data from CARES, gather detailed staffing plan from Providers, export 311 log data and compile on a map.

Also found in: Public Perception and Action

Area of Interest

Public Perception and Action

Community Engagement

Press and Media

Public Action

Enhancements: page 177

While homeless canvassing and outreach efforts are conducted in public, it is largely invisible to most New Yorkers and there are many misconceptions about the homeless population.

Community Engagement

Providers interact with numerous community stakeholders including Community Boards,libraries, precincts, BIDs, elected officials, and religious institutions.

For example, Providers interact with Community Boards both to find out about new individuals, and to communicate about the services they provide and discuss particular individuals. One Provider shared that they visited a Community Board to urge them to ask residents to stop receiving mail on behalf of an individual as it was helping to keep him outside.

DHS says that helping well-known people on the street does a lot for messaging and good will in communities as they can see the effectiveness of outreach services.

Members of the public frequently engage with the outreach teams when they are out on the street and in the subway system to provide information, complain about individuals, inquire about services, and ask questions.

Some of the engagements observed by the research team while shadowing the outreach teams include:

- A taxi driver who parks his cab near where a homeless man was staying approached the outreach team to ask questions and offer information. He said that the homeless man had threatened him and was sometimes sleeping in an abandoned car.
- A business owner who said the homeless man the outreach team was engaging was a long time neighborhood nuisance and was defecating in front of the local school.
- A woman who wanted to make a donation to the Provider.
- A woman who wanted to find out about services and what could be done for homeless on Staten Island.

Community Engagement (continued)

- A woman filming the outreach team on her cell phone for hours. The outreach team said that she uploads them onto YouTube and says the City is not doing anything to help the homeless.

See pages 27-28 for more information on appearance and presence of outreach teams.

Press and Media

The news media can play both a helpful and harmful role. The press often violates the privacy and respect of homeless individuals, or reports that individuals are homeless when they are not. However, there have been good pieces that highlight the outreach teams and other programs like Safe Havens. The Providers stressed that the story needs to get out and the public needs to know what the City is doing to address homelessness and more about the services available, including funding and locations.

Public Action

All Providers urge that the public should not give money, food, or other items to homeless individuals as it makes it harder for the outreach teams to entice them to come inside. The more comfortable an individual feels in a neighborhood, the less likely they are to be willing to receive services, and they can also start to blend in, with the community failing to see the medical and psychiatric needs of the individual.

Providers recommend that the public call 311 for homeless assistance and give as much description as possible of the individual including their exact location, time observed, and description of their demographics, appearance, and behavior. See pages 15-16 for more on 311 response.

Enhancements

Public Perception and Action

These enhancements were generated by DHS, Providers, and the Mayor's Office of Operations and include ideas for policy, funding, program design, service delivery, technology and data. They are organized by long-term, medium-term, and short-term timeframes.

LONG-TERM

Develop more publicly accessible bathrooms and showers.

Next Step: Look into solutions from other municipalities, such as shower bus in San Francisco.

Also found in: Cross-Agency Initiatives

MEDIUM -TERM

Update the 311 app to prompt users to add more detailed descriptions of the individual in need and enter a location without knowing the exact address.

For example, create specific prompt language or more special fields to indicate gender, race, height, weight, clothes, subway line and station. Allow for a dragging pin on the map and searches by subway station.

Next Step: HOME-STAT canvassing team working with 311.

Also found in: Identifying Street Homeless; Data and Systems

Create a new DHS Outreach staff member position to streamline communication of media pieces and rapid response to the public.

Next Step: Explore creating a new position so the DHS analysts spend less time on media-related matters.

MEDIUM -TERM (continued)

Provide more staffing at HRA centers. HRA to also consider staffing up during busy time periods.

Next Step: HRA to develop plan to increase staff and analyze current flow at job centers.

Also found in: Cross-Agency Initiatives

Create policy so that one visit to HRA will cover all necessary required steps so clients do not have to go back multiple times.

All HRA centers should have the same procedures and offer the same types of Public Assistance-related services.

Next Step: Convene HRA, DHS and Providers to create an ideal process flow. Follow with a plan of action.

Also found in: Cross-Agency Initiatives

SHORT -TERM

Create a map of 311 requests for homeless assistance, locations of known clients and staff overlay for each area.

Next Step: Export "contact" data from CARES, gather detailed staffing plan from Providers, export 311 log data and compile on a map.

Also found in: Data and Systems

Post signs in the subway platforms and cars letting New Yorkers know what they can do to help the homeless.

Next Step: Follow-up with Providers and MTA to get their thoughts.

SHORT -TERM (continued)

Increase public awareness through a digital media series that surfaces the complexities of the issues and highlights what the City is doing and what New Yorkers can do to help.

Understanding trends that lead to chronicity. The 2013 NYT Dasani series sparked a public conversation about homelessness and this could mirror that approach.

Next Step: Build media elements into public-facing HOME-STAT dashboards. Explore a wider campaign and press pieces.

Potential Enhancements

The stakeholder insights research echoes the need for the initial HOME-STAT initiative, including accuracy and transparency in data coupled with individualized case management. This report offers additional potential enhancements that would further improve the conditions for everyone on the street and the City Agencies and Providers who serve them.

The enhancements were generated through interviews and workshops by DHS, Providers, and the Mayor's Office of Operations. They include ideas for policy, funding, program design, service delivery, technology, and data. They are organized by long-term, medium-term, and short-term timeframes. Each enhancement should be reviewed for feasibility and impact to create a final recommendation set and service design plan that will enhance service delivery for homeless New Yorkers.

LONG-TERM

Create a new type of center for Providers to bring individuals for assessment.

Provide beds for 40-50 people and a 5-7 day assessment period for psychiatric screening, medical evaluation, and eligibility determination. Then create a plan for the client that is most appropriate for their circumstances which will put them in a transitional bed that keeps them engaged and moved in the direction towards permanent housing, rather than resisting their placement.

Next Step: Design new program requirements and plan.

Homeless Contact and Response

Explore developing a mobile tool for outreach teams to use in their vehicles to add notes and reference client information.

Providers have expressed that they do not like using tech directly with clients, as it can be distancing, and their work is focused on building relationships. This will require strong internet access in the field.

Next Step: Work with Providers, DHS business, and DHS IT to develop requirements. Prototype solutions.

Homeless Contact and Response; Data and Systems

Develop a series of hubs (mental health, medical, substance abuse) that have a relationship with the Providers and can be used as an alternative.

For example, hospitals usually do not want someone who is going through withdrawal and will also try to discharge them before they go into withdrawal, but they should not go back onto the street.

Next Step: Look into existing City “hub” programs and pull insights from existing wet shelters operating successfully.

Health Crises

LONG-TERM (continued)

Allow Providers to more easily gain guardianship for those hurting themselves medically.

Next Step: Explore existing guardianship laws and specific cases where guardianship may have been beneficial.

Health Crises

Create a track for street homeless individuals with Adult Protective Services (APS).

Next Step: Meet with DHS, APS, and Providers to discuss cases that could use protective services and how to best create a track for them.

Health Crises

Improve shelter conditions to make them more friendly to the chronically street homeless who have better outcomes in smaller settings.

Specific goals should be:

- reduce burglary
- fewer large spaces
- improve staff service delivery
- reduce overcrowding
- consider more programs with no more than 60 beds

Next Step: Gather leadership for a review of existing shelters that are good settings for chronic population.

Moving onto Caseload

LONG-TERM (continued)

Improve services and discharge planning for youth turning 21 in shelter so that they do not end up on the street and become chronically homeless.

Next Step: Review current youth shelter discharge protocol.

Moving onto Caseload

Provide better accessibility in Safe Havens for disabled individuals.

Next Step: Identify stakeholders for reviewing building accessibility and potential modifications.

Moving onto Caseload

Create transitional and permanent housing solutions for individuals with pets.

Explore with new Safe haven sites.

Next Step: Look into new pet-friendly "navigation center" in San Francisco.

Moving onto Caseload; Applying for Housing

Implement wet shelter model to provide services to inebriates and street homeless with substance abuse issues who are otherwise service-resistant.

Next Step: Research other municipalities with wet shelters.

Moving onto Caseload

LONG-TERM (continued)

Create a portal showing housing available in the City by category, including drop-in and respite bed tracking.

It would provide all details and regulations about a housing opportunity and include photos. Providers would use the portal to showcase opportunities to clients and have them be fully informed before attending an interview. The housing options should promote themselves to clients in hopes of clients selecting them to be their transitional or permanent home.

Next Step: Convene housing providers. Prototype layouts for housing information.

Moving onto Caseload; Finding Housing Opportunities; Data and Systems

Create a portal showing programs available in the City (like detox) to help Providers make better recommendations to their clients.

Next Step: Locate all available programs in the City.

Moving onto Caseload; Data and Systems

Implement more fee waivers for citizenship applications and documents.

Next Step: Find out which fees pose the biggest barriers.

Gathering Vital Documents; Special Populations

LONG-TERM (continued)

Allow required and validated documents from a City agency, such as HRA, stand in place as acceptable documents for another agency, such as HPD or DHS.

Next Step: Convene a working group to develop a plan of action.

Applying for Housing

Provide more options and safe places for LGBTQ youth and adults to take shelter and receive comprehensive services.

Next Step: Identify current resources for LGBTQ and determine where they should be expanded.

Homeless Contact and Response; Moving onto Caseload; Special Populations

Create a housing subsidy for undocumented New Yorkers, who often have income, but are reluctant to pay fair market rent and are ineligible for supportive or gen-pop housing.

Next Step: Discuss subsidy with MOIA.

Special Populations

Design a Public Assistance process and track just for chronically homeless people to better deliver services where they are and within their needs.

Next Step: Convene HRA, DHS and Providers to create an ideal process flow. Follow with a plan of action.

Applying for Public Assistance; Cross-Agency Initiatives

LONG-TERM (continued)

Allow all Provider-vouched for, chronic street homeless clients to be on the Homebound Public Assistance track until proven otherwise.

Next Step: Follow-up on the most common reasons that clients are rejected from Homebound Public Assistance.

Applying for Public Assistance; Cross-Agency Initiatives

Balance the Public Assistance allotment between Safe Haven ($45/month) and street client's ($264/month) to keep from discouraging people to move inside.

Next Step: Follow-up on the factors that affect the allotment decisions for Public Assistance.

Applying for Public Assistance; Cross-Agency Initiatives

Create a resource guide/database to the different processes and policies for entitlements and housing.

Be able to track types of inquiries for understanding where pain points are.

Next Step: Work with Providers, DHS, and HRA business to identify top questions and needs.

Applying for Public Assistance; Applying for Permanent Housing; Data and Systems

Providers expressed a strong need for more housing so that clients do not have to wait as long to get matched with permanent housing.

Next Step: Review past housing applications to establish which populations need the most new units.

Applying for Permanent Housing

LONG-TERM (continued)

In new housing agreements (potential NY/NY4), ensure that breakdown of units allocated for a specific category (ie Cat A, Cat E) matches the need of the actual population looking for housing (ie enough Cat E units).

Next Step: Review past housing applications to establish which populations need the most new units and work with HPD's supportive housing task force to streamline the selection process.

Applying for Permanent Housing

Shorten the statutory time period (currently 45 days for singles) to receive benefits so that clients are not left waiting and can begin the housing application and matching process.

Next Step: Review Public Assistance wait times with HRA.

Also found in: Cross-Agency, Applying for Permanent Housing

Give Providers the ability to place clients in housing outside of New York City, like in Long Island or Westchester.

Next Step: Look into feasibility of placements and transitioning care to Providers in potential areas outside of NYC using city subsidies.

Getting Linked with Housing

LONG-TERM (continued)

Change the dynamic between housing providers and clients to be more client-focused.

Process should be less about housing provider accepting the client and more about the client accepting the placement.

Next Step: Establish a first meeting at the leadership level to begin developing shared goals between DHS and other agencies managing the housing providers.

Getting Linked with Housing

Conduct research on the history of how landlords are choosing which housing subsidies to accept from clients.

Next Step: Meet with HPD to discuss who can lead this research.

Streamline Certificate of Occupancy process especially with new sites coming along.

Next Step: Work with City agencies including DOB, HPD, DHS, and others who oversee new buildings to enhance the COO process.

Getting Linked with Housing; Cross Agency Initiatives

Provide training for clients to reintroduce them to living back in a home including activities of daily living, money management, and keeping a key.

Next Step: Create a program that will hire staff to assist in training clients before they are housed and once they are placed.

Transitioning into Housing and Aftercare

LONG-TERM (continued)

Transition housing all housing providers to a pay for success model (if they are not already using one).

Next Step: Follow-up on current housing provider contract structures and performance incentives.

Transitioning into Housing and Aftercare

Create better avenues to banking and financial counseling, similar to the NYCHA Family Self-Sufficiency Program.

Next Step: Follow-up on current state, results and learnings from the NYCHA program.

Transitioning into Housing and Aftercare

Consider shifting DHS Outreach unit to HRA as much of their work is social services delivery.

Next Step: Establish a first meeting at the leadership level to begin developing shared goals.

Cross-Agency Initiatives

Develop more publicly accessible bathrooms and showers.

Next Step: Look into solutions from other municipalities, such as shower bus in San Francisco.

Cross-Agency Initiatives; Public Perception and Action

LONG-TERM (continued)

Explore further reporting mechanisms in HHS Accelerator, including tracking budgets over time.

Next Step: Develop HHS Accelerator reporting needs with DHS and Accelerator team.

Data and Systems

Transition housing and housing subsidy applications currently on paper to digital with the option to pre-populate information from other City data systems.

Next Step: Create a working group with HPD IT to begin plan of action.

Applying for Housing and Data and Systems

Unify provider databases with DHS data system so there is a centralized portal with information across Providers and includes information on client, their location, vulnerability, entitlements, documents, and hospital status.

Historical and real-time. "If this is the only thing that comes out of this meeting, everyone will be happy."

Next Step: Develop catalog of data and systems in use. Work with Providers, DHS business, and DHS IT on a plan for a centralized solution.

Data and Systems

LONG-TERM (continued)

Make enhancements to the CARES system.

- Allow changes to fields (e.g. to correct misspelled names).
- Allow privileges.
- Allow Providers to consolidate duplicate entries.
- Eliminate the character limit for case notes and other fields.
- Develop the ability to print history and other reports.
- Allow for wildcard search as part of client search function.
- Display which Provider Outreach team the homeless person is working with. Currently, the field just says "outreach".
- Add stabilization beds.
- Track the progress of clients' housing packets.
- Allow import function to eliminate Provider double data entry.

Next Step: Review updates with DHS IT and DHS Business

Data and Systems

MEDIUM -TERM

Supply additional translation services / language line to the outreach teams.

The biggest needs are for Mandarin, Creole, and the hearing impaired.

Next Step: Identify which teams need additional services, explore services available through the City, or look into hiring staff who speak the languages.

Identifying Street Homeless

MEDIUM -TERM (continued)

Revise the MTA rules and rights to enforce loitering.

The subway is sheltering a large number of people who are resistant to leave and enforcement would help along with outreach. Also look into policies in other municipalities that limit people taking the trains back and forth.

Next Step: Review policies with MTA leadership.

Identifying Street Homeless

Revise current practice of distributing funds based on HOPE counts.

Next Step: Explore alternative ways to allocate funding.

Identifying Street Homeless

More funding to increase number of staff, to retain staff, to train staff, and increase office space.

Next Step: Review current budgets and funding.

Identifying Street Homeless

Expand canvass approach to non-public spaces (i.e. hospitals, libraries).

Next Step: Explore best routes for getting reports from hospitals and libraries or having City workers canvass key locations.

Identifying Street Homeless

MEDIUM -TERM (continued)

Utilize credible messengers by bringing on formerly chronically homeless to engage resistant clients.

Next Step: Learn more about the BronxWorks peer program with a similar model. Look to some existing criminal justice projects that use credible messengers.

Identifying Street Homeless; Homeless Contact and Response

Formally acknowledge the efforts of individuals who were formerly chronically street homeless and now act as "credible messengers" and are encouraging currently homeless individuals to seek out help and services.

Next Step: Recruit credible messengers to be part of the peer program (see previous enhancement).

Identifying Street Homeless; Homeless Contact and Response

Provide more staffing at HRA centers. HRA to also consider staffing up during busy time periods.

Next Step: HRA to develop plan to increase staff and analyze current flow at job centers.

Cross-Agency Initiatives

Create better transportation options for clients to transitional housing or service providers.

Allow clients to take city-funded transportation, like Access-a-Ride or create positions for designated drivers just for these clients. This would maximize the time of Providers to work with more clients and do other work.

Next Step: Review transportation options and City services available.

Identifying Street Homeless; Moving onto Caseload

MEDIUM -TERM (continued)

Create a dedicated overnight outreach team at the Parsons/Archer stop on the E train.

Next Step: Explore capacity of BRC and NYPD HOU to expand outreach at this longterm hotspot.

Identifying Street Homeless; Cross-Agency Initiatives

Create training elements to fold into existing MTA training and outreach.

Help share success stories, outreach and case management process, and policy and procedures.

Next Step: DHS, BRC, and MTA explore points to disperse information and engage staff.

Identifying Street Homeless; Moving onto Caseload; Cross-Agency Initiatives

Create a team with the capability to work with clients who move into abandoned buildings.

Next Step: Explore which units across the City work with abandoned buildings and develop a partnership with outreach providers.

Identifying Street Homeless; Cross-Agency Initiatives

Update the 311 app to prompt users to add more detailed descriptions of the individual in need.

For example, create specific prompt language or more special fields to indicate gender, race, height, weight, clothes, subway line and station.

Next Step: HOME-STAT canvassing team working with 311.

Identifying Street Homeless; Data and Systems; Public Perception and Action

MEDIUM -TERM (continued)

Provide overarching training to outreach workers and case managers.

There should be a shared understanding of what's available and where it is, as well as the goals and expectations of programs and resources. The outreach approaches should remain unique to the Providers and the populations they serve.

Next Step: Review prior resources such as Coalition for the Homeless' "Homeless Outreach Resource" and training currently given by Providers. Develop a new curriculum and schedule trainings.

Homeless Contact and Response

Utilize NYPD HOU as a transport resource across all Providers.

Coordinate outreach so that officers can transport individuals when needed.

Next Step: Determine how many officers could work with each Provider.

Homeless Contact and Response

Segment shelter populations by people with similar goals, values and lifestyles.

For example, create shelters that specialize in services for elderly people or people being released from jails or LGBTQ, etc.

Next Step: Propose how to segment current Shelters or identify new sites.

Homeless Contact and Response; Moving onto Caseload

MEDIUM -TERM (continued)

Develop a coordinated discharge process specifically for street homeless with hospitals.

Next Step: Bring hospital stakeholders to the table, perhaps start with HHC.

Health Crises

Explore re-hiring a DHS Medical Director and hiring high level medical personnel at each Provider.

Ideally there would be one Medical Director for each borough and for after hours.

Next Step: Work with HR to fill the Medical Director role. Explore budget for additional Medical Directors and medical personnel at Providers.

Health Crises

Create a standardized psychosocial form for Providers.

Next Step: Develop a standardized design based on all existing psychosocial forms.

Health Crises; Moving onto Caseload

Remove curfew requirement in shelters, or lessen requirement that belongings will be discarded.

People are rushing out of the hospital when they need services in order to meet curfew and not lose their belongings. Others will not go to shelter at all due to curfew.

Next Step: Establish a first meeting at the leadership level to begin developing shared goals and potential policy changes.

Health Crises; Moving onto Caseload

MEDIUM -TERM (continued)

Add "preventive social services" to hospitals (ERs) including housing, entitlements, etc.

Health Crises; Applying for Public Assistance

Explore creating mobile vans that go to where individuals are on street, Providers' offices, and transitional housing locations.

Services could include: medical, psychiatric services, idNYC, Homebound Public Assistance and other HRA services. Hours and location would vary by needs. The vans could also serve high need and visible areas with non-chronics (e.g. Union Square). The program need to avoid encouraging people to stay outside while still serving service resistant street homeless.

Next Step: Review current mobile services in the City to see how they could be extended.

Health Crises; Applying for Public Assistance

Potentially at HHC hospitals and also health homes, provide a liaison who can access case notes about an individual.

Emergency rooms often turn people away before they can get full information.

Next Step: Look into CARES and Worker Connect use cases for HHC serving homeless individuals

Health Crises; Data and Systems

Create positions for designated psychiatrists and psychiatric services that serve Providers specifically.

Next Step: Discuss additional psychiatric services from DOHMH and see if the ThriveNYC network of mental health providers can be available for these clients.

Health Crises

MEDIUM -TERM (continued)

Create an alert system that lets Providers know when a client has been arrested or admitted to the hospital.

For arrests, the alert could let Provider's know that the client's name has been added to DOC's inmate lookup database. For HHC hospitals, CARES could pull data from a City source.

Next Step: Explore data sources and possibility of using the Worker Connect Common Client Index.

Health Crises; Data and Systems; Special Populations

Create new roles for case management for the non-chronic street homeless.

Next Step: Review funding and staffing needed for new roles.

Moving onto Caseload

Bring all services to Safe Haven clients: medical, psychiatric, nutrition, and applications.

Next Step: Identify highest priority services for clients.

Moving onto Caseload

Provide more services in shelters, including mental health and counseling, education services, job training, and life skills.

Next Step: Catalog current services, gather stakeholders to address gaps and propose additional programming.

Moving onto Caseload

MEDIUM -TERM (continued)

Allow for direct shelter placements by Providers that circumvent the 30th street shelter or other assessment shelters through a smoother process that can be more common.

Many individuals want to stay in their "home" borough or may get lost in the system.

Next Step: Establish a first meeting at the leadership level to begin developing shared goals and potential policy changes.

Moving onto Caseload

Change the way that transitional beds are allocated to each Provider, so that clients don't have go onto waiting lists while other Providers have openings.

Next Step: Review transitional housing allocations and protocol to improve efficiency to give a bed to anyone who wants to come inside.

Moving onto Caseload

Create more Safe Havens.

There are particularly high needs to provide transitional beds for women and to provide facilities in Queens.

Next Step: Explore creating new Safe Havens and targeting them to particular areas and needs.

Moving onto Caseload

MEDIUM -TERM (continued)

Create Safe Havens that are segmented by age and provide living spaces for different genders.

Many clients want to be with other people their age and will be more likely to come in if they know they are sharing a room with peers.

Next Step: Propose how to segment current Safe Havens or build into the plan for new sites.

Moving onto Caseload

Refresh the vulnerability index across all teams to inform prioritization for housing.

It should be consistent across teams and take into account the non-chronically homeless.

Next Step: Collect all vulnerability indices and prioritization processes from teams; determine which groups to target for housing.

Moving onto Caseload; Data and Systems

Enhance immigration services for all Providers.

Next Step: Estimate how many undocumented clients are working with each outreach team. Meet with MOIA to explore options available through the City.

Consider appointing an immigration liaison to assist with specialized barriers for undocumented clients, including assisting with obtaining documentation and funding for fees.

Gathering Vital Documents; Special Populations

MEDIUM -TERM (continued)

Make IDNYC more user-friendly for the street homeless.

Ideas include a mobile application that goes out to homeless, opportunity to get ID without making an appointment, and avenues for clients to get IDs before they may get other photo IDs.

Next Step: Meet with IDNYC team to discuss population needs and programmatic solutions.

Gathering Vital Documents; Applying for Public Assistance; Cross-Agency Initiatives

Revise the benefit card replacement process so that Provider staff and clients do not get disconnected from HRA's information line nor need to go into an HRA Center to receive one.

Next Step: Determine why the Providers and clients can't use the 311 card replacement service request.

Gathering Vital Documents; Applying for Public Assistance; Cross-Agency Initiatives

Make HRA centers more friendly.

Allow lunch breaks or snacks, provide food on site, limit the length of time people are required to stay for appointments, and text clients or case managers when their turn comes while waiting.

Next Step: Explore possibilities for improving the user experience. Develop relationships with food vendors.

Applying for Public Assistance; Cross-Agency Initiatives

MEDIUM -TERM (continued)

Create policy so that one visit to HRA will cover all necessary required steps so clients do not have to go back multiple times.

All HRA centers should have the same procedures and offer the same types of Public Assistance-related services.

Next Step: Convene HRA, DHS and Providers to create an ideal process flow. Follow with a plan of action.

Applying for Public Assistance; Cross-Agency Initiatives

Make work requirements and substance abuse treatment for Public Assistance more flexible for the street homeless.

It is challenging for those still on the street to keep appointments if they do not have a home, place to shower, or keep clean clothing. HRA should consider a suspension until a client is housed. Providers report that current requirements undermine the relationship between caseworker and client.

Next Step: Review work requirements with HRA.

Applying for Public Assistance; Cross-Agency Initiatives

Tie the Public Assistance and Housing application processes more closely together.

This could help avoid situations where Public Assistance is turned off for clients in the middle of their housing application process.

Next Step: Establish a first meeting at the leadership level to begin developing shared goals.

Applying for Public Assistance; Applying for Permanent Housing; Cross-Agency Initiatives

MEDIUM -TERM (continued)

Extend the 2010e application/packet expiration date to at least one year.

Next Step: Meet with PACT on updates to 2010e process. Follow up on which policies govern the expiration of 2010e packets.

Applying for Permanent Housing; Cross-Agency Initiatives

Base housing placements on mental and physical condition of client on application date, and consider services and supports they have needed to achieve an improved state by expiration date.

Next Step: Meet with PACT on updates to 2010e process.

Applying for Permanent Housing; Cross-Agency Initiatives

Increase communication and access between Providers and the PACT team to get clients into appropriate housing.

Next Step: Establish a first meeting at the leadership level to begin developing shared goals.

Applying for Permanent Housing; Cross-Agency Initiatives

Allow Providers to update an expired or expiring housing packet without the risk that the client's housing designation with change to an inappropriate designation.

Next Step: Establish a first meeting at the leadership level to begin developing shared goals.

Applying for Permanent Housing; Cross-Agency Initiatives

MEDIUM -TERM (continued)

Allow for HRA PACT and Providers to case conference around housing packets.

Next Step: Research current PACT workflow to understand the potential impact on current process.

Applying for Permanent Housing; Cross-Agency Initiatives

Reduce barriers to Section 8 briefing.

Streamline the process and implement in small steps. Consider virtual briefings (e.g. Skype).

Next Step: Meet with HPD on updates to briefings process.

Getting Linked with Housing; Cross-Agency Initiatives

Publish transparent data about housing providers' client acceptances and rejections for permanent housing.

Create metrics and goals for outreach Providers based on those housing provider rates.

Next Step: Source Provider acceptance and rejection data.

Getting Linked with Housing; Data and Systems

Develop a mechanism to hold housing providers accountable for unreasonable rejections.

Next Step: Collect data from DHS and Providers on housing providers and historical placements.

Getting Linked with Housing

MEDIUM -TERM (continued)

Standardize aftercare across the Providers and programs.

Current range is 90 days to 1 year based on grant reporting requirements rather than City-rewarded incentives.

Next Step: Review aftercare across providers, establish metrics for success and ways to monitor.

Transitioning into Housing and Aftercare

Develop a standard set of services for each subset of supportive housing.

For example: All NY/NYIII Category A providers should provide on-site psychiatric and medical services. All Cat E providers should have linkage to substance abuse treatment programs in their community.

Next Step: Conduct an assessment of what helps individuals become successful in supportive housing and explore if funding can stipulate what services the housing providers should deliver.

Transitioning into Housing and Aftercare

Create additional programs for clients to enroll or participate in while waiting for housing to become available and after they are placed in permanent housing.

For example: volunteer opportunities, education or training programs, public service opportunities or working for soup kitchens or Providers, etc.

Next Step: Follow-up with NYC Service and outreach Providers on their thoughts.

Transitioning into Housing and Aftercare

MEDIUM -TERM (continued)

Work with Rikers for improved discharge planning of known homeless individuals so they do not go back onto the street.

Next Step: Work with DOC to identify a liaison.

Special Populations

Find a liaison at DOT on encampment issues, confirming jurisdiction of property lines.

Next Step: Work with DOT to identify a liaison.

Cross-Agency Initiatives

Hire a DHS Data Manager who will oversee all internal reporting mechanisms and respond to any internal or external inquiries for data.

They will also serve as the liaison on data with DHS OIT, Mayor's Office of Operations, City Hall, and Department of Social Services.

Next Step: Work with HR to develop Data Manager role.

Also found in: Cross-Agency Initiatives

MEDIUM -TERM (continued)

Define a set of metrics to measure progress and performance across all Providers.

Variables could include:

- number of housing packages in process, including tracking all required elements
- number of placements from street to transitional housing
- number of people placed in permanent housing and maintained for 2 years (currently 1 year)

Next Step: Review current metrics and data points being tracked by Providers and DHS to find commonalities and create an agreed upon set of metrics. Decide on where and how they will be tracked.

Data and Systems

Weekly talk show with a homeless expert to answer questions from the public and explain actual processes used by the City and Providers.

Next Step: Build media elements into public-facing HOME-STAT dashboards.

Public Perception and Action

Increase public awareness of the complexity and reality of the issue through a public conversation.

The conversation should educate New Yorkers about what they can do to help individuals in need and explain actual challenges faced by clients/providers.

Next Step: Consider community meetings or information that could be taken to existing convenings, such as Community Board meetings. Look into precedent of Project Hospitality hosting a community talk to discuss perceptions of homelessness.

Public Perception and Action

MEDIUM -TERM (continued)

Create a new DHS Outreach staff member position to streamline communication of media pieces and rapid response to the public.

Next Step: Explore creating a new position so the DHS analysts spend less time on media-related matters.

Public Perception and Action

SHORT -TERM

Provide additional training for 311 and DHS dispatchers regarding special cases for homeless assistance.

For example, reporting a person in need of assistance on a subway train.

Identifying Street Homeless

Fund more staff to check on vulnerable clients during weather emergencies (Code Red/Blue).

Next Step: Review staffing plans for weather emergencies.

Identifying Street Homeless

SHORT -TERM (continued)

Make outreach teams visible while still being trustworthy to homeless individuals by developing an appropriate balance between the HOME-STAT vests and no branding at all.

Differentiate between Outreach Providers and HOME-STAT canvassers.

Next Step: Create and evaluate design proposals with Providers utilizing HOME-STAT logo.

Identifying Street Homeless

Streamline coordination between outreach shifts to ensure more targeted service delivery to vulnerable clients and clients uncounted or duplicated.

Next Step: Review hand off procedures and standardize across providers.

Identifying Street Homeless

Create a unified street sheet for the City with a cohesive brand and up-to-date information.

Next Step: Follow-up with Providers to get their thoughts.

Identifying Street Homeless

Expand encampment definition to include just one individual.

For example, homeless man's encampment between highway and soccer field in Queens.

Next Step: Review encampment definitions to be shared across all parties.

Identifying Street Homeless; Cross-Agency Initiatives

SHORT -TERM (continued)

Provide parking permits with more expansive options so that outreach teams can pull over where needed to interact with individuals.

Next Step: Contact DOT about enhanced permits.

Identifying Street Homeless; Cross-Agency Initiatives

City-issued checklist for: Code Blue/Red, 9.58 removals, and Emergencies.

Next Step: Develop standardized checklists and protocols and design a set of shared resources.

Homeless Contact and Response; Health Crises

Provide training for doctors about 9.58 removals and the street chronic population to prevent them treating someone for 24 hours and then releasing to the street.

Next Step: Gather training materials from Providers and State Department of Health. Work with HHC to develop curriculum.

Health Crises

Improve knowledge of 9.58s with first responders (FDNY, EMS, and NYPD) through training as part of their on-boarding and ongoing development.

Next Step: Review BRC curriculum created for MTA and NYPD and develop a training model.

Health Crises

SHORT -TERM (continued)

Appoint someone to own relationship between Providers, NYPD, and EMS. Every case to have an assigned executive leader.

Next Step: Identify a key contact for NYPD and EMS. Providers mention that Louis Molina is a helpful contact to escalate issues when NYPD is unresponsive.

Health Crises; Cross-Agency Initiatives

Give Providers access to number of deceased homeless individuals.

Next Step: Confirm existing data sharing channels between DHS and DOHMH.

Health Crises; Data and Systems

Publish transparent information online about the current state of the SPOA Assertive Community Treatment and Care Coordination.

Currently, the NYC.gov page links to a Center for Community Services page that states they no longer run the program and links back to the NYC.gov page.

Next Step: Find out who is responsible for managing ACT and Care Coordination services in New York City.

Health Crises; Data and Systems

Host an "Outreach Team Roadshow" to educate internal DHS groups (especially shelters), Safe Havens and other key stakeholders about the Street Homeless team and provider's services.

Next Step: Develop communication materials.

Moving onto Caseload; Cross-Agency Initiatives

SHORT -TERM (continued)

Assign one HRA Center in each borough that serves homeless individuals.

Next Step: Identify best centers per borough with HRA.

Applying for Public Assistance; Cross-Agency Initiatives

Implement a protocol for DHS Analysts to contact HRA when there are issues with Public Assistance or housing applications.

Next Step: Create new position at DHS to manage new protocol and identify key HRA contacts for PA and 2010E issues.

Applying for Public Assistance; Cross-Agency Initiatives

Designate HRA Center Supervisors or leadership as key contacts for Provider staff and leadership to escalate issues and give feedback.

Next Step: Identify staff contacts from Providers and HRA.

Applying for Public Assistance; Cross-Agency Initiatives

Develop common tracking terms for defining clients and outcomes.

Next Step: Identify all current definitions and propose a consolidated and shared set.

Data and Systems

SHORT -TERM (continued)

Remove filter in Worker Connect that is blocking Outreach information from the CARES data.

Next Step: Review legal history on CARES and Worker Connect Integration.

Data and Systems

Promote Worker Connect as a resource and conduct a training for the DHS Outreach team.

Next Step: Review provisioned users and schedule a training.

Data and Systems

Create a map of 311 requests for homeless assistance, locations of known clients and staff overlay for each area.

Next Step: Export "contact" data from CARES, gather detailed staffing plan from Providers, export 311 log data and compile on a map.

Data and Systems; Public Perception and Action

Post signs in the subway platforms and cars letting New Yorkers know what they can do to help the homeless.

Next Step: Follow-up with Providers and MTA to get their thoughts.

Public Perception and Action

SHORT -TERM (continued)

Increase public awareness through a digital media series that surfaces the complexities of the issues and highlights what the City is doing and what New Yorkers can do to help.

Understanding trends that lead to chronicity. The 2013 NYT Dasani series sparked a public conversation about homelessness and this could mirror that approach.

Next Step: Build media elements into public-facing HOME-STAT dashboards. Explore a wider campaign and press pieces.

Public Perception and Action